ONGO WEIGHT LOSS · BOOK ONE

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The Foundation.

Understanding the Project Before You Start the Work

For every new Ongo teammate: what we treat, what the law allows, who we serve, and how the work fits together.

PREPARED BY

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Team Lead, Ongo Weight Loss

Edition: October 2026 · Book One of the Ongo onboarding series

ONGO WEIGHT LOSS · ONBOARDING SERIES

The Foundation

Understanding the Project Before You Start the Work

Book One of the Ongo onboarding series

Prepared by	Ashish Osten, Team Lead, Ongo Weight Loss
Edition	October 2026 (second edition, rebuilt in the series format)
Written for	Every new Ongo team member, in every role
Read next	Book Two: Earned, Not Built (off-page SEO)
Inside	26 chapters in 4 parts · 260 practice MCQs · 15 illustrations · quick-reference cards and checklists · full 100-competitor scan · A–Z glossary · dated sources for every claim

Important

This book is internal orientation material. It is not medical, legal or regulatory advice for you or for any patient. Real medical questions go to Ongo's licensed providers; legal and regulatory questions go to compliance and counsel.

Drug approvals, FDA enforcement, prices and competitor offers change often. Every claim carries a dated source and an evidence label so you can recheck it. When this book and a newer official source disagree, the official source wins.

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ABOUT THIS BOOK

What's inside

> **Before you start:** this is the first book in the Ongo onboarding series. It doesn't teach you HOW to do your job. It teaches you WHAT this project is, who it serves, what is and isn't legal in our industry, and how the work fits together. Read it cover to cover before opening any other document, including Book Two.

How every chapter is built

Every chapter follows the same pattern, so a newbie always knows where to look.

Part of chapter	What it gives you
Trivia hook	One surprising, sourced fact that shows why the topic matters
Plain explanation	The original Book One content, in very easy English
Desi analogy (Hinglish)	Rotates between restaurant, IT office, cricket, wedding and mohalla
Ongo example	How the idea applies to ongoweightloss.com specifically
Illustration / chart / table	Simple drawings that make the idea land at a glance
Evidence labels	Where each claim comes from (official, peer-reviewed, industry, Ongo)
Do's and Don'ts	Quick rules
Pros and Cons	Honest trade-offs
Where freshers go wrong	Common mistake, why it happens, how to avoid it, how to fix it
Thumb rules + Notes to remember	One-line rules to keep
Glossary	Every new term in the chapter
10 MCQs	Answer key at the foot of the chapter, flipped upside-down
Sources	Every claim linked to where it came from

The four parts

Part	Chapters	What you'll learn
Part 1 · Understanding the Subject	1–6	Obesity, how GLP-1s work, every drug in the family, compounding, when GLP-1s help and when they don't
Part 2 · Understanding the Law	7–9	The FDA and compounding, state rules, and why Ongo offers only compounded medication today
Part 3 · Problem, Solution and How We Work	10–15	The barriers patients face, how Ongo answers them, privacy, the platform, and the journey from booking to the door
Part 4 · The Business and Your Role	16–26	Who we are, what we do and don't do, our sites, our audience, eligibility, pricing, voice, competitors, site fixes, and your first week

What's in this edition

Section	Contents
Chapters	26 chapters in 4 parts
Practice	260 MCQs with flipped answer keys
Illustrations	15 diagrams and charts
Back matter	Quick-reference cards, pre-publish checklist, the full competitor scan, A–Z glossary, all answer keys, and a sources index

BEFORE YOU BEGIN

Front Matter

A Letter to the Reader

You have just joined a healthcare brand in one of the most regulated, most advertised and most closely watched corners of U.S. medicine: **GLP-1 weight-loss telehealth.**

Whatever your role (writing, design, video, code, ads, social, management), you are now part of a team that helps real patients living with a chronic disease find their way through a confusing, expensive and fast-changing system. **The work matters. Accuracy matters. Tone matters.**

The book is told as a story:

- **Part 1** explains obesity, what GLP-1 medications are and how they work, every drug in the family (approved and not), what compounded medication is, and when these drugs help and when they don't.
- **Part 2** explains the law: the FDA, compounding, the states, and why Ongo offers only compounded medication today.
- **Part 3** describes the problem patients face, how we solve it, and exactly how Ongo works from booking to delivery.
- **Part 4** explains the business: who we are, what we will and won't do, our ideal U.S. patient, our pricing, our voice, our 100 competitors, and what still needs fixing on our own site.

Read it once this week. Come back when you forget something. By the end of the week, the language and logic of this brand should feel familiar.

> 📝 **A note before we begin.** Nothing in this book is medical or legal advice for you or for any patient. It is internal orientation material. Real medical questions go to our licensed providers; legal and regulatory questions go to compliance and counsel.
>
> ⚠️ **Edition: October 2026.** Drug approvals, FDA enforcement and competitor offers changed many times in 2025–2026. Treat every dated fact as "true as of October 2026" and re-check it before you publish it.

How to Read This Book

The four parts

Part	Read it when
Part 1	Before you write, design or film anything about medication
Part 2	Before you write any claim about the FDA, pharmacies or states
Part 3	Before you touch the patient journey, tracking or privacy
Part 4	Before you talk about Ongo, its prices, its audience or competitors

The boxes you'll see

Box	Meaning
💡 **Trivia**	A surprising, sourced fact that opens each chapter
🍛 **Desi analogy**	A Hinglish story that makes the idea stick
🟧 **Ongo example**	How the idea applies to ongoweightloss.com
⚠️ **Warning**	Where people get into trouble
👍 **Thumb rule**	A one-line rule to remember
📝 **Note to remember**	A key fact to keep

Using the MCQs for onboarding

- Each chapter ends with 10 multiple-choice questions.
- Answer them before checking the key at the foot of the chapter.
- Team leads can use them as a short quiz after each part. A score of 8/10 or better means the reader is ready to move on.

The Evidence Label Legend

Source labels: where the claim comes from

Label	Meaning	Examples
🟢 **Official**	A government body, regulator, drug label or platform's own documentation	FDA, CDC, HHS, FTC, Google
🟡 **Peer-reviewed / medical reference**	A published clinical study or medical textbook	NEJM trials, StatPearls
🟠 **Industry**	Reputable news, legal analysis or industry research	Law-firm alerts, KFF polls, trade press
🔵 **Ongo / expert**	Ongo's own site, internal data, leadership confirmation, or expert judgment	Pricing page, competitor scan

How strong is each label?

Label	How much to trust it	What to do before publishing
🟢	Highest	Check it's still current
🟡	High	Name the study and the product it tested
🟠	Medium	Prefer the original official source if one exists
🔵	Varies	Confirm with the owner (operations, compliance, leadership)

Risk level: what can go wrong

Most risk in this book comes from three places: **sameness claims** (compounded vs. brand), **privacy** (PHI in the wrong place) and **prices** (inconsistent numbers). The "Where freshers go wrong" box in every chapter shows how to avoid and fix each one.

How the Answer Keys Work

Every chapter's answer key sits at the foot of the chapter, printed **upside down**. Try the quiz first, then turn the page (or your screen) around to check. All 26 keys are also collected in the back matter.

01

PART 1

Understanding the Subject

The disease, the medicine family, compounding, and the honest limits.

1 · What Is Overweight and Obesity?

2 · What GLP-1 Medications Are and How They Work

3 · The GLP-1 Family: Every Drug, Approved and Not Approved

4 · Compounded Medications: What They Are, How They Work, How They're Made

5 · When GLP-1 Medication Helps

6 · When GLP-1 Medication Won't Help, or Isn't Safe

PART 1 · CHAPTER 1

Chapter 1: What Is Overweight and Obesity?

Before anything else, the team needs a clean, unhyped definition of the condition we treat.

> 💡 **Trivia**
>
> Between August 2021 and August 2023, about **40.3% of U.S. adults** had obesity, and about **9.4%** had severe obesity. [1] ● Official
>
> That's roughly 4 in every 10 adults you pass on the street. Our audience isn't a niche. It's a big share of the country, and every one of them has been judged about their weight at some point.

1.1 The plain definition

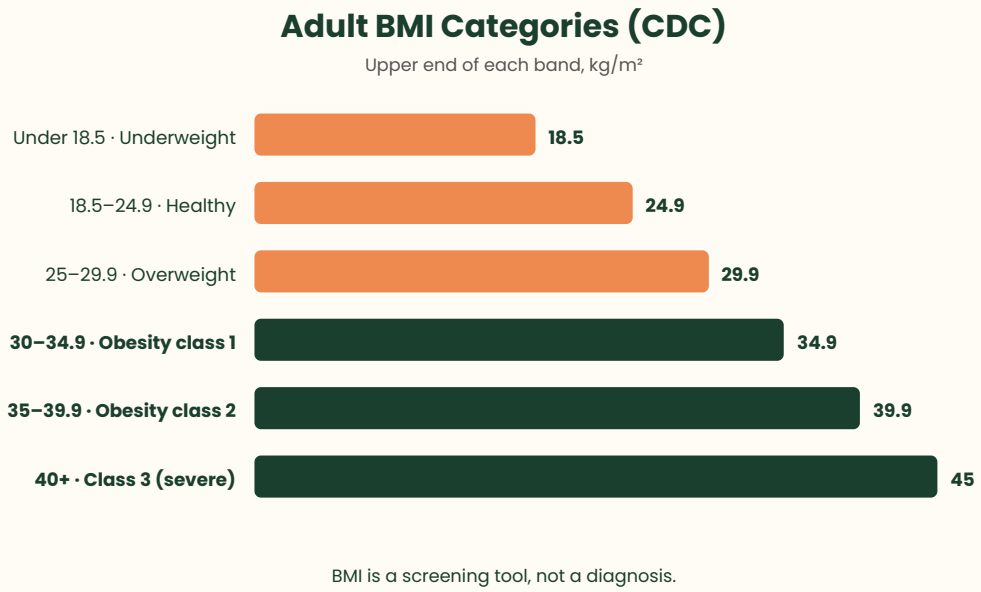
Obesity is a **chronic, relapsing disease** in which excess body fat harms health. The American Medical Association voted to recognize obesity as a disease in **June 2013**. [2] ● Industry

It is **not** a character flaw, a lack of willpower or a cosmetic issue. That framing is the foundation of everything we publish.

> 🍛 **Desi analogy: mohalla ki sugar wali aunty**
>
> Mohalle mein sab jaante hain ki Sharma aunty ko sugar hai. Koi nahi kehta "aunty, aapne mehnat nahi ki." Sab kehte hain "dawai time pe lo, doctor ko dikhao."
>
> **Obesity ke saath bhi wahi izzat chahiye.** Yeh bimari hai, laziness nahi. Hamari har line isi soch se likhi jaati hai.

1.2 How clinicians measure it: BMI

Clinicians usually start with **Body Mass Index (BMI)**: weight in kilograms divided by height in meters squared. CDC's adult categories: [3] ● Official



BMI (adults)	Category
Below 18.5	Underweight
18.5 – 24.9	Healthy weight range
25.0 – 29.9	Overweight
30.0 – 34.9	Obesity, class 1
35.0 – 39.9	Obesity, class 2
40.0 and above	Obesity, class 3 (severe)

BMI is a screening tool, not a diagnosis. It doesn't separate muscle from fat, and lower cut-offs are often used for people of Asian descent. Providers also look at waist size, medical history and weight-related conditions. ● Expert

1.3 What drives it

Driver	Example
Genetics	Family history of obesity
Hormones and metabolism	How the gut and brain signal hunger and fullness
Medications	Some antidepressants, antipsychotics, steroids, insulin
Lifestyle and environment	Sleep, stress, food access, shift work
Life stages	Pregnancy, menopause

> 🟧 **Ongo example**
>
> When you write about causes, list **several** drivers together. Never write a page that implies "people with obesity just eat too much." That's inaccurate and off-brand.

1.4 Why it matters medically

Excess weight raises the risk of type 2 diabetes, high blood pressure, heart disease, stroke, sleep apnea, fatty liver disease (MASLD), osteoarthritis and several cancers. **Losing even 5–10% of body weight can improve many of these.** 🔵 Expert

How common obesity is in the U.S., and who it affects most, is covered in Chapter 19.

1.5 Person-first language

✅ Say	❌ Never say
"A person with obesity"	"Obese people"
"People living with overweight"	"Fat", "chubby", "heavy people"
"Weight-related health conditions"	"Lazy", "no self-control"
"Managing weight"	"Fixing your body"

> 📝 **Note to remember:** Never imply weight is a personal failure, in a blog, an ad, a video caption or a social reply.

1.6 Do's and Don'ts

✅ Do	❌ Don't
Call obesity a chronic disease	Call it a lifestyle choice
Explain BMI as a screening tool	Treat BMI as a full diagnosis
List multiple causes	Blame food or willpower alone
Use person-first language	Use shaming or slang words

1.7 Pros and cons of using BMI in our content

✅ Pros	❌ Cons
Simple, widely understood	Doesn't separate muscle from fat
Matches FDA label thresholds (Chapter 20)	Can mislead for some ethnic groups and athletes
Easy to build into a calculator	Patients may read it as a judgment

1.8 Where freshers go wrong in Chapter 1 topics

Mistake	Why it happens	Avoid it by	Fix it by
Writing "obese people"	It's common online	The person-first table	Editing every instance before publishing
Calling BMI a diagnosis	Over-simplifying	Saying "screening tool"	Adding the provider-review line

Mistake	Why it happens	Avoid it by	Fix it by
Blaming diet alone	Copying generic fitness blogs	Listing several drivers	Rewriting the causes section
Using before/after "shame" hooks	They get clicks	Brand voice rules (Chapter 23)	Replacing with supportive copy

1.9 Thumb rules

👍 Obesity is a disease, not a decision.

👍 BMI opens the conversation; the provider closes it.

👍 Person first, condition second.

1.10 Notes to remember

📝 About 40.3% of U.S. adults have obesity; about 9.4% have severe obesity (CDC, 2021–2023).

📝 BMI 30+ is obesity; 25–29.9 is overweight.

Chapter 1 Glossary

Term	Simple meaning
Obesity	A chronic disease of excess body fat that harms health; usually screened as BMI 30+
Overweight	BMI 25–29.9
BMI	Body Mass Index: weight (kg) ÷ height (m)². A screening tool, not a diagnosis
Severe obesity	BMI 40 or higher (class 3)

Term	Simple meaning
Person-first language	Putting the person before the condition ("a person with obesity")
MASLD	Metabolic dysfunction-associated steatotic liver disease (fatty liver)

Chapter 1 Quiz

1. How does Ongo describe obesity?

- A) A lack of willpower
- B) A cosmetic issue
- C) A chronic, relapsing disease
- D) A lifestyle choice

2. When did the AMA recognize obesity as a disease?

- A) 2020
- B) 2025
- C) 1996
- D) 2013

3. What share of U.S. adults had obesity in 2021–2023 (CDC)?

- A) About 40%
- B) About 60%
- C) About 25%
- D) About 10%

4. Which BMI range is "obesity, class 1"?

- A) 25.0–29.9
- B) 35.0–39.9
- C) 40 and above
- D) 30.0–34.9

5. What is BMI?

- A) A blood test

- B) A screening tool, not a diagnosis
- C) A full medical diagnosis
- D) A measure of muscle mass

6. Which phrase follows our language rules?

- A) "Fat loss customers"
- B) "Chubby patients"
- C) "Obese people"
- D) "A person with obesity"

7. Losing how much body weight can improve many weight-related conditions?

- A) At least 50%
- B) Nothing helps below 30%
- C) Even 5–10%
- D) Exactly 1%

8. Which is NOT listed as a driver of obesity?

- A) Choosing to be unhealthy on purpose
- B) Some medications
- C) Genetics
- D) Sleep and stress

9. Severe obesity is...

- A) Any BMI with diabetes
- B) BMI 40 or higher
- C) BMI 25 or higher
- D) BMI 30 exactly

10. In the mohalla analogy, how should we treat obesity?

- A) As something to hide
- B) As a joke
- C) With the same respect as diabetes: a condition needing care
- D) As laziness

Sources

1. CDC National Center for Health Statistics, Obesity and Severe Obesity Prevalence in Adults: United States, August 2021–August 2023 (Data Brief No. 508) (September 2024)
2. Obesity Medicine Association, AMA adopts policy to recognize obesity as a disease (June 2013)
3. CDC, Adult BMI Categories

Answer key, Chapter 1: 1-C · 2-D · 3-A · 4-D · 5-B · 6-D · 7-C · 8-A · 9-B · 10-C

PART 1 · CHAPTER 2

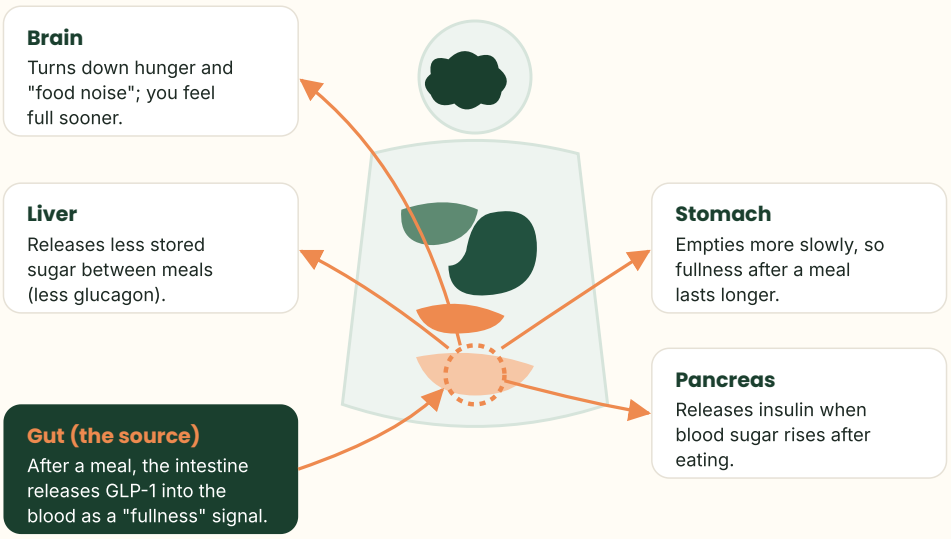
Chapter 2: What GLP-1 Medications Are and How They Work

GLP-1 medications copy a hormone your body already makes after you eat. They are the reason this industry exists.

> 💡 **Trivia**
>
> Natural GLP-1 is broken down by an enzyme called **DPP-4** within **minutes** of being released; about two minutes is the figure most textbooks quote. [1][2] ● Medical reference
>
> That tiny lifespan is the whole story of this chapter: the medicine works because it's built to survive much, much longer.

2.1 The hormone

GLP-1 (glucagon-like peptide-1) is released by the **gut** when food arrives. It sends messages to four places at once: [1] ●

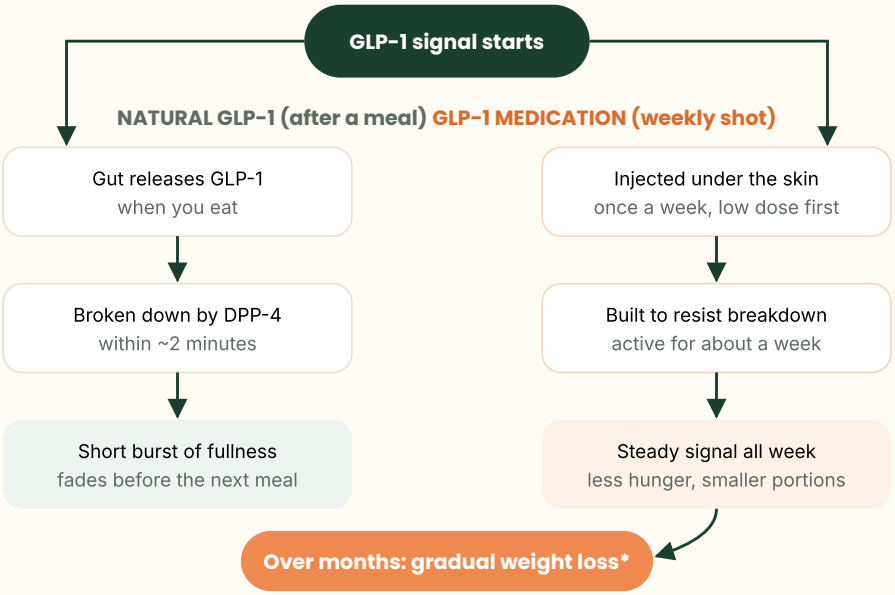


Simplified illustration for team training — not to anatomical scale.

Organ	Message	Effect
Brain	"You've had enough"	Less hunger, fewer cravings, less "food noise"
Stomach	"Slow down"	Empties more slowly, so you feel full longer
Pancreas	"Release insulin"	Insulin when blood sugar rises (and only then)
Liver	"Hold back"	Less stored sugar (glucose) released into the blood

2.2 Why the natural hormone isn't enough, and what the medicine changes

Natural GLP-1 is a **short burst** that fades before your next meal. A GLP-1 **medication** is engineered to resist DPP-4, so one weekly injection keeps the "full" signal running for about a week. [1]



Treatment always starts at a **low dose** and increases step by step (**titration**) so the stomach and gut can adjust. That's why Ongo talks about starting low: it's standard practice, not a weaker treatment. [3] ● Official

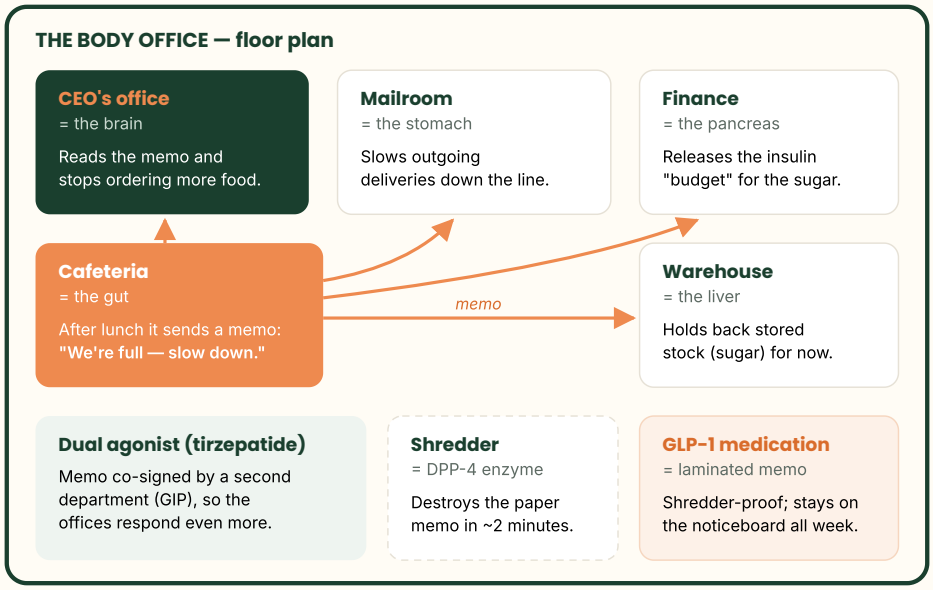
2.3 Single vs. dual vs. triple agonists

Type	Activates	Example	Status
GLP-1 receptor agonist (single)	GLP-1 only	Semaglutide	Approved brands exist
Dual agonist	GIP + GLP-1	Tirzepatide	Approved brands exist
Triple agonist	GLP-1 + GIP + glucagon	Retatrutide	In trials
Amylin combination	Amylin + GLP-1	CagriSema (cagrilintide + semaglutide)	Under FDA review

GIP is a second gut hormone that adds to the fullness and insulin effects. **Amylin** is a pancreatic fullness hormone.

2.4 The analogy: your body is an office

If you need to explain GLP-1s to a patient, a friend or a new designer, use this.



Office part	Body part	What it does
Cafeteria	Gut	Sends a memo after lunch: *"We're full, slow down."*
CEO	Brain	Reads the memo and stops ordering food
Mailroom	Stomach	Slows outgoing deliveries
Finance	Pancreas	Releases the insulin budget for the sugar that came in
Warehouse	Liver	Holds back stored stock
Shredder	DPP-4 enzyme	Destroys the paper memo in about two minutes
Laminated memo	GLP-1 medication	Can't be shredded; stays on the noticeboard all week
Co-signed memo	Dual agonist (tirzepatide)	A second department (GIP) signs, so offices respond more strongly

Stopping the medication takes the laminated memo off the board. The shredder is back in charge, which is why weight often returns after treatment stops (Chapter 6).

> 🍛 **Desi analogy: IT office ka circular**
>
> Office mein HR ek circular bhejta hai: "Aaj se lunch ke baad snacks band." Agar circular email se aaye toh log do minute mein bhool jaate hain. Agar notice board pe **laminate karke chipka diya**, toh poora hafta sab follow karte hain.
>
> **GLP-1 dawai woh laminated notice hai.** Body ka natural signal email jaisa hai: jaldi gayab.
>
> ⚠️ **What the analogy is not.** It's a teaching aid, not a medical claim. Don't put "laminated memo" language in clinical copy, and never use it to promise a result.

2.5 Do's and Don'ts

✅ Do	❌ Don't
Explain the four messages (brain, stomach, pancreas, liver)	Say GLP-1s "burn fat"
Explain titration as standard care	Call low doses "weak"
Separate single, dual and triple agonists clearly	Mix up semaglutide and tirzepatide
Use the office analogy for training	Use it as a medical claim

2.6 Pros and cons of explaining the mechanism in content

✅ Pros	❌ Cons
Builds trust and understanding	Easy to over-simplify into a false claim
Strong entity coverage for SEO	Needs medical review every time
Helps patients expect side effects	Can drift into promising results

2.7 Where freshers go wrong in Chapter 2 topics

Mistake	Why it happens	Avoid it by	Fix it by
"GLP-1 melts fat"	Copying ad hooks	Using the four-message table	Rewriting to "reduces hunger"
Calling tirzepatide a GLP-1 only	Lumping drugs together	Saying "dual GIP/GLP-1"	Correcting entity mentions
Saying titration means "weak dose"	Misunderstanding	Explaining step-up dosing	Medical-review edit
Using the office analogy in clinical copy	It's catchy	Keeping it for training	Removing it from the page

2.8 Thumb rules

👍 Natural GLP-1: two minutes. Medicine: about a week.

👍 Semaglutide = single. Tirzepatide = dual.

👍 Analogies teach; they never promise.

2.9 Notes to remember

📝 GLP-1 tells the brain, stomach, pancreas and liver to slow down after a meal.

📝 Stopping the medication removes the "laminated memo", and weight often returns.

Chapter 2 Glossary

Term	Simple meaning
GLP-1	Glucagon-like peptide-1, a gut hormone that affects insulin, stomach emptying and fullness

Term	Simple meaning
GLP-1 receptor agonist	A drug that activates the GLP-1 receptor (e.g. semaglutide)
GIP	A second gut hormone; tirzepatide activates both GIP and GLP-1 receptors
Dual agonist	A drug that activates two hormone receptors
Triple agonist	A drug that activates three receptors (GLP-1, GIP and glucagon)
Amylin	A pancreatic hormone that adds fullness
DPP-4	The enzyme that breaks down natural GLP-1 within minutes
Titration	Starting low and increasing the dose step by step
Food noise	Patient term for constant thoughts about food

Chapter 2 Quiz

1. Where is GLP-1 released?

- A) The gut, when food arrives
- B) The brain, during sleep
- C) The skin, after exercise
- D) The liver, at night

2. Which enzyme breaks down natural GLP-1?

- A) DPP-4
- B) Lipase
- C) Insulin
- D) Amylase

3. Roughly how long does natural GLP-1 last?

- A) About a month
- B) About a day
- C) About a week
- D) About two minutes

4. What does GLP-1 tell the stomach?

- A) Stop digesting completely
- B) Make more acid
- C) Empty more slowly
- D) Empty faster

5. Tirzepatide is a...

- A) Amylin-only drug
- B) Single GLP-1 agonist
- C) Triple agonist
- D) Dual GIP/GLP-1 agonist

6. Why does treatment start at a low dose?

- A) So the body can adjust (titration)
- B) Because low doses are weaker products
- C) Because the law requires a fixed dose
- D) To save money

7. In the office analogy, what is the shredder?

- A) The pancreas
- B) The medication
- C) The brain
- D) The DPP-4 enzyme

8. Retatrutide is...

- A) An amylin-only pill
- B) A compounded product Ongo sells
- C) A triple agonist still in trials
- D) An approved weight brand

9. Can the office analogy be used in clinical copy to promise results?

- A) Yes, always
- B) Only in ads
- C) Only on social media
- D) No; it's a teaching aid only

10. In the IT office circular analogy, the GLP-1 medicine is…

- A) The laminated notice that stays all week
- B) The lunch break
- C) The HR manager
- D) The email people forget

Sources

1. StatPearls (NCBI Bookshelf), Glucagon-Like Peptide-1 Receptor Agonists
2. Holst JJ, The physiology of glucagon-like peptide 1, Physiological Reviews (2007): GLP-1 is rapidly inactivated by DPP-4
3. Drugs.com, Wegovy prescribing information (dose-escalation schedule)

Answer key, Chapter 2: 1-A · 2-A · 3-D · 4-C · 5-D · 6-A · 7-D · 8-C · 9-D · 10-A

PART 1 · CHAPTER 3

Chapter 3: The GLP-1 Family: Every Drug, Approved and Not Approved

Patients search for and compare all of these. The team must know which exist, who makes them, what they're approved for, and that Ongo offers none of the brand-name products.

> 💡 **Trivia**
>
> In just over three months, the FDA approved **two new GLP-1 pills for weight**: the Wegovy pill (oral semaglutide) on **December 22, 2025**, and Foundayo (orforglipron) on **April 1, 2026**. [1] [2] 🟠 Industry
>
> This list changes every few months. A page that was accurate in January can be wrong by April.

3.1 The one rule before the tables

Ongo offers **only compounded semaglutide and compounded tirzepatide** (Chapter 4). We offer **none** of the brand-name products below. 🔵 Ongo

> ⚠️ **Status check.** This list is current as of October 2026. New approvals, discontinuations and trial results arrive every few months. Re-check before publishing any approval date or status.

3.2 FDA-approved for chronic weight management

Brand (generic name)	Maker	Type	Form	Approved for	Status note
Wegovy (semaglutide 2.4 mg)	Novo Nordisk	GLP-1	Weekly injection	Weight management (2021); heart-risk reduction; MASH liver disease	Most-searched weight brand
Wegovy HD (semaglutide 7.2 mg)	Novo Nordisk	GLP-1	Weekly injection	Higher-dose weight management	Approved Mar 19, 2026 [3]
Wegovy pill (oral semaglutide 25 mg)	Novo Nordisk	GLP-1	Daily tablet	Weight management	Approved Dec 22, 2025 [1]
Zepbound (tirzepatide)	Eli Lilly	Dual GIP/GLP-1	Weekly injection or vial	Weight management (Nov 2023); sleep apnea with obesity	Main rival to Wegovy
Foundayo (orforglipron)	Eli Lilly	GLP-1 (small molecule)	Daily pill	Weight management	Approved Apr 1, 2026; no food/water timing rules [2]
Saxenda (liraglutide 3 mg)	Novo Nordisk	GLP-1	Daily injection	Weight management (2014; teens 12+)	Being discontinued in the U.S.; last wholesaler shipments late January 2027 [4]
Generic liraglutide 3 mg	Teva and others	GLP-1	Daily injection	Weight management	First generic weight GLP-1 (Aug 2025) [5]

3.3 FDA-approved for type 2 diabetes (not labeled for weight loss)

Brand (generic name)	Maker	Type	Form	Status note
Ozempic (semaglutide)	Novo Nordisk	GLP-1	Weekly injection	Also kidney and heart indications; widely used off-label for weight
Rybelsus (oral semaglutide)	Novo Nordisk	GLP-1	Daily tablet	First oral GLP-1 (2019)
Mounjaro (tirzepatide)	Eli Lilly	Dual GIP/GLP-1	Weekly injection	Same molecule as Zepbound
Trulicity (dulaglutide)	Eli Lilly	GLP-1	Weekly injection	Diabetes and heart-risk reduction
Victoza (liraglutide 1.8 mg) + generics	Novo Nordisk; Teva	GLP-1	Daily injection	Generic available
Byetta / Bydureon BCise (exenatide)	AstraZeneca	GLP-1	Twice-daily / weekly injection	Oldest GLP-1 (2005); check current U.S. availability
Xultophy (insulin degludec + liraglutide)	Novo Nordisk	Insulin + GLP-1	Daily injection	Combination product
Soliqua (insulin glargine + lixisenatide)	Sanofi	Insulin + GLP-1	Daily injection	Combination product
Adlyxin (lixisenatide) / Tanzeum (albiglutide)	Sanofi / GSK	GLP-1	Injection	Discontinued in the U.S.

3.4 NOT FDA-approved (in trials, under review, or compounded)

Drug	Maker	Type	Status (Oct 2026)
CagriSema (cagrilintide + semaglutide)	Novo Nordisk	Amylin + GLP-1	Application submitted Dec 2025; FDA review in 2026 [6]

Drug	Maker	Type	Status (Oct 2026)
Retatrutide	Eli Lilly	Triple GLP-1/GIP/glucagon	Phase 3 trials
Survodutide	Boehringer Ingelheim	Dual GLP-1/glucagon	Phase 3 (weight and liver disease)
Mazdutide	Innovent / Lilly	Dual GLP-1/glucagon	Approved in China only; not in the U.S.
VK2735	Viking Therapeutics	Dual GLP-1/GIP	Phase 3 (injection) and oral trials
MariTide (maridebart cafraglutide)	Amgen	GLP-1 agonist + GIP blocker, monthly	Phase 3
Petrelintide / Amycretin	Zealand–Roche / Novo Nordisk	Amylin / GLP-1 + amylin	Phase 2–3
Compounded semaglutide	State-licensed pharmacies	GLP-1	Legal to prescribe when clinically justified; **not FDA-approved**. Ongo offers this [7]
Compounded tirzepatide	State-licensed pharmacies	Dual GIP/GLP-1	Legal to prescribe when clinically justified; **not FDA-approved**. Ongo offers this [7]
"Research peptides", GLP-1 drops, patches, supplements	Unlicensed sellers	—	Illegal or unproven; we never sell or endorse them

> 🍛 **Desi analogy: cricket team selection**
>
> Playing XI (FDA-approved), bench pe baithe players (trials mein), aur gully cricket ke log jo jersey pehen ke khud ko "India team" bolte hain (research peptides). **Commentator ka kaam hai sahi batana kaun kis team mein hai.**
>
> Hum commentator hain, team ke owner nahi. Hum sab ke baare mein bata sakte hain, par yeh nahi keh sakte ki Virat hamari team mein khel raha hai.

3.5 The rule for brand names

We **can** name these drugs to educate and compare. We can **never**:

- imply Ongo sells them,
- say our compounded products are "the same as", "generic" or "equivalent to" any of them,
- apply their trial results to our products. [8] 🟢 Official

3.6 Do's and Don'ts

✅ Do	❌ Don't
Date every approval or status you mention	Copy an old list without checking
Say "Ongo does not offer this brand" on comparison pages	Let a brand page imply we sell it
Name the maker and the approved use	Call Ozempic a weight-loss drug on its label
Warn against research peptides	Link to or review unlicensed sellers

3.7 Pros and cons of covering every brand in our content

✅ Pros	❌ Cons
Matches what patients actually search	High risk of implying we sell brands
Strong entity and topical coverage	Goes stale quickly
Builds trust through honesty	Needs constant medical and compliance review

3.8 Where freshers go wrong in Chapter 3 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Ozempic for weight loss" in a heading	It's how people search	Saying "approved for type 2 diabetes"	Rewriting the heading with the label use

Mistake	Why it happens	Avoid it by	Fix it by
Missing the "we don't offer this" line	Assuming it's obvious	The brand-name rule	Adding the line above the fold
Quoting an approval date from memory	Overconfidence	Checking the source	Updating with a dated citation
Calling compounded tirzepatide "generic Zepbound"	Shorthand	The brand rule	Correcting immediately; tell your lead

3.9 Thumb rules

👍 Name every brand; sell none of them.

👍 No date, no status.

👍 "Generic", "same as", "equivalent": three words we never pair with compounded.

3.10 Notes to remember

📝 Weight-approved: Wegovy (injection, HD, pill), Zepbound, Foundayo, Saxenda, generic liraglutide.

📝 Diabetes-only labels: Ozempic, Rybelsus, Mounjaro, Trulicity, Victoza and others.

Chapter 3 Glossary

Term	Simple meaning
Semaglutide	GLP-1 drug; brands Wegovy (weight), Ozempic and Rybelsus (diabetes), made by Novo Nordisk
Tirzepatide	Dual GIP/GLP-1 drug; brands Zepbound (weight) and Mounjaro (diabetes), made by Eli Lilly

Term	Simple meaning
Orforglipron	Lilly's small-molecule daily GLP-1 pill, brand Foundayo
Liraglutide	Older daily GLP-1; brands Saxenda and Victoza, now also generic
Oral GLP-1	A GLP-1 taken as a pill (Rybelsus, Wegovy pill, Foundayo)
Off-label	Prescribing a drug for a use not on its FDA label
Phase 3	Large late-stage trial before an FDA application
Research peptides	Unapproved products sold "for research"; never for patients

Chapter 3 Quiz

1. Which brand-name GLP-1 products does Ongo offer?

- A) All of them
- B) None
- C) Ozempic only
- D) Wegovy and Zepbound

2. When was the Wegovy pill approved?

- A) It isn't approved
- B) June 2021
- C) December 22, 2025
- D) April 1, 2026

3. Foundayo is...

- A) Lilly's daily orforglipron pill for weight
- B) A research peptide
- C) A compounded injection
- D) A diabetes-only drug

4. Ozempic's FDA label is for...

- A) Type 2 diabetes (plus kidney and heart indications)

- B) Sleep apnea
- C) Chronic weight management
- D) Nothing; it's unapproved

5. Zepbound and Mounjaro share which molecule?

- A) Tirzepatide
- B) Semaglutide
- C) Dulaglutide
- D) Liraglutide

6. What's happening to Saxenda in the U.S.?

- A) Nothing
- B) It's becoming a pill
- C) It's being relaunched
- D) It's being discontinued; last shipments late January 2027

7. What is CagriSema's status (Oct 2026 edition)?

- A) Banned
- B) Submitted to the FDA; under review in 2026
- C) Approved since 2021
- D) Sold by Ongo

8. Which phrase may we NEVER use about our compounded products?

- A) "Prepared by a licensed pharmacy"
- B) "Not FDA-approved"
- C) "Generic Zepbound"
- D) "Prescribed when appropriate"

9. Research peptides, GLP-1 drops and patches are…

- A) Approved alternatives
- B) Safe for teens
- C) Illegal or unproven; we never sell or endorse them
- D) What Ongo ships

10. In the cricket analogy, Ongo's role is…

- A) The gully player in an India jersey

- B) The team owner of every player
- C) The commentator who tells you accurately who's on which team
- D) The umpire who approves drugs

Sources

1. Pharmaceutical Commerce, FDA approves Novo Nordisk's Wegovy pill (December 24, 2025)
2. Healio, FDA approves orforglipron, an oral GLP-1 for adults with obesity (April 1, 2026)
3. HCPLive, FDA approves higher-dose semaglutide (Wegovy HD) 7.2 mg for obesity (March 19, 2026)
4. Novo Nordisk (NovoMedLink), Saxenda: U.S. discontinuation notice
5. Pharmacy Times, FDA approves generic version of liraglutide injection for weight loss (August 28, 2025)
6. Drugs.com, CagriSema FDA approval status
7. U.S. FDA, FDA's concerns with unapproved GLP-1 drugs used for weight loss (content current as of October 1, 2026)
8. McDermott, FDA takes aim at misleading online claims for compounded GLP-1 drugs (June 2026)

Answer key, Chapter 3: 1-B · 2-C · 3-A · 4-A · 5-A · 6-D · 7-B · 8-C · 9-C · 10-C

PART 1 · CHAPTER 4

Chapter 4: Compounded Medications: What They Are, How They Work, How They're Made

Ongo's only two treatments are compounded. Every team member must be able to explain what that means, honestly.

> 💡 **Trivia**
>
> As of May 31, 2026, the FDA had received **990 adverse event reports** linked to compounded semaglutide and **more than 730** linked to compounded tirzepatide, many involving **dosing errors**. The FDA adds that under-reporting is likely. [1] ● Official
>
> That's why our kits, tutorials and dosing instructions are not "extras". They are patient safety.

4.1 What compounding is

Compounding is when a **licensed pharmacist prepares a medication for one specific patient's prescription**, instead of a manufacturer mass-producing it. It has existed for centuries: a pharmacist making a liquid version of a pill for a child who can't swallow tablets, or leaving out a dye someone is allergic to. [2] ● Official

Compounded semaglutide and compounded tirzepatide contain the same *active molecules* as the brands, but they are **different products**:

- they are **not FDA-approved**,
- the FDA **hasn't evaluated** them for safety, effectiveness or quality,
- the brands' clinical trials were **not** done on them. [1] ● Official

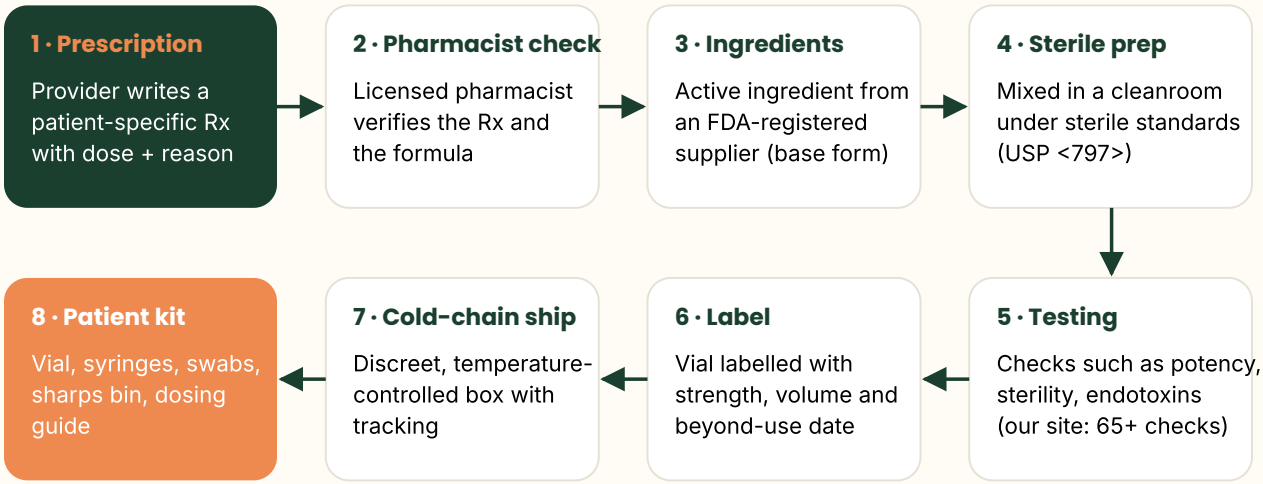
> **Desi analogy: shaadi ka darzi vs. showroom suit**
>
> Showroom ka branded suit factory mein bana hai, quality check hua hai, size fixed hai. Shaadi ke liye mohalle ka darzi tumhare naap se suit silta hai: fitting perfect, daam kam.
>
> Par darzi yeh nahi bol sakta ki "yeh Raymond ka suit hai." **Kapda same ho sakta hai, product alag hai.** Compounded dawai bhi aisi hi hai.

4.2 How they work in the body

The molecule does the work, so the mechanism is the one in Chapter 2: semaglutide activates the GLP-1 receptor; tirzepatide activates GIP and GLP-1.

What differs is the **product**: the concentration, the inactive ingredients, the vial-and-syringe format, and the quality controls behind it.

4.3 How they're prepared



Step	What happens
1. Prescription	A licensed provider writes a patient-specific prescription with a documented clinical reason

Step	What happens
2. Pharmacist check	The pharmacist reviews the dose, the patient's allergies and the prescription
3. Ingredients	The active ingredient comes from an FDA-registered supplier, in its base form. **Salt forms** (e.g. semaglutide sodium or acetate) must not be used [1] 🟢
4. Sterile preparation	Injectables are made in a cleanroom under **USP <797>** sterile-compounding standards [3] 🟢
5. Testing	Batches are tested for potency, sterility and contaminants. Ongo's site says "65+ quality checks"; ask compliance what those checks are before you repeat the number 🔵
6. Labeling	The vial carries the patient name, dose, the compounding pharmacy's name and a beyond-use date (BUD)
7. Cold-chain shipping	Packed with insulation and cold packs to protect the medication
8. Patient kit	Vial(s), syringes, alcohol swabs and instructions or tutorials arrive at the door

4.4 503A vs. 503B

Type	Who regulates	What it can do
503A pharmacy	State boards of pharmacy	Prepares medication for individual prescriptions. Ongo's partner pharmacy works this way
503B outsourcing facility	Registered with the FDA	Can make larger batches under stricter manufacturing rules (cGMP)

Source: [2] 🟢 Official

> 🟧 **Our partner pharmacy**
>
> Public page copy says **"our licensed U.S. partner pharmacy"**, never the pharmacy's name. Any change to how the pharmacy is disclosed on labels or pages goes through compliance, because FDA warning letters have cited telehealth brands for putting their own name on labels instead of identifying the compounder. [4] 🟠

4.5 Compounded vs. brand at a glance

FDA-approved brand (e.g. Wegovy, Zepbound)	Compounded (what Ongo offers)
Reviewed and approved by the FDA	Not FDA-approved; not FDA-evaluated for safety, effectiveness or quality
Made by the manufacturer in FDA-inspected plants	Prepared by a state-licensed pharmacy for one patient's prescription
Fixed doses in pens or tablets	Doses prepared to the prescription, usually vial + syringe
Its own clinical-trial results	Brand trial results can't be claimed for it
Higher cash price; sometimes insured	Lower cash price; not insured

4.6 Do's and Don'ts

✅ Do	❌ Don't
Say "not FDA-approved" clearly and near the claim	Hide the disclosure in a footer
Explain the vial-and-syringe format	Show brand-style pens
Stress dosing instructions and tutorials	Treat units vs. milligrams casually
Say "licensed U.S. partner pharmacy"	Name the pharmacy or call it "FDA-approved"

4.7 Pros and cons of compounded medication (honest view)

✅ Pros	❌ Cons
Lower cash price	Not FDA-approved or FDA-evaluated
Dose can be personalized	Dosing errors are a known risk
Available without insurance	Quality depends on the pharmacy

✅ Pros	❌ Cons
Shipped to the door	Rules and enforcement keep changing

4.8 Where freshers go wrong in Chapter 4 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Same active ingredient as Ozempic"	It sounds reassuring	The brand rule (Chapter 3)	Removing the line; compliance check
Naming the partner pharmacy publicly	Internal habit	"Licensed U.S. partner pharmacy"	Editing and telling your lead
Repeating "65+ quality checks" without detail	Copying the site	Asking compliance first	Adding what the checks are, or removing
Calling the pharmacy "FDA-approved"	Confusing drugs and pharmacies	Remembering the FDA approves drugs, not pharmacies	Correcting the copy

4.9 Thumb rules

👍 Same molecule, different product.

👍 Not FDA-approved, said clearly, every time.

👍 The kit and the tutorial are safety features, not extras.

4.10 Notes to remember

📝 503A = patient-specific, state-licensed. 503B = FDA-registered, larger batches.

📝 Salt forms (sodium, acetate) must not be used to compound semaglutide.

Chapter 4 Glossary

Term	Simple meaning
Compounded medication	A drug prepared by a licensed pharmacy for one patient's prescription; not FDA-approved
API	Active pharmaceutical ingredient, the molecule that does the work
Salt form	e.g. semaglutide acetate; treated by the FDA as a different, unapproved ingredient
503A pharmacy	State-licensed compounding pharmacy filling patient-specific prescriptions
503B outsourcing facility	FDA-registered facility that can compound larger batches
USP <797>	The national standard for sterile compounding
Cleanroom	A controlled, filtered space where sterile medications are prepared
BUD	Beyond-use date: the date after which a compounded medication shouldn't be used
Cold chain	Temperature-controlled shipping

Chapter 4 Quiz

1. What is compounding?

- A) Buying research peptides online
- B) Mixing supplements at home
- C) A manufacturer mass-producing a brand drug
- D) A pharmacist preparing a medication for one patient's prescription

2. Are compounded semaglutide and tirzepatide FDA-approved?

- A) No
- B) Only tirzepatide
- C) Yes
- D) Only when shipped cold

3. Which ingredient forms must NOT be used to compound semaglutide?

- A) Salt forms such as sodium or acetate
- B) Sterile water
- C) Any FDA-registered supplier's ingredient
- D) The base form

4. Which standard governs sterile compounding?

- A) USP <797>
- B) HIPAA
- C) CAN-SPAM
- D) ISO 9001

5. A 503A pharmacy...

- A) Doesn't need a license
- B) Prepares medication for individual prescriptions under state licensing
- C) Makes large batches as an FDA-registered outsourcing facility
- D) Only sells brand pens

6. How should public copy refer to our pharmacy partner?

- A) "The Wegovy pharmacy"
- B) By its full name in every headline
- C) "Our licensed U.S. partner pharmacy"
- D) "Our FDA-approved pharmacy"

7. What does BUD stand for?

- A) Basic unit dose
- B) Brand use document
- C) Batch under development
- D) Beyond-use date

8. Why are kits and tutorials important?

- A) Dosing errors with compounded products are a known risk
- B) They aren't important
- C) They replace the provider
- D) They're required for marketing only

9. Can brand trial results be claimed for compounded products?

- A) No
- B) Only for tirzepatide
- C) Only in emails
- D) Yes, the molecule is the same

10. In the darzi analogy, what can't the tailor say?

- A) "I made this for you"
- B) "This costs less"
- C) "This suit fits you"
- D) "This is a Raymond suit"

Sources

1. U.S. FDA, FDA's concerns with unapproved GLP-1 drugs used for weight loss (content current as of October 1, 2026)
2. U.S. FDA, Human drug compounding laws
3. U.S. Pharmacopeia, General Chapter <797> Pharmaceutical Compounding: Sterile Preparations
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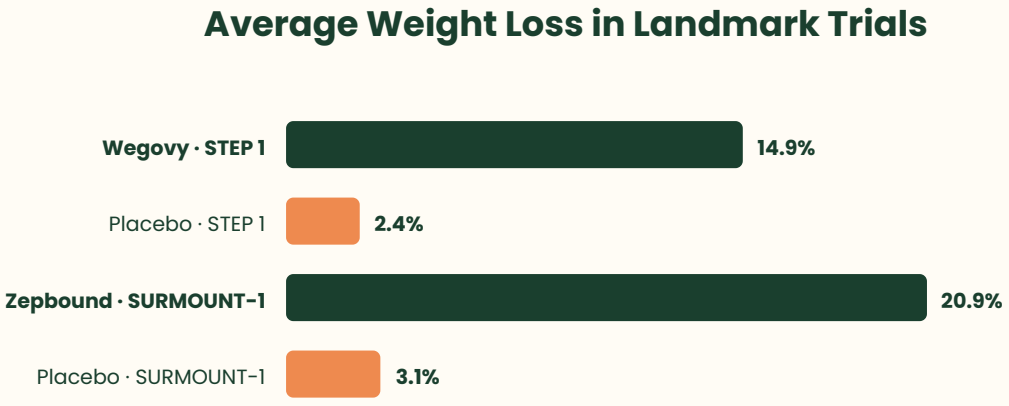
Answer key, Chapter 4: 1-D · 2-A · 3-A · 4-A · 5-B · 6-C · 7-D · 8-A · 9-A · 10-D

Chapter 5: When GLP-1 Medication Helps

We're honest about the upside. These are among the most effective weight-management medicines ever studied.

> 💡 **Trivia**
>
> In the SELECT trial, adults with heart disease and overweight or obesity who took Wegovy had about **20% fewer major heart events** (heart attack, stroke or heart-related death) than those on placebo. [3] ● Peer-reviewed
>
> GLP-1s are not "vanity drugs". For the right patient, they protect the heart.

5.1 What the major trials showed: for FDA-approved products only



Brand products only (68 and 72 weeks). Never attribute these to compounded medication.

Trial	Product	Who	Result
STEP 1	Wegovy (semaglutide 2.4 mg)	Adults with obesity or overweight, no diabetes	About **14.9%** average weight loss over 68 weeks vs. about **2.4%** on placebo, with lifestyle support [1] 🟡
SURMOUNT-1	Zepbound (tirzepatide)	Adults with obesity or overweight, no diabetes	About **20.9%** average loss on the 15 mg dose over 72 weeks vs. about **3.1%** on placebo [2] 🟡
SELECT	Wegovy	Adults with heart disease and overweight or obesity	About **20%** fewer major heart events [3] 🟡

> ⚠️ **The rule regulators are enforcing right now**
>
> Those numbers belong to **Wegovy and Zepbound**. We may NOT attribute them to compounded semaglutide or tirzepatide, call compounded products "clinically proven", or imply they match the brands. **Always name the product studied.** [4] 🟠 Industry

5.2 When it tends to help most

Situation	Why it helps
Adults with obesity, or overweight plus a weight-related condition (Chapter 20)	They match the label criteria the trials studied
People who keep hitting the same wall with diet and exercise	Often driven by hunger and "food noise", which GLP-1s reduce
People with prediabetes, type 2 diabetes, high blood pressure, high cholesterol, sleep apnea or fatty liver	Weight loss improves these conditions
People ready to pair medication with eating changes, protein, strength training, sleep and follow-up	The trials included lifestyle support
People who can keep taking it long-term	Obesity is chronic, and so is its treatment

> 🪔 **Desi analogy: cricket mein naya bowler**
>
> Team ka bowling attack kamzor hai. Naya fast bowler aata hai aur wickets girne lagte hain. Par **fielding kharab ho, toh catch chhootenge hi.**
>
> GLP-1 woh fast bowler hai. Khana, protein, exercise aur neend fielding hain. **Dono saath ho tabhi match jeetoge.**

5.3 How to write about benefits

✅ Compliant	❌ Risky
"In the STEP 1 trial of Wegovy, adults lost about 15% of body weight on average."	"Lose up to 15% with our semaglutide."
"GLP-1 medications can reduce hunger."	"Our shot kills cravings forever."
"Many patients see results; individual results vary."	"Guaranteed weight loss."

5.4 Do's and Don'ts

✅ Do	❌ Don't
Name the trial and the brand product	Write "studies show our medication..."
Pair benefits with lifestyle support	Promise results without effort
Mention the heart-risk finding for Wegovy only	Extend SELECT to compounded products
Say "individual results vary"	Use "guaranteed" or "up to" claims for our products

5.5 Pros and cons of using trial data in our content

✅ Pros	❌ Cons
Strong, credible evidence	Easily misattributed to compounded products

✅ Pros	❌ Cons
Answers "does it work?" honestly	FDA warning letters target exactly this misuse
Builds E-E-A-T	Needs precise wording and review

5.6 Where freshers go wrong in Chapter 5 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Up to 20% weight loss" on our tirzepatide page	Copying competitors	Naming Zepbound and SURMOUNT-1	Removing the claim; compliance review
Saying "clinically proven" for compounded	Marketing habit	The brand rule	Rewriting with "not FDA-approved"
Skipping "results vary"	Space limits	Keeping a standard disclaimer	Adding the disclaimer
Benefits with no limits nearby	Sales mindset	Pairing with Chapter 6	Adding a matching limit

5.7 Thumb rules

👍 Name the product studied, every single time.

👍 Benefit on the left, limit on the right.

👍 Medicine is the bowler; lifestyle is the fielding.

5.8 Notes to remember

📝 STEP 1: about 15% (Wegovy). SURMOUNT-1: about 21% on the top dose (Zepbound). SELECT: about 20% fewer heart events (Wegovy).

📝 None of these numbers can be claimed for compounded products.

Chapter 5 Glossary

Term	Simple meaning
STEP 1	Wegovy's main weight-loss trial (68 weeks)
SURMOUNT-1	Zepbound's main weight-loss trial (72 weeks)
SELECT	Wegovy's heart-outcomes trial
Placebo	An inactive treatment used for comparison
Major adverse cardiovascular events (MACE)	Heart attack, stroke or heart-related death
"Clinically proven"	A claim we never make for compounded products

Chapter 5 Quiz

1. About how much weight did adults lose on average in STEP 1 (Wegovy)?

- A) About 2%
- B) About 40%
- C) About 5%
- D) About 15%

2. Which trial studied Zepbound for weight loss?

- A) SUSTAIN
- B) SELECT
- C) SURMOUNT-1
- D) STEP 1

3. What did SELECT show?

- A) GLP-1s cure diabetes
- B) About 20% fewer major heart events with Wegovy
- C) No effect on the heart
- D) Compounded semaglutide works like Wegovy

4. Can we write "lose up to 20% with our compounded tirzepatide"?

- A) Only on social media
- B) Yes, if it's in small print
- C) Yes, the molecule is the same
- D) No; that attributes brand trial results to our product

5. Who tends to benefit most?

- A) People who won't change any habits
- B) Adults with obesity, or overweight plus a weight-related condition
- C) Anyone wanting to lose 5 pounds before a wedding
- D) Teens under 18

6. What should benefits always be paired with?

- A) Nothing
- B) A matching limit and "results vary" context
- C) A countdown timer
- D) A discount code

7. Why does long-term use matter?

- A) Because the drug stops working after a week
- B) It doesn't matter
- C) To sell more months
- D) Obesity is chronic, and so is its treatment

8. "Clinically proven" may be used for…

- A) Products shipped cold
- B) Compounded semaglutide only
- C) None of our compounded products
- D) Any product we sell

9. The trial results in this chapter come from…

- A) Ongo's patients
- B) Compounded pharmacies
- C) Social media surveys
- D) FDA-approved brand products only

10. In the cricket analogy, lifestyle changes are…

- A) The fielding that makes the bowler's work count
- B) The opposing team
- C) The umpire
- D) Not important

Sources

1. Wilding JPH et al., Once-Weekly Semaglutide in Adults with Overweight or Obesity (STEP 1), New England Journal of Medicine (2021)
2. Jastreboff AM et al., Tirzepatide Once Weekly for the Treatment of Obesity (SURMOUNT-1), New England Journal of Medicine (2022)
3. Lincoff AM et al., Semaglutide and Cardiovascular Outcomes in Obesity without Diabetes (SELECT), New England Journal of Medicine (2023)
4. Igility, What all 113 FDA telehealth warning letters actually say (analysis of FDA letters, 2025–2026); example: FDA warning letter, Remedy Meds (Sept 9, 2025)

Answer key, Chapter 5: 1-D · 2-C · 3-B · 4-D · 5-B · 6-B · 7-D · 8-C · 9-D · 10-A

PART 1 · CHAPTER 6

Chapter 6: When GLP-1 Medication Won't Help, or Isn't Safe

Patients trust a brand that warns them. Wherever we list a benefit, we list a matching limit nearby.

> 💡 **Trivia**
>
> In the STEP 1 extension, adults who **stopped** semaglutide regained about **two-thirds** of the weight they had lost within a year. [2] 🟡 Peer-reviewed
>
> GLP-1s manage a chronic disease. They don't cure it. Saying so honestly is part of our brand.

6.1 Who should not use it, or needs special care

Brand GLP-1 labels list these contraindications and warnings. Providers apply the same caution to compounded products. [1] 🟢 Official

Situation	Why it matters
Personal or family history of **medullary thyroid carcinoma (MTC)** or **MEN 2**	Boxed warning on brand labels (thyroid C-cell tumors in rodents)
Pregnancy, trying to conceive, or breastfeeding	Not recommended
History of **pancreatitis**; severe stomach or bowel disease, including gastroparesis	Higher risk of serious problems
History of an **eating disorder**	Needs careful clinical judgment
Diabetic retinopathy, gallbladder disease, kidney disease	Extra monitoring needed
People on **insulin or sulfonylureas**	Higher low-blood-sugar risk
Known **allergy** to the drug or its ingredients	Risk of severe reaction
Anyone **under 18**	Outside Ongo's scope 🔵

6.2 Side effects

How common	Examples
Very common	Nausea, vomiting, diarrhea, constipation, reflux, tiredness; worst when a dose goes up [1] 🟢
Less common but serious	Pancreatitis, gallbladder problems, kidney injury from dehydration, severe allergic reactions, low blood sugar [1] 🟢
Surgery	Patients may need to tell their anesthesiologist, because of aspiration risk [1] 🟢
Body composition	Muscle loss along with fat, especially without enough protein and strength training 🔵

6.3 When it won't do the job

Situation	Why
Cosmetic goals	"Lose 10 lbs before the wedding" with a BMI below the thresholds
Stopping early	Many people regain much of the weight within a year of stopping [2] 🟡
No lifestyle change	The medicine reduces hunger; it doesn't build muscle, fix sleep or change habits
Lower response	"Non-responders" exist in every trial
Compounding risks	Dosing mistakes (units vs. milligrams) and products from unlicensed sellers [3] 🟢

> 🪔 **Desi analogy: shaadi ka diet plan**
>
> Shaadi se do hafte pehle cousin bolti hai "mujhe bhi woh injection chahiye, bas 4 kilo kam karne hain." **Yeh medical treatment nahi, mehendi ki tension hai.**
>
> Aur jo log beech mein dawai band kar dete hain, unka haal wahi hota hai jo shaadi ke baad diet ka hota hai: sab wapas.

6.4 Why we publish the limits

- Patients trust a brand that warns them.
- Google holds health ("YMYL") content to the highest standard. [4] 🟢 Official
- Regulators expect balanced risk information.

> 🟧 **Ongo example**
>
> On every medication page, the "Is it right for me?" section must list who should **not** use it, the common side effects, and the line: *"A licensed provider decides whether treatment is appropriate for you."*

6.5 Do's and Don'ts

✅ Do	❌ Don't
List contraindications near the CTA	Hide safety info behind a link
Mention side effects are worst when doses go up	Say "no side effects"
Explain weight may return after stopping	Call GLP-1s a cure
Warn about unlicensed sellers	Mock patients who were scammed

6.6 Pros and cons of publishing limits

✅ Pros	❌ Cons
Builds trust and E-E-A-T	Some visitors leave
Meets regulator expectations	Takes more space on the page
Filters out patients who won't qualify	Needs medical review

6.7 Where freshers go wrong in Chapter 6 topics

Mistake	Why it happens	Avoid it by	Fix it by
Writing "no side effects"	Wanting to reassure	Listing common effects honestly	Medical-review rewrite
Targeting "lose weight for a wedding" keywords	High search volume	Our audience rules (Chapter 19)	Re-targeting to medical intent
Ignoring weight regain	It feels negative	Explaining long-term care	Adding a "what happens if I stop" section
Listing benefits only	Sales copy habit	The benefit/limit pairing rule	Adding limits nearby

6.8 Thumb rules

👍 Every benefit gets a matching limit.

👍 "No side effects" is never true.

👍 Stopping is part of the plan, not the end of the story.

6.9 Notes to remember

📝 MTC/MEN 2 history, pregnancy and pancreatitis history are key red flags.

📝 About two-thirds of lost weight returned within a year after stopping semaglutide (STEP 1 extension).

Chapter 6 Glossary

Term	Simple meaning
Contraindication	A reason a treatment should not be used
Boxed warning	The FDA's strongest label warning; brand GLP-1s carry one for thyroid C-cell tumors
MTC / MEN 2	Medullary thyroid carcinoma / Multiple Endocrine Neoplasia type 2
Pancreatitis	Inflammation of the pancreas
Gastroparesis	Very slow stomach emptying
Aspiration	Stomach contents entering the lungs, a risk during anesthesia
Non-responder	A person who loses little weight on a treatment
YMYL	"Your Money or Your Life": topics held to the highest standard

Chapter 6 Quiz

1. Which history is a boxed-warning red flag for GLP-1s?

- A) Medullary thyroid carcinoma (MTC) or MEN 2
- B) Wearing glasses
- C) Mild headaches
- D) Seasonal allergies

2. When are GI side effects usually worst?

- A) Never
- B) Only at night
- C) When a dose goes up
- D) After a year at the same dose

3. What happened when people stopped semaglutide in the STEP 1 extension?

- A) They kept all the weight off
- B) Nothing was measured
- C) They lost more weight

- D) They regained about two-thirds of lost weight within a year

4. Can Ongo treat someone under 18?

- A) Yes, if BMI is high
- B) Only with brand drugs
- C) Yes, with a parent's card
- D) No; it's outside our scope

5. "Lose 10 lbs before the wedding" with a BMI below thresholds is…

- A) A medical emergency
- B) Required for eligibility
- C) Our ideal patient
- D) A cosmetic goal, not an appropriate use

6. Why tell an anesthesiologist about GLP-1 use?

- A) It speeds recovery
- B) It's only paperwork
- C) Aspiration risk during surgery
- D) It prevents allergies

7. Which phrase should never appear?

- A) "A licensed provider decides"
- B) "Individual results vary"
- C) "No side effects"
- D) "Side effects can include nausea"

8. Why do we publish limits?

- A) To reduce traffic
- B) Patients trust a brand that warns them, and YMYL standards demand it
- C) It's optional decoration
- D) Because competitors do

9. What can increase muscle loss on GLP-1s?

- A) Not enough protein and strength training
- B) Sleeping well
- C) Drinking water

- D) Walking

10. In the shaadi analogy, what happens when people stop early?

- A) The wedding is cancelled
- B) They stay slim forever
- C) The weight comes back, like a post-wedding diet
- D) Nothing changes

Sources

1. Drugs.com, Wegovy prescribing information (boxed warning, contraindications, warnings and adverse reactions)
2. Wilding JPH et al., Weight regain and cardiometabolic effects after withdrawal of semaglutide: the STEP 1 trial extension, Diabetes, Obesity and Metabolism (2022)
3. U.S. FDA, FDA's concerns with unapproved GLP-1 drugs used for weight loss (content current as of October 1, 2026)
4. Google Search Central, Creating helpful, reliable, people-first content (last updated October 5, 2026)

Answer key, Chapter 6: 1-A · 2-C · 3-D · 4-D · 5-D · 6-C · 7-C · 8-B · 9-A · 10-C

02

PART 2

Understanding the Law

The FDA, compounding, the states, and why we offer compounded medication today.

7 · Federal Law: The FDA and Compounding (Status as of October 2026)

8 · State Laws: The Three Categories of States

9 · Why We Don't Offer FDA-Approved Brands Right Now

Chapter 7: Federal Law: The FDA and Compounding (Status as of October 2026)

The split that matters most in our world is FDA-approved drugs vs. compounded drugs, and the federal rules on both.

> 💡 **Trivia**
>
> On **June 16, 2026**, the FDA sent **25 warning letters** to telehealth companies in one day over compounded GLP-1 claims, after more than 40 letters in a late-2025 wave. [5] 🟠 Industry
>
> Our industry isn't lightly watched. It's one of the FDA's top enforcement targets.

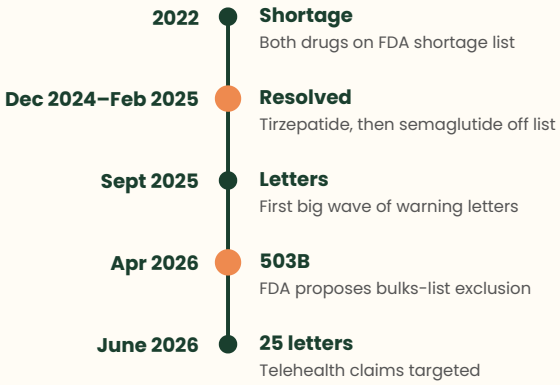
> ⚠️ **This chapter is time-sensitive.** Compounded GLP-1 rules and FDA enforcement changed repeatedly through 2025 and 2026. Before anyone publishes a regulatory claim, confirm the current position with compliance.

7.1 The basics

Rule	What it means
Prescription-only	Semaglutide and tirzepatide need a licensed prescriber's authorization, after an evaluation
The FDA approves drugs, not pharmacies	No pharmacy is "FDA-approved" [1] 🟢
Compounding is legal within limits	503A and 503B (Chapter 4). Neither may routinely make "essentially copies" of an approved drug, except while it's on the FDA shortage list [1] 🟢

7.2 How we got here

How Compounded GLP-1 Rules Changed



When	What happened
2022	Semaglutide and tirzepatide go on the FDA shortage list; compounded versions spread through telehealth
Dec 2024 – Feb 2025	FDA declares both shortages resolved (tirzepatide first, then semaglutide on Feb 21, 2025) [2]
Feb – May 2025	Grace periods for shortage-era copies end [2]
Sept 2025	First big wave of FDA warning letters to telehealth sellers [3]
Feb – Mar 2026	FDA statement on compounded GLP-1s; more warning letters (example: SkinnyRx, Feb 20, 2026) [6]
Apr 30, 2026	FDA proposes leaving semaglutide, tirzepatide and liraglutide off the 503B bulks list [4]
June 16, 2026	25 warning letters to telehealth companies [5]
Sept 18, 2026	Warning letter to a large compounding pharmacy calling its GLP-1 combinations "essentially copies" [7]

7.3 What it means today

Compounded semaglutide and tirzepatide are **still legal** when a licensed provider documents a **patient-specific clinical reason** and a properly licensed pharmacy fills it. The space is under heavy and growing enforcement. 🔵 Expert

> 🍛 **Desi analogy: restaurant aur food inspector**
>
> Restaurant chalana legal hai. Par jab food inspector baar baar aa raha ho, toh menu pe "100% desi ghee" tabhi likho jab sach mein ho. **Ek galat line, aur notice seedha aata hai.**
>
> FDA hamara food inspector hai, aur aajkal har hafte round pe hai.

7.4 What the warning letters flag: memorize this list

Flagged claim or practice	Example
Sameness	"Generic", "the same as" or "equivalent to" Ozempic, Wegovy, Mounjaro or Zepbound
Ingredient sameness	"The same active ingredient as the brands"
Borrowed evidence	Brand trial results, or "clinically proven"
Approval claims	Compounded drugs "FDA-approved" or "FDA-evaluated"
Facility claims	An "FDA-approved" or "FDA-licensed" pharmacy
Label claims	The telehealth brand's name on the label without naming the compounding pharmacy
Salt forms	e.g. semaglutide acetate

Sources: [3][5][8] 🟠🟢

Advertising is also policed by the **FTC** and by ad platforms. Google Ads allows telemedicine advertisers only if they're accredited by **LegitScript**. [9] 🟢 Ongo is LegitScript-certified; that depends on our claims staying compliant. 🔵

> 📝 **How we frame it on the site:** "Compounded medications are not FDA-approved and have not been evaluated by the FDA for safety, effectiveness or quality. Your provider decides whether a compounded medication is appropriate for you."

7.5 Do's and Don'ts

✅ Do	❌ Don't
Use the standard compounded disclaimer	Write "FDA-approved pharmacy"
Check claims against the warning-letter list	Say "same active ingredient as Ozempic"
Ask compliance before any regulatory statement	Quote rules from a competitor's site
Keep the label disclosure accurate	Put "Ongo" on a vial image without the compounder

7.6 Pros and cons of a strict compliance posture

✅ Pros	❌ Cons
Protects LegitScript and ad accounts	Slower copy approvals
Avoids warning letters	Can feel less "salesy" than competitors
Builds long-term patient trust	Needs constant updates as rules change

7.7 Where freshers go wrong in Chapter 7 topics

Mistake	Why it happens	Avoid it by	Fix it by
"FDA-approved pharmacy"	Sounds trustworthy	Remembering the FDA approves drugs only	Replacing with "licensed U.S. partner pharmacy"
Using old shortage-era copy	Old drafts reused	Dating every regulatory statement	Rewriting with current status

Mistake	Why it happens	Avoid it by	Fix it by
Copying competitor claims	"Everyone says it"	The warning-letter list	Removing the claim; telling your lead
Stating rules as permanent	Not knowing the timeline	"As of [date]" wording	Adding dates and compliance review

7.8 Thumb rules

👍 The FDA approves drugs, never pharmacies.

👍 Any sameness claim is a warning-letter claim.

👍 Every regulatory sentence gets a date and a compliance check.

7.9 Notes to remember

📝 Shortages resolved: tirzepatide (Dec 2024), semaglutide (Feb 21, 2025).

📝 Compounded GLP-1s remain legal with a documented patient-specific clinical reason.

Chapter 7 Glossary

Term	Simple meaning
FDA-approved	A specific drug the FDA reviewed for safety, effectiveness and quality. Pharmacies are never "FDA-approved"
Drug shortage list	The FDA list that temporarily allowed wide compounding until 2024–2025
"Essentially a copy"	The FDA's term for a compounded drug too similar to an approved one
Warning letter	A formal FDA notice of violations
503B bulks list	The list of bulk ingredients outsourcing facilities may use

Term	Simple meaning
LegitScript	Certification Google requires for telemedicine advertisers; Ongo is certified
FTC	Federal Trade Commission, which polices advertising claims

Chapter 7 Quiz

1. What does the FDA approve?

- A) Telehealth websites
- B) Drugs, not pharmacies
- C) Ad campaigns
- D) Pharmacies, not drugs

2. When did the FDA declare the semaglutide shortage resolved?

- A) February 21, 2025
- B) It hasn't
- C) October 2026
- D) June 2021

3. How many warning letters went to telehealth companies on June 16, 2026?

- A) 25
- B) 2
- C) 500
- D) None

4. Are compounded semaglutide and tirzepatide still legal to prescribe?

- A) Only in one state
- B) Yes, with no conditions
- C) Yes, with a documented patient-specific clinical reason and a licensed pharmacy
- D) No, they're banned

5. Which claim appears in FDA warning letters?

- A) "Same as Ozempic"
- B) "Individual results vary"

- C) "Not FDA-approved"
- D) "Your provider decides"

6. What did the FDA propose on April 30, 2026?

- A) Leaving semaglutide, tirzepatide and liraglutide off the 503B bulks list
- B) Approving compounded semaglutide
- C) Ending all warning letters
- D) Banning all telehealth

7. Which certification does Google Ads require for telemedicine advertisers?

- A) A Google partner badge
- B) None
- C) LegitScript
- D) ISO 9001

8. Who else polices advertising claims besides the FDA?

- A) The DMV
- B) Nobody
- C) The IRS
- D) The FTC

9. Before publishing any regulatory claim, you should…

- A) Confirm the current position with compliance
- B) Copy a competitor
- C) Ask social media
- D) Use last year's draft

10. In the restaurant analogy, the FDA is…

- A) The chef
- B) The delivery driver
- C) The food inspector who's on rounds every week
- D) A regular customer

Sources

1. U.S. FDA, Human drug compounding laws
2. U.S. FDA, FDA clarifies policies for compounders as national GLP-1 supply begins to stabilize
3. Igility, What all 113 FDA telehealth warning letters actually say (analysis of FDA letters, 2025–2026)
4. Orrick, FDA moves to shut the door on large-scale compounding of GLP-1 drugs (May 2026)
5. McDermott, FDA takes aim at misleading online claims for compounded GLP-1 drugs (June 2026)
6. U.S. FDA, Warning letter: Lean Rx Inc. dba SkinnyRx (February 20, 2026)
7. Frier Levitt, FDA warning letter on compounded GLP-1 products (September 2026)
8. Health Law Alliance, Compounded GLP-1 marketing after the shortage ended
9. Google Ads Help, Healthcare and medicines policy

Answer key, Chapter 7: 1-B · 2-A · 3-A · 4-C · 5-A · 6-A · 7-C · 8-D · 9-A · 10-C

Chapter 8: State Laws: The Three Categories of States

Every state adds its own rules on telehealth prescribing, which pharmacies can ship in, and how health data is protected.

> 💡 **Trivia**
>
> In telehealth, what usually matters is where the **patient** is at the time of the visit, not where the provider sits. Some states require a valid license **in the state where the patient is located**. [1] ● Official
>
> A provider in Texas seeing a patient who is travelling in Ohio may need an Ohio license for that visit.

8.1 The three categories



Category	What it means	Patient access
1. Not served	No licensed provider, pharmacy can't ship there, or a state rule blocks the model	No evaluations or shipping; no page should promise service

Category	What it means	Patient access
2. Served with extra rules	Special consent, labs, follow-up or pharmacy rules, or strict health-data laws (Washington, Nevada, Connecticut)	Served; pages and flows must reflect the extra rules
3. Standard	Standard telehealth prescribing and pharmacy shipping	Served under our default flow

- Ongo internal model: confirm each state's category with operations.

8.2 Three things every state controls

Control	Plain meaning
Provider licensure	The prescriber must be licensed where the patient is at the time of the visit [1]
Pharmacy licensure	The pharmacy must be licensed to ship into that state
Privacy	Some states regulate "consumer health data" beyond HIPAA, such as Washington's My Health My Data Act [2]

Ongo publishes a **Consumer Health Privacy Notice** and a **California Privacy Notice** for this reason.

> 🪔 **Desi analogy: shaadi ka venue permission**
>
> Shaadi Delhi mein ho ya Jaipur mein, har shehar ki police permission alag hoti hai: kahin DJ 10 baje tak, kahin 11 baje tak. **Ek hi plan sab jagah copy-paste nahi hota.**
>
> Har state ka telehealth rule bhi alag hai. Isliye har state page operations se confirm karke hi banta hai.

8.3 Our coverage claim

The site says **all 50 states**. Before writing a state or city page, confirm with operations that the state is actively served.

A "Texas" page can't be an "Arizona" page with the names swapped. That's a **doorway page**, and Google's spam policies forbid it. [3] ● Official

8.4 Do's and Don'ts

✅ Do	❌ Don't
Confirm the state's category before writing	Promise service in a state we don't serve
Mention extra consent or lab steps where they apply	Use one template for every state
Link the privacy notices from forms	Collect health data without the right notice
Write genuinely different state content	Swap city names on identical pages

8.5 Pros and cons of state pages

✅ Pros	❌ Cons
Match local searches ("semaglutide Texas")	High doorway-page risk
Can explain real state rules	Need ongoing legal and ops checks
Help patients trust local availability	A wrong promise is a compliance problem

8.6 Where freshers go wrong in Chapter 8 topics

Mistake	Why it happens	Avoid it by	Fix it by
Building 50 identical state pages	Programmatic SEO habit	Writing unique, useful content only	Consolidating or noindexing thin pages
Promising service in a blocked state	Assuming "all 50" is permanent	Checking with operations	Removing the promise immediately
Sending quiz answers to ad pixels	Not knowing state privacy laws	Chapter 12 rules	Removing the tag; telling your lead

Mistake	Why it happens	Avoid it by	Fix it by
Ignoring extra-rule states	Not reading the category table	Using the three categories	Updating the page and flow

8.7 Thumb rules

> 👍 The patient's location decides the rules.
>
> 👍 No ops confirmation, no state page.
>
> 👍 Same page with a new city name = doorway page.

8.8 Notes to remember

> 📝 Three categories: not served, served with extra rules, standard.
>
> 📝 Three controls: provider licensure, pharmacy licensure, privacy.

Chapter 8 Glossary

Term	Simple meaning
Licensure	Legal permission to practice or operate in a state
Consumer health data	Health data covered by state laws like Washington's My Health My Data Act
Doorway page	A thin page built only to rank (e.g. near-identical city pages). Forbidden
Served state	A state where Ongo actively evaluates and ships
Privacy notice	A published explanation of how we collect and use data

Chapter 8 Quiz

1. In telehealth, licensing usually depends on...

- A) Where the provider lives
- B) Where the patient is located at the time of the visit
- C) Where the website is hosted
- D) Where Ongo's office is

2. A "Category 1" state means...

- A) Standard service
- B) Served with extra rules
- C) Not served: no evaluations or shipping
- D) The busiest state

3. Which state law is an example of consumer health data rules beyond HIPAA?

- A) CAN-SPAM
- B) The Clean Air Act
- C) The Highway Act
- D) Washington's My Health My Data Act

4. What must happen before writing a state page?

- A) The page gets 1,000 visits
- B) Operations confirms the state is actively served
- C) Nothing; the site says 50 states
- D) A competitor publishes one first

5. A "Texas" page copied as an "Arizona" page with names swapped is...

- A) Good local SEO
- B) A doorway page
- C) Required by law
- D) A privacy notice

6. Which is one of the three things every state controls?

- A) Logo colors
- B) Font choice
- C) Ad budgets

- D) Pharmacy licensure

7. Why does Ongo publish a Consumer Health Privacy Notice?

- A) To sell data
- B) Some states regulate consumer health data beyond HIPAA
- C) For decoration
- D) Google requires it for ranking

8. Category 2 states need...

- A) No changes
- B) Brand drugs only
- C) Pages and flows that reflect the extra rules
- D) Fewer disclosures

9. Is "all 50 states" safe to repeat on every page without checking?

- A) Only in ads
- B) Only on Fridays
- C) No; confirm with operations first
- D) Yes, always

10. In the shaadi venue analogy, what's the lesson?

- A) Weddings need DJs
- B) Permissions don't matter
- C) Each state has different rules, so one plan can't be copy-pasted
- D) All cities are the same

Sources

1. HealthIT.gov, Are there state licensing issues related to telehealth?
2. Future of Privacy Forum, A new paradigm for consumer health data privacy in Washington State (2023)
3. Google Search Central, Spam policies for Google web search (last updated August 28, 2026)

Answer key, Chapter 8: 1-B · 2-C · 3-D · 4-B · 5-B · 6-D · 7-B · 8-C · 9-C · 10-C

PART 2 · CHAPTER 9

Chapter 9: Why We Don't Offer FDA-Approved Brands Right Now

Patients, partners and new team members ask this constantly. Here is the internal answer: the business reasons, who controls the brand drugs, how the brand makers protect their names, and what Google's ad rules really say.

> 💡 **Trivia**
>
> By August 2025, Novo Nordisk had filed **132 complaints in federal courts across 40 U.S. states** against compounding pharmacies, telehealth companies and medical spas over copies of its semaglutide drugs. [1] ● Industry
>
> The maker of Ozempic and Wegovy isn't just competing in the market. It's in court, constantly, defending it.

> ⚠️ **Confirm before you quote.** Everything in this chapter is internal explanation. Confirm the exact wording with leadership and compliance before using any of it publicly, and never repeat the legal points below to patients, journalists or on social media.

9.1 The business reasons

Reason	Explanation
Price	Brand GLP-1s still generally cost more on cash pay than a compounded plan. In late 2025, Wegovy's self-pay price through NovoCare dropped to $349 a month and Zepbound vials through LillyDirect to $299–$449 [12] ●. Ongo's semaglutide plans run $179–$249 a month (Chapter 22). Most of our patients don't have insurance coverage for weight medication
Self-pay model	Ongo doesn't bill insurance. Brand drugs often depend on insurance, prior authorization and appeals: the exact hassle our patients come to us to avoid

Reason	Explanation
One pharmacy partner, one workflow	Our operations run through a licensed 503A partner pharmacy that prepares and ships compounded medication to the door, with our kit and tutorials
Personalized dosing	Compounding lets a provider prescribe a dose and step-up schedule for the individual, including slower, lower-dose starts for patients who struggle with side effects
Focus	Two treatments done well, before adding more
Brand control	The brand drugs are controlled by two companies that decide who sells them and on what terms (9.2–9.4)
Advertising	Our launch-phase ad strategy keeps brand names out of ads and commercial pages (9.5–9.6)

● Ongo internal

9.2 The brand market: two companies, protected by patents

A **patent** gives its owner the legal right to stop anyone else from making or selling an invention for a limited time. In plain terms, it's a **legal, time-limited monopoly** on a molecule. ● Expert

Molecule	Who owns the brands	U.S. patent protection
Semaglutide (Ozempic, Wegovy, Rybelsus)	Novo Nordisk only	Novo's own annual report: compound patent expiry in the U.S. is **2032** [2] ●
Tirzepatide (Mounjaro, Zepbound)	Eli Lilly only	Compound patent expected to run to about **2036**, with later patents beyond that [3] ●

What this means in practice:

- There is **no FDA-approved generic semaglutide or tirzepatide** on the U.S. market today. Every FDA-approved product with these molecules comes from Novo Nordisk or Eli Lilly.
- So the branded GLP-1 weight-loss market is effectively controlled by **two companies**. People in the industry often call it a duopoly.

- Until the patents expire, any other company that wants to offer the **branded** drug has to go through the maker's own channels or partnership deals (9.4).

> 🪔 **Desi analogy: mohalle ka ek hi authorised dealer**
>
> Mohalle mein Royal Enfield ki bike sirf authorised showroom se milti hai. Company decide karti hai kaun dealer banega, kis rate pe bechega. Agar koi garage wala apni banayi bike pe "Royal Enfield" ka logo laga de, **toh company ka legal notice seedha aata hai.**
>
> Wegovy aur Zepbound ke saath bhi yahi hai. Company tay karti hai kaun bechega. Aur jo unka naam apne product pe use kare, uske khilaaf case.

9.3 How the brand makers protect their names and drugs

Company	What they've done	Legal basis
Novo Nordisk	Since **2023**, lawsuits against medical spas, clinics and compounding pharmacies that advertised compounded semaglutide with references to **Ozempic or Wegovy**, plus cease-and-desist letters to stop ads mentioning Novo, Ozempic or Wegovy. Most cases settled with orders to correct advertising [4] 🟠	Lanham Act: **trademark infringement** and **false advertising**
Novo Nordisk	**132 federal complaints in 40 states** by August 2025, alleging defendants led patients to believe compounded products were FDA-approved or as safe and effective as Novo's drugs [1] 🟠	Deception; state corporate-practice-of-medicine laws
Novo Nordisk	**February 9, 2026:** sued telehealth company Hims & Hers for **patent infringement** over compounded semaglutide, asking the court to permanently stop the sales [5] 🟢 Company statement	Patent (U.S. Patent 8,129,343)
Eli Lilly	**April 2025:** sued four telehealth companies (Fella Health, Willow Health, Henry Meds and Mochi Health); earlier sued compounding pharmacies over compounded tirzepatide [6] 🟠	Deceptive and unfair trade practices, false or misleading advertising, unlicensed practice of medicine

> 🟧 **What this means for Ongo**
>
> The brand makers watch exactly how compounded sellers use **their brand names**. Using "Ozempic", "Wegovy", "Zepbound" or "Mounjaro" to sell or advertise compounded medication is the fastest way to invite a cease-and-desist letter or a lawsuit, on top of the FDA problems in Chapter 7.

9.4 Correction: the makers do let others offer their drugs, on their terms

It's a common belief that Novo and Lilly "don't want anyone to offer their medicines." **That isn't accurate.** They control **who** offers them and **how**:

Company	Approved channels
Novo Nordisk	Its own self-pay pharmacy, **NovoCare**. In May 2025, Novo announced Wegovy access through telehealth partners Ro, LifeMD and Hims & Hers [7] 🟠 (the Hims partnership later ended, and Novo has since sued Hims [5])
Eli Lilly	Its own self-pay channel, **LillyDirect**. Lilly partnered with telehealth companies such as LifeMD and Teladoc Health to offer single-vial Zepbound, with the telehealth companies contracting directly with Lilly's self-pay pharmacy [8] 🟠

So the accurate internal line is:

> 📝 **The brand makers control their brand drugs through patents, trademarks, their own self-pay pharmacies and selected partners. They act aggressively against anyone who uses their names or copies their products. Ongo isn't one of their partners, and we've chosen to build around compounded medication for now.**

9.5 Google Ads: what the official rules really say

We plan to run Google Ads, so this matters. Here's what Google's **own policy pages** say (checked October 2026):

Rule	What Google says	Source
Who may advertise telemedicine	In the U.S., telemedicine providers must be accredited by **LegitScript's Healthcare Merchant Certification Program**, and advertisers must also be **certified with Google**	[9] 🟢
Prescription drug terms	Google restricts prescription drug terms in ads, landing pages and keywords. For U.S. campaigns, advertisers **don't need certification to use these terms in ads and landing pages**, but **certification is required to keyword-target them**	[10] 🟢
Trademarks	Google restricts trademarks used **in ad text**, after a **complaint from the trademark owner**. A trademark that appears **only on the landing page** doesn't trigger the restriction. Informational pages have an exception	[11] 🟢

> ⚠️ **Important correction.** Google's official policies do **not** say a website can't mention brand names like Wegovy or Ozempic. The rules are about **certification** (to advertise telemedicine and to target drug-name keywords) and about **trademarks in ad text** when the owner complains.

9.6 Ongo's launch-phase rule: no brand names on commercial pages or ads

So why keep brand names off our site at launch? Because it's the **safest choice** when three risks stack up, even though Google doesn't strictly require it:

Risk	Why brand names make it worse
Trademark complaints and lawsuits	Novo and Lilly actively pursue compounded sellers who use their brand names (9.3)
FDA "sameness" claims	Brand names next to compounded products drift into "same as" or "equivalent to" claims, the exact language in FDA warning letters (Chapter 7)
Ad review during certification	While LegitScript and Google certification are pending, drug-name keywords can't be targeted anyway, and brand terms on landing pages invite extra scrutiny and disapprovals 🔵

The rule, for now: 🔵 Ongo internal

Where	Brand names (Wegovy, Ozempic, Rybelsus, Zepbound, Mounjaro, Foundayo, Saxenda)
Google Ads text and keywords	❌ No
Money pages, pricing, How It Works, landing pages	❌ No
Meta titles and descriptions of commercial pages	❌ No
Future educational or comparison pages (Chapter 18)	✅ Only after the launch phase, after medical and compliance review, and always with "Ongo does not offer this medication"

9.7 The compliance caveat: read this twice

Business reasons are **not** a legal justification for compounding. Every compounded prescription needs a **patient-specific clinical reason** documented by the provider. [13] 🟢 Official

- We never say "compounded because it's cheaper" in clinical copy.
- We never imply brand drugs are worse.
- We never use the words "monopoly" or "duopoly", or comment on Novo or Lilly, in public copy. That stays inside this book.

9.8 Things to fix

Issue	What to do
The site footer says "FDA-approved medications are prescribed when clinically appropriate"	That contradicts what we offer. Raise it with your lead before publishing anything that repeats it 🔵
Any brand name on a commercial page or ad	Remove it under the launch-phase rule (9.6)
A patient needs or wants a brand-name drug	The honest answer: Ongo doesn't offer it today. They can ask their own doctor or use the manufacturer's program

9.9 How to answer the question, by audience

Who asks	What to say
Patient	"Ongo currently offers compounded semaglutide and compounded tirzepatide, prescribed when a licensed provider decides they're appropriate. We don't offer brand-name medications."
Journalist / partner	Route to leadership; don't improvise, and never discuss lawsuits or competitors
New teammate	Point them to this chapter

9.10 Do's and Don'ts

✅ Do	❌ Don't
Say plainly that we don't offer brands	Hint that we can "get" Wegovy
Refer brand seekers to their doctor or the maker's program	Badmouth brand drugs or their makers
Keep brand names out of ads and commercial pages	Bid on "Ozempic" or "Wegovy" keywords
Get certification before targeting drug-name keywords	Assume uncertified drug keywords will run
Keep business and legal reasons internal	Say "Novo has a monopoly" in public copy

9.11 Pros and cons of offering compounded only

✅ Pros	❌ Cons
Simple operations and pricing	Heavier regulatory scrutiny
Personalized dosing	Can't serve patients who want brands
Lower cash price for most patients	Brand self-pay prices are closing the gap
No dependence on a brand maker's partnership terms	Brand makers actively sue compounded sellers

9.12 Where freshers go wrong in Chapter 9 topics

Mistake	Why it happens	Avoid it by	Fix it by
"We use compounded because it's cheaper" in clinical copy	Honest but non-compliant	The compliance caveat	Rewriting around provider judgment
Adding "Ozempic alternative" to an ad or landing page	High search volume	The launch-phase rule	Removing it; telling your lead
Saying "Google bans brand names on websites"	Mixing up the policies	Reading 9.5	Correcting internal docs
Writing "Big Pharma monopoly" in a blog	Strong opinion	Keeping legal commentary internal	Removing it; compliance review
Answering a journalist directly	Wanting to help	Routing to leadership	Telling your lead immediately

9.13 Thumb rules

👍 Business reasons stay inside; clinical reasons go on the record.

👍 Their names, their rules: no brand names in our ads or commercial pages.

👍 Google requires certification, not silence. The silence is our own safety choice.

9.14 Notes to remember

📝 Seven reasons: price, self-pay, one pharmacy workflow, personalized dosing, focus, brand control, advertising.

📝 Semaglutide's U.S. compound patent runs to 2032 (Novo); tirzepatide's to about 2036 (Lilly).

📝 Novo and Lilly sell through their own pharmacies and chosen telehealth partners, and sue those who misuse their names.

Chapter 9 Glossary

Term	Simple meaning
Self-pay	Patient pays directly; no insurance billing
Prior authorization	Insurer approval required before covering a drug
Manufacturer program	A brand maker's direct cash-pay pharmacy (e.g. NovoCare, LillyDirect)
Clinical reason	A documented medical justification for a specific patient
Partner pharmacy	The licensed U.S. pharmacy that prepares and ships our medication
Patent	A time-limited legal right to stop others making or selling an invention
Duopoly	A market controlled by two companies
Trademark	A protected brand name or logo (e.g. "Wegovy")
Lanham Act	The U.S. federal law on trademarks and false advertising
Cease-and-desist letter	A formal demand to stop an activity before a lawsuit
Keyword targeting	Choosing search terms that trigger an ad
Google Ads certification	Google's approval to advertise restricted healthcare services

Chapter 9 Quiz

1. Does Ongo offer brand-name GLP-1s today?

- A) Only on request
- B) Only Wegovy
- C) Yes, all of them
- D) No

2. Is "it's cheaper" a legal justification for compounding?

- A) Yes
- B) Only in some states

- C) Only for tirzepatide
- D) No; each prescription needs a patient-specific clinical reason

3. When does semaglutide's U.S. compound patent expire, according to Novo's annual report?

- A) 2018
- B) 2032
- C) 2050
- D) It already has

4. Which is one of Ongo's reasons for not offering brands?

- A) Personalized dosing
- B) Brands are unsafe
- C) Avoiding all regulation
- D) Doctors prefer it

5. What's wrong with the current site footer?

- A) It says FDA-approved medications are prescribed, which contradicts our offer
- B) It's too short
- C) Nothing
- D) It's the wrong color

6. A patient wants Zepbound. What's the honest answer?

- A) "Zepbound is dangerous"
- B) "Ours is the same thing"
- C) "We can get it for you"
- D) Ongo doesn't offer it; ask your own doctor or use the maker's program

7. What legal basis did Novo use against spas and clinics that referenced Ozempic and Wegovy for compounded products?

- A) Tax law
- B) The Lanham Act: trademark infringement and false advertising
- C) HIPAA
- D) Traffic law

8. Should our copy imply brand drugs are worse?

- A) Never

- B) Only in ads
- C) Only in emails
- D) Yes, to win patients

9. According to Google's official policy, what does a U.S. advertiser need before keyword-targeting prescription drug names?

- A) Nothing
- B) A bigger budget
- C) A trademark of its own
- D) Certification (LegitScript for telemedicine, plus Google certification)

10. In the authorised-dealer analogy, what happens if a garage puts the company's logo on its own bike?

- A) The company sends a reward
- B) Nothing
- C) A legal notice arrives
- D) The bike becomes official

Sources

1. Pharmaceutical Technology, Novo Nordisk files 14 new lawsuits against unapproved drugs (August 6, 2025)
2. Novo Nordisk, Form 20-F, fiscal year 2024 (filed with the U.S. SEC): "Compound patent expiry in the U.S. for the semaglutide branded products... is 2032"
3. GreyB, Mounjaro patent expiration
4. IPWatchdog, A review of Novo Nordisk's Ozempic and Wegovy trademark enforcement campaign (July 23, 2024)
5. Novo Nordisk, Company statement: Novo Nordisk takes legal action against Hims & Hers (February 9, 2026)
6. Holland & Knight, Eli Lilly strikes back against pharmacy compounders and telehealth platforms (June 13, 2025)
7. HLTH, Novo Nordisk partners with telehealth firms to expand Wegovy access in the US (May 1, 2025)
8. Fierce Healthcare, Eli Lilly expands telehealth access to low-cost Zepbound with Teladoc Health, LifeMD tie-ups

9. Google Ads Help, Healthcare and medicines policy (U.S. section: online pharmacies and telemedicine)
10. Google Ads Help, Healthcare and medicines: Restricted drug terms
11. Google Ads Help, Trademarks policy
12. Fierce Pharma, Lilly joins Novo in GLP-1 self-pay price cuts, lowering costs of single-dose Zepbound vials (November 2025)
13. U.S. FDA, Human drug compounding laws

Answer key, Chapter 9: 1-D · 2-D · 3-B · 4-A · 5-A · 6-D · 7-B · 8-A · 9-D · 10-C

03

PART 3

Problem, Solution and How We Work

Why patients struggle, how Ongo answers it, and the steps from booking to delivery.

10 · The Problem We Have

11 · How We Solve It

12 · HIPAA and Health Privacy: The Backbone

13 · How the Platform Works

14 · How We Work: From Booking to Your Door

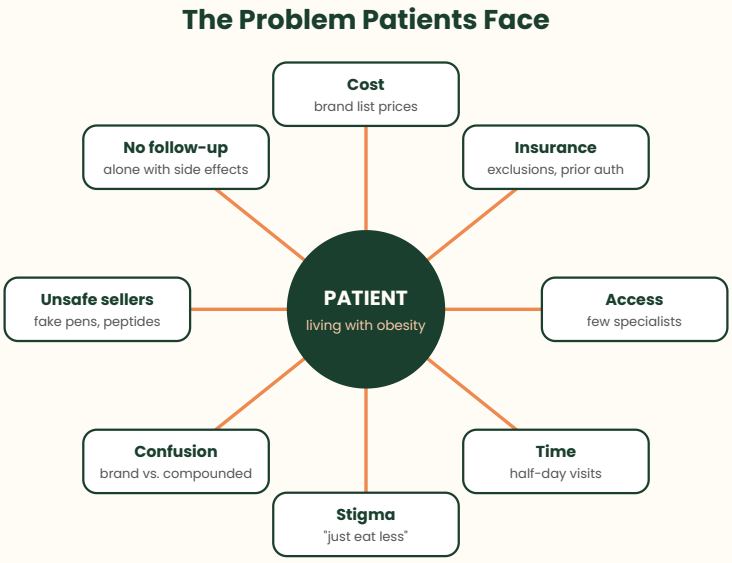
15 · Why Online Is Better Than Offline (For Most of Our Patients)

Chapter 10: The Problem We Have

Before describing what Ongo does, understand the problem the brand was built to fix.

> 💡 **Trivia**
>
> In a November 2025 KFF poll, **56% of GLP-1 users** said the drugs were difficult to afford, and about **27% of insured users** said they paid the full cost themselves. [1] ● Industry
>
> Even people with insurance struggle. Cost isn't a side issue; it's the first wall most patients hit.

10.1 Eight walls patients hit



The problem is access and continuity, not willpower.

Wall	What it looks like
Cost	Brand GLP-1s historically cost over $1,000 a month at list price. Cash programs have lowered that, but it's still out of reach for many [1][2] ●

Wall	What it looks like
Insurance	Many plans exclude weight medication, demand prior authorization, or cover it only for diabetes
Access	Obesity-medicine specialists are few; waits are long; rural patients may have none nearby
Time	Primary-care visits are short and weight often isn't raised; a clinic visit costs half a day off work
Stigma	Many patients have been told to "just eat less". They avoid asking for help
Confusion	Wegovy vs. Ozempic vs. compounded vs. pills vs. "research peptides": patients can't tell what's legitimate
Unsafe sellers	Counterfeit pens and unlicensed peptide sites fill the gap, with real harm [3] 🟢
No follow-up	Even patients who get a prescription often get no help with side effects or dose increases

📝 **The result.** Millions of people who could benefit from supervised treatment go without, or turn to unsafe sources. **The problem is access and continuity, not willpower.**

🪔 **Desi analogy: mohalla ka pani ka tanker**

Mohalle mein pani ki kami hai. Log bhookhe nahi, pyaase hain. Agar sarkari tanker time pe na aaye, toh log nakli bottle wale se pani kharidte hain, aur beemar padte hain.

Patients ke saath bhi yahi hota hai. Sahi ilaaj tak pahunch na mile toh woh nakli "peptide" bechne walon ke paas chale jaate hain. Ongo woh tanker hai jo time pe, sahi pani le kar aata hai.

10.2 How each wall shows up in search

Wall	Example searches	Content we can create
Cost	"semaglutide cost without insurance"	Honest pricing explainer (Chapter 22)
Insurance	"does insurance cover Wegovy"	Coverage explainer that says Ongo is self-pay

Wall	Example searches	Content we can create
Access	"weight loss doctor near me"	How telehealth works (Chapter 14)
Confusion	"compounded vs. Wegovy"	Brand vs. compounded guide (Chapter 4)
Unsafe sellers	"are GLP-1 drops real"	Safety guide warning against unlicensed sellers

- Expert judgment

10.3 Do's and Don'ts

✅ Do	❌ Don't
Describe the problem with empathy	Blame patients for the problem
Use real, sourced cost data	Exaggerate prices to scare people
Warn about unsafe sellers	Name and shame individual patients
Connect each problem to an honest solution	Overpromise that we fix everything

10.4 Pros and cons of problem-first content

✅ Pros	❌ Cons
Patients feel understood	Can slide into fear marketing
Matches real search intent	Needs accurate, dated numbers
Natural path to our solution	Must avoid attacking competitors

10.5 Where freshers go wrong in Chapter 10 topics

Mistake	Why it happens	Avoid it by	Fix it by
"You failed every diet because..."	Trying to relate	Blaming the system, not the person	Rewriting with empathy
Quoting old list prices as current	Copying old articles	Dating every price	Updating with a sourced figure
Fear-based headlines about counterfeits	Clickbait habit	Calm, factual warnings	Rewriting the headline
Calling competitors "scams"	Frustration	Talking about unlicensed sellers in general	Removing names; compliance check

10.6 Thumb rules

👍 Blame the barriers, never the patient.

👍 Every problem gets a calm, honest answer.

👍 Date every price.

10.7 Notes to remember

📝 Eight walls: cost, insurance, access, time, stigma, confusion, unsafe sellers, no follow-up.

📝 The problem is access and continuity, not willpower.

Chapter 10 Glossary

Term	Simple meaning
List price	The manufacturer's official price before discounts or insurance

Term	Simple meaning
Prior authorization	Insurer approval required before covering a drug
Counterfeit	A fake product made to look like the real brand
Continuity of care	Ongoing follow-up after the first prescription
Search intent	What a person actually wants when they type a search

Chapter 10 Quiz

1. What share of GLP-1 users said the drugs were difficult to afford (KFF, Nov 2025)?

- A) 56%
- B) 0%
- C) 5%
- D) 95%

2. What is the core problem, according to this chapter?

- A) Access and continuity, not willpower
- B) The drugs don't work
- C) Doctors don't care
- D) Patients are lazy

3. Which is one of the eight walls?

- A) Too many specialists
- B) Free medication
- C) Stigma
- D) Short waits

4. Why are unsafe sellers a problem?

- A) They aren't a problem
- B) They're FDA-approved
- C) Counterfeit pens and unlicensed peptides cause real harm
- D) They're too expensive

5. "No follow-up" means...

- A) Too many check-ins
- B) Patients get no help with side effects or dose increases after the prescription
- C) Doctors call daily
- D) Patients never get a prescription

6. How should we write about the problem?

- A) With fear headlines
- B) By shaming patients
- C) By naming competitors as scams
- D) With empathy, blaming the barriers, not the patient

7. Even insured patients sometimes…

- A) Never need prior authorization
- B) Pay the full cost themselves
- C) Get the drugs free forever
- D) Can't use telehealth

8. What content answers the "confusion" wall?

- A) A discount banner
- B) A countdown timer
- C) Nothing
- D) A brand vs. compounded guide

9. Brand list prices historically were…

- A) Always free with insurance
- B) Exactly $179
- C) Under $50 a month
- D) Over $1,000 a month

10. In the water tanker analogy, Ongo is…

- A) The fake bottle seller
- B) The tanker that brings the right water on time
- C) The broken pipe
- D) The thirsty resident

Sources

1. KFF, Poll: 1 in 8 adults say they are currently taking a GLP-1 drug… even as half say the drugs are difficult to afford (November 14, 2025)
2. Fierce Pharma, Lilly joins Novo in GLP-1 self-pay price cuts (November 2025)
3. U.S. FDA, FDA's concerns with unapproved GLP-1 drugs used for weight loss (content current as of October 1, 2026)

Answer key, Chapter 10: 1-A · 2-A · 3-C · 4-C · 5-B · 6-D · 7-B · 8-D · 9-D · 10-B

Chapter 11: How We Solve It

Every problem in Chapter 10 gets a specific, honest answer.

> 💡 **Trivia**
>
> HHS lists faster appointments, less travel and less time off work among the main benefits of telehealth, and also says it may not suit every person or condition. [1] ● Official
>
> Even the government's own telehealth page names the limits. We should too.

11.1 Problem → answer

Problem	How Ongo answers it
Cost	Flat monthly plans from $179 to $249; no insurance needed (Chapter 22)
Insurance hassle	Self-pay; no prior authorization or appeals
Access	100% online, licensed U.S. providers; site says all 50 states (confirm with operations)
Time	Online screening and a video visit from home; medication shipped to the door
Stigma	Private, judgment-free care; person-first language everywhere
Confusion	Plain-English education that clearly separates brand from compounded
Unsafe sellers	Real prescription, licensed U.S. partner pharmacy, quality checks, LegitScript certification
No follow-up	Weekly check-ins, dose adjustments, biweekly doctor Q&A, support until you stop safely

● Ongo internal (live site, October 2026) [2]

11.2 Telehealth is the delivery model

Telehealth **removes** geography, waiting rooms and lost work time. [1] 🟢

It **keeps** what matters:

- a licensed provider who can say no,
- a real health review,
- documentation,
- follow-up,
- HIPAA-protected privacy.

> ⚠️ **Telehealth is not a shortcut.** It's more accessible, not less rigorous. "No questions asked" prescribing is exactly what regulators target.
>
> 🍛 **Desi analogy: online food order vs. kitchen ki safai**
>
> Zomato pe order karna aasaan hai, par kitchen ki safai, FSSAI license aur fresh khana: yeh sab waisa hi hona chahiye jaise restaurant mein baith ke khaate.
>
> **Telehealth order karne ka tareeka badalta hai, ilaaj ka standard nahi.**

11.3 How to talk about the solution

✅ Compliant	❌ Risky
"See a licensed provider from home."	"Get your prescription in minutes, no questions asked."
"No insurance needed."	"Cheaper than any pharmacy."
"Weekly check-ins with our care team."	"24/7 doctor on call" (unless true and confirmed)

11.4 Do's and Don'ts

✅ Do	❌ Don't
Match every claim to a real service	Invent features to beat competitors

✅ Do	❌ Don't
Say the provider can decline	Imply approval is automatic
Confirm check-in frequency before quoting it	Mix "weekly" and "biweekly" carelessly
Mention LegitScript where relevant	Use fake trust badges

11.5 Pros and cons of the telehealth model

✅ Pros	❌ Cons
Fast, private, convenient	No physical exam
Reaches rural patients	Depends on internet and devices
Easy follow-up	Heavy regulatory scrutiny

11.6 Where freshers go wrong in Chapter 11 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Instant prescription"	Speed sells	"A provider decides" line	Rewriting the CTA
Quoting all 50 states without checking	Repeating the site	Ops confirmation	Adding a state-availability note
Inventing features	Competitive pressure	Checking with the product team	Removing the claim
Mixing check-in frequencies	Two pages say different things	Chapter 25 fix list	Using the confirmed wording

11.7 Thumb rules

👍 Accessible, never automatic.

👍 Every promise must exist in the product.

👍 Telehealth changes the route, not the standard.

11.8 Notes to remember

📝 Eight problems, eight answers: price, self-pay, online access, time, privacy, education, safety, follow-up.

📝 A provider who can say no is a feature, not a bug.

Chapter 11 Glossary

Term	Simple meaning
Telehealth	Healthcare delivered remotely by video, phone or secure messaging
Provider	A licensed prescriber (MD, DO, NP or PA, depending on state)
Check-in	A scheduled follow-up on progress and side effects
Dose adjustment	A provider changing the dose based on response
Trust signal	Evidence of legitimacy, such as LegitScript certification or a named reviewer

Chapter 11 Quiz

1. How does Ongo answer the cost problem?

- A) Brand drugs at a discount
- B) Flat monthly plans from $179 to $249, no insurance needed
- C) Insurance billing

- D) Free medication for everyone

2. Telehealth removes...

- A) Geography, waiting rooms and lost work time
- B) The provider's judgment
- C) Privacy
- D) Documentation

3. Telehealth keeps...

- A) Paper forms by mail
- B) Guaranteed prescriptions
- C) Long waiting rooms
- D) A licensed provider who can say no

4. "No questions asked" prescribing is...

- A) Our main selling point
- B) Exactly what regulators target
- C) A trust signal
- D) Required by law

5. Which line is compliant?

- A) "Cheaper than any pharmacy."
- B) "Get your prescription in minutes, no questions asked."
- C) "See a licensed provider from home."
- D) "Guaranteed approval."

6. How does Ongo answer the unsafe-sellers problem?

- A) Real prescription, licensed partner pharmacy, quality checks, LegitScript
- B) Ignoring it
- C) Selling peptides
- D) Shipping pens with no prescription

7. What does HHS say about telehealth's limits?

- A) It replaces all doctors
- B) It has no limits
- C) It may not suit every person or condition

- D) It's illegal

8. Before quoting check-in frequency, you should...

- A) Copy a competitor
- B) Leave it out forever
- C) Pick the higher number
- D) Confirm the wording, because pages differ

9. Is a provider declining a patient a failure?

- A) Only on weekends
- B) No; it's a sign of real medical care
- C) Yes, always
- D) Only for tirzepatide

10. In the online food order analogy, what doesn't change?

- A) The standard of the kitchen (the standard of care)
- B) The app you order from
- C) The payment method
- D) The delivery route

Sources

1. Telehealth.HHS.gov, Why use telehealth?
2. Ongo Weight Loss, Homepage, How It Works and Pricing (checked October 2026)

Answer key, Chapter 11: 1-B · 2-A · 3-D · 4-B · 5-C · 6-A · 7-C · 8-D · 9-B · 10-A

PART 3 · CHAPTER 12

Chapter 12: HIPAA and Health Privacy: The Backbone

HIPAA (1996) sets the legal standard for protected health information. For a health brand, privacy isn't a feature; it's the foundation.

> 💡 **Trivia**
>
> HHS has warned that tracking technologies (like ad pixels) on health websites can send protected health information to vendors, which can break HIPAA if there's no proper agreement or patient authorization. [2] ● Official
>
> A single tag in Google Tag Manager can become a privacy incident. Marketing and privacy are the same job here.

12.1 What HIPAA requires, in plain language

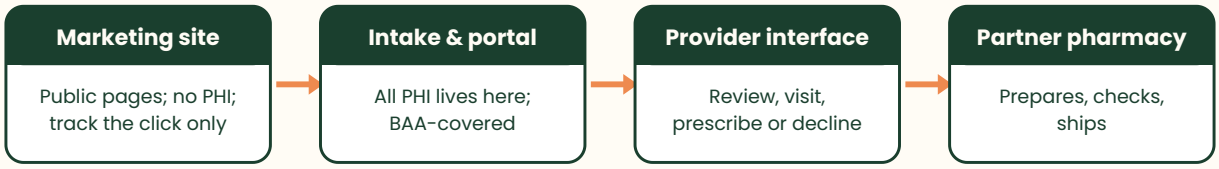
Requirement	Plain meaning
Safeguards	Administrative, physical and technical safeguards protect electronic PHI; encryption in storage and transit is the industry standard [1] ●
Access control	Only authorized people can access PHI, and access is logged [1] ●
Business Associate Agreements	Every vendor touching PHI signs a BAA [3] ●
Breach notification	Breaches are reported within strict deadlines [4] ●

12.2 Beyond HIPAA

Rule	What it covers
State consumer-health-data laws	e.g. Washington's My Health My Data Act can cover quiz answers, page views and pixel data [5] 🟠
FTC Health Breach Notification Rule	Health apps and similar services not covered by HIPAA must report breaches; updated in 2024 [6] 🟢
Tracking-technology guidance	Sending health data to ad platforms is an active enforcement area [2] 🟢

12.3 What this means for our team

The PHI Boundary



PHI never crosses back into analytics, CMS, Slack or spreadsheets.

Rule	Example
PHI lives ONLY in the patient platform	Never in the codebase, CMS, spreadsheets, email, Slack, design files or screenshots
No PHI to analytics or ads	No names, emails, phones, weights, BMIs, medications or quiz answers go to GA4, Meta, TikTok or any pixel
Consent for patient stories	Photos, before/afters and quotes need documented written consent and "results vary" context

🔵 Ongo policy

> 🪔 **Desi analogy: IT office ka server room**
>
> Office mein sab cafeteria mein ghoom sakte hain, par server room mein sirf badge wale. Agar koi client ka data pen drive mein daal ke cafeteria le aaye, **toh naukri bhi jaayegi aur client bhi.**
>
> Patient portal hamara server room hai. Marketing site cafeteria hai. Data kabhi bahar nahi aata.
>
> ⚠️ **A privacy mistake is not a small mistake.** It can mean penalties, lost ad accounts, lost LegitScript certification and lost patient trust.

12.4 Do's and Don'ts

✅ Do	❌ Don't
Track the CTA click, not the answers	Send quiz answers to GA4 or pixels
Keep PHI inside the portal	Paste patient details into Slack or email
Get written consent for stories	Use a patient photo "because they posted it"
Ask before adding any new tag	Install pixels on intake pages

12.5 Pros and cons of strict privacy rules

✅ Pros	❌ Cons
Protects patients and the business	Less detailed marketing data
Keeps ad accounts and certification safe	More approvals for tags and tools
Builds trust	Requires training for every role

12.6 Where freshers go wrong in Chapter 12 topics

Mistake	Why it happens	Avoid it by	Fix it by
Adding a Meta pixel to the intake page	Following an ad tutorial	Tag approval process	Removing it; reporting to your lead today
Screenshotting a patient record for a bug report	Speed	Using test data	Deleting the screenshot; reporting it
Sending BMI as a GA4 event parameter	Wanting better data	"Track the click only" rule	Removing the parameter; data deletion request
Using a review photo without consent	Assuming public = permission	Written consent rule	Taking it down

12.7 Thumb rules

👍 The portal is the server room. Nothing leaves.

👍 Track the click, never the answer.

👍 No consent, no story.

12.8 Notes to remember

📝 HIPAA: safeguards, access control, BAAs, breach notification.

📝 Beyond HIPAA: state health-data laws, the FTC rule, tracking-technology guidance.

Chapter 12 Glossary

Term	Simple meaning
HIPAA	The U.S. health-privacy law (1996)
PHI	Protected health information

Term	Simple meaning
BAA	Business Associate Agreement: the vendor contract for PHI
Breach	Unauthorized access to or disclosure of PHI
Tracking technology	Pixels, tags or scripts that collect user activity
Health Breach Notification Rule	An FTC rule requiring breach notices from non-HIPAA health apps

Chapter 12 Quiz

1. Where may PHI live?

- A) In design files
- B) Only in the patient platform
- C) In Slack, if the channel is private
- D) In the CMS

2. What should GA4 track on the intake flow?

- A) Patients' names
- B) Patients' medications
- C) Patients' BMI
- D) The CTA click or start, never the answers

3. What is a BAA?

- A) The contract a vendor signs before touching PHI
- B) A medication
- C) A Google ranking factor
- D) A type of ad

4. What did HHS warn about tracking technologies?

- A) They're required
- B) They're always safe
- C) They improve privacy
- D) They can send PHI to vendors and break HIPAA

5. Washington's My Health My Data Act can cover...

- A) Quiz answers, page views and pixel data
- B) Only hospitals
- C) Nothing online
- D) Only paper records

6. A patient posted a before/after on Instagram. Can we reuse it?

- A) Yes, if they tagged us
- B) Yes, if we crop it
- C) Yes, it's public
- D) Only with documented written consent and "results vary" context

7. Which consequence can follow a privacy mistake?

- A) Lost ad accounts and LegitScript certification
- B) Higher rankings
- C) Nothing
- D) Free advertising

8. Which FTC rule covers breaches by health apps outside HIPAA?

- A) The Endorsement Guides
- B) The Health Breach Notification Rule
- C) The Fake Reviews Rule
- D) CAN-SPAM

9. You accidentally screenshotted patient data. What now?

- A) Ignore it
- B) Delete it and report it to your lead today
- C) Keep it for later
- D) Post it in Slack

10. In the server room analogy, the patient portal is...

- A) The parking lot
- B) The cafeteria
- C) The reception desk
- D) The server room only badge holders enter

Sources

1. U.S. HHS, Summary of the HIPAA Security Rule
2. U.S. HHS, Use of online tracking technologies by HIPAA covered entities and business associates
3. U.S. HHS, Business associate contracts: sample provisions
4. U.S. HHS, Breach Notification Rule
5. Future of Privacy Forum, A new paradigm for consumer health data privacy in Washington State (2023)
6. Federal Register, Health Breach Notification Rule (final rule) (May 30, 2024)

Answer key, Chapter 12: 1-B · 2-D · 3-A · 4-D · 5-A · 6-D · 7-A · 8-B · 9-B · 10-D

PART 3 · CHAPTER 13

Chapter 13: How the Platform Works

Four layers, one patient experience, and one line that PHI never crosses.

> 💡 **Trivia**
>
> Ongo's marketing site is built on **Next.js**, while patient screening and logins happen on separate routes (/weightloss-onboard/ and /login/). 🔵 Ongo internal
>
> That split isn't just technical. It's the line between "public website" and "protected health information".

13.1 The four layers

Layer	What it is	Who uses it
1. Marketing site	https://ongoweightloss.com/, built on Next.js. Where patients learn and decide	Everyone
2. Intake and patient portal	The screening (/weightloss-onboard/) and logged-in area (/login/). All PHI lives here	Patients
3. Provider interface	Where the provider reviews the intake, runs the visit and prescribes or declines	Providers
4. Pharmacy layer	The partner pharmacy receives the prescription, prepares, checks, packs and ships	Pharmacy

Ask the development lead which vendor powers layers 2 and 3 and where the BAA boundary sits. 🔵

> 🪔 **Desi analogy: shaadi ka pandal**
>
> Bahar ka gate aur reception sab ke liye khula hai (marketing site). Andar mandap mein sirf parivaar (patient portal). Pandit ji ka asan alag hai (provider interface). Aur halwai ka kitchen peeche (pharmacy).
>
> **Har jagah ka kaam alag, entry alag.** Reception wala mandap ki baatein bahar nahi sunata.

13.2 What each team touches

Team	Layer(s)	Rule
Content, SEO, design	1	No PHI, ever
Analytics	1, plus the click into 2	Track the start, never the answers
Developers	1 and integrations	Respect the BAA boundary
Medical team	3	Clinical decisions only
Operations	3 and 4	Fulfilment, shipping, support

> 📝 **The trust line.** The marketing site is for everyone. The portal is for the patient. PHI never crosses that line.

13.3 Do's and Don'ts

✅ Do	❌ Don't
Know which layer you work in	Assume the marketing site and portal share data
Ask where the BAA boundary sits	Add scripts that run inside the portal
Link to /weightloss-onboard/ from CTAs	Link to mid-flow steps
Treat the portal as off-limits for SEO	Try to index portal pages

13.4 Pros and cons of separating layers

✅ Pros	❌ Cons
Clear privacy boundary	Harder end-to-end analytics
Each team knows its zone	More integrations to maintain
Easier compliance reviews	Handoffs can break the patient experience

13.5 Where freshers go wrong in Chapter 13 topics

Mistake	Why it happens	Avoid it by	Fix it by
Linking to a mid-flow intake step	Copying a URL from the browser	Linking only to the entry point	Updating the link
Trying to index portal pages	"More pages = more SEO"	Knowing the portal is noindexed by design	Reverting the change
Adding a chat widget inside the portal	Good intentions	Asking the dev lead first	Removing it until approved
Not knowing the vendor	Never asked	Asking in week one	Writing it in your notes

13.6 Thumb rules

> 👍 Four layers, one line: PHI stays inside.
>
> 👍 Link to the front door, never a room inside.
>
> 👍 If you don't know your layer, ask.

13.7 Notes to remember

📝 Marketing site → intake/portal → provider interface → pharmacy.

📝 Only the marketing site is an SEO asset.

Chapter 13 Glossary

Term	Simple meaning
Next.js	The web framework Ongo's marketing site is built on
Intake	The online screening a patient completes
Patient portal	The logged-in area where PHI lives
Provider interface	The system providers use to review and prescribe
Noindex	A tag telling Google not to show a page in search results
Integration	A connection that passes data between systems

Chapter 13 Quiz

1. Where do patients learn and decide?

- A) The marketing site
- B) The pharmacy layer
- C) The provider interface
- D) Slack

2. Where does all PHI live?

- A) The blog
- B) The intake and patient portal
- C) Google Analytics
- D) The CMS

3. Who uses the provider interface?

- A) Providers, to review, visit and prescribe or decline
- B) The SEO team
- C) Patients only
- D) Social media managers

4. What does the pharmacy layer do?

- A) Runs ads
- B) Evaluates patients
- C) Receives the prescription, prepares, checks, packs and ships
- D) Writes blog posts

5. Which URL should CTAs link to?

- A) The intake entry point (/weightloss-onboard/)
- B) The provider interface
- C) The pharmacy portal
- D) A mid-flow intake step

6. Is the patient portal an SEO asset?

- A) Yes, the most important one
- B) Only on mobile
- C) Only the login page
- D) No

7. What should you ask the development lead?

- A) Which vendor powers layers 2 and 3 and where the BAA boundary sits
- B) Their favorite color
- C) Whether to add a pixel to the portal
- D) Nothing

8. Analytics may track…

- A) The start of the intake, never the answers
- B) Patient names
- C) Every intake answer
- D) Prescriptions

9. The marketing site is built on…

- A) WordPress
- B) Excel
- C) Paper
- D) Next.js

10. In the shaadi pandal analogy, the mandap is…

- A) The parking area
- B) The patient portal, for family only
- C) The kitchen
- D) The reception, open to all

Sources

1. Ongo Weight Loss, Homepage and site routes (checked October 2026)
2. Google Search Central, Block search indexing with noindex

Answer key, Chapter 13: 1-A · 2-B · 3-A · 4-C · 5-A · 6-D · 7-A · 8-A · 9-D · 10-B

Chapter 14: How We Work: From Booking to Your Door

This is the patient journey every team member must be able to describe in their own words.

> 💡 **Trivia**
>
> The FDA has received adverse-event reports where patients **measured doses incorrectly** with compounded injectables, sometimes because of confusion between units and milligrams. [2] ● Official
>
> That's why steps 07 and 08 below (the kit, and "start with your prescribed dose") matter as much as the visit itself.

14.1 The five core steps



Then: kit arrives → start your prescribed dose → weekly check-ins → dose adjustments → plan to stop safely

Step	What happens
1. Book an appointment	The patient completes a short online screening (health, goals, medications) and books a visit
2. Meet the doctor	A licensed provider in the patient's state meets them by video or phone

Step	What happens
3. The doctor evaluates	Reviews history, conditions, medications and any labs, and decides whether treatment is medically appropriate. **The doctor can say no**
4. Prescription is sent	If appropriate, the prescription is emailed to the patient and sent to our licensed partner pharmacy
5. Medication is shipped	The pharmacy prepares, quality-checks and ships the medication to the patient's door, usually within 3–5 business days

● Ongo internal [1]

14.2 The full journey on our How It Works page

#	Step (as on the site)	What happens
01	Take the screening	Health, goals and where you are in your weight-loss journey
02	Talk with a physician	If medication is appropriate, the provider prescribes what fits you
03	Get your personalized plan	Medication, dosing approach, progress tracking and support in one plan
04	Your medication is prepared	By state-licensed pharmacies, to your treatment plan
05	Quality & safety checks	Checks before the medication leaves the facility ("65+")
06	Securely packed & shipped	Packed to withstand temperature, weather and drops
07	Receive your kit	Supplies, instructions/tutorials and information to get started
08	Start with your prescribed dose	Exactly as prescribed; never change the dose yourself
09	Weekly check-ins	Share how you're responding and any side effects; the plan is adjusted
10	Discontinue sustainably	The provider helps plan a safe final step when it's time

Source: [1] ● Ongo live site

> 🍛 **Desi analogy: cricket ki innings**
>
> Toss (screening), team selection (doctor evaluation), batting order (plan), pitch report (quality checks), aur phir match shuru (pehli dose). Har over ke baad captain field set karta hai (weekly check-in). Aur innings ka end bhi plan se hota hai (discontinue sustainably).
>
> **Bina toss ke koi match nahi khelta, aur bina evaluation ke koi dawai nahi milti.**
>
> ⚠️ **The boundary we hold.** A consultation is a real medical evaluation. Providers can and do decline patients. A prescription is never guaranteed.

14.3 How to describe the journey

✅ Compliant	❌ Risky
"If your provider decides treatment is appropriate..."	"Once you're approved..."
"Usually ships within 3–5 business days."	"Arrives tomorrow, guaranteed."
"Use exactly the dose your provider prescribes."	"Adjust the dose to speed things up."

14.4 Do's and Don'ts

✅ Do	❌ Don't
Describe all five core steps in order	Skip the evaluation step in copy
Say a prescription isn't guaranteed	Imply approval once you pay
Explain the kit and tutorials	Show brand-style pens
Mention ending treatment safely	Pretend treatment never ends

14.5 Pros and cons of a detailed process page

✅ Pros	❌ Cons
Reduces anxiety and support tickets	Must stay in sync with operations
Builds trust and conversions	Numbers (like "65+") need proof
Strong "how it works" search match	Long pages need good design

14.6 Where freshers go wrong in Chapter 14 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Get approved in 24 hours"	Speed sells	"A provider decides" wording	Rewriting the promise
Using pen images for our product	Stock photos	Vial-and-syringe imagery	Replacing the image
Missing the "discontinue" step	Seems negative	Keeping all 10 steps	Adding it back
Quoting delivery as guaranteed	Over-simplifying	"Usually 3–5 business days"	Correcting the copy

14.7 Thumb rules

👍 Book → meet → evaluate → prescribe → ship.

👍 "If appropriate" belongs in every journey sentence.

👍 The prescribed dose, exactly; never self-adjust.

14.8 Notes to remember

📝 The prescription is emailed to the patient and sent to the partner pharmacy.

📝 Shipping usually takes 3–5 business days.

Chapter 14 Glossary

Term	Simple meaning
Screening	The short online health questionnaire
Evaluation	The provider's medical review of the patient
Treatment plan	Medication, dosing approach, tracking and support in one plan
Kit	Supplies, instructions and tutorials shipped with the medication
Discontinuation	Stopping treatment safely with provider guidance

Chapter 14 Quiz

1. What is the first core step?

- A) Book an appointment (complete the screening)
- B) Ship the medication
- C) Start the dose
- D) Pay for six months

2. Can the doctor say no?

- A) No, payment guarantees approval
- B) Only for tirzepatide
- C) Only on weekends
- D) Yes; providers can and do decline patients

3. Where is the prescription sent?

- A) Posted on social media
- B) Emailed to the patient and sent to our licensed partner pharmacy
- C) To any pharmacy the patient likes
- D) Nowhere

4. How long does shipping usually take?

- A) Three months
- B) About 3–5 business days
- C) One hour
- D) It's guaranteed next day

5. What is step 08 on the How It Works page?

- A) Skip doses when busy
- B) Double the dose
- C) Buy extra vials
- D) Start with your prescribed dose

6. What do weekly check-ins do?

- A) Share response and side effects so the plan can be adjusted
- B) Replace the evaluation
- C) Nothing
- D) Sell more products

7. Why does the kit matter?

- A) Dosing errors with compounded injectables are a known risk
- B) It's for marketing photos
- C) It replaces the provider
- D) It's just packaging

8. Which wording is compliant?

- A) "Once you're approved..."
- B) "If your provider decides treatment is appropriate..."
- C) "Instant approval"
- D) "Guaranteed prescription"

9. What is step 10?

- A) Cancel without telling anyone
- B) Switch to a brand
- C) Stop suddenly
- D) Discontinue sustainably, with a safe final step

10. In the cricket innings analogy, the toss is…

- A) The trophy
- B) The screening that starts everything
- C) The final over
- D) The pitch report

Sources

1. Ongo Weight Loss, How It Works (checked October 2026)
2. U.S. FDA, FDA's concerns with unapproved GLP-1 drugs used for weight loss (content current as of October 1, 2026)

Answer key, Chapter 14: 1-A · 2-D · 3-B · 4-B · 5-D · 6-A · 7-A · 8-B · 9-D · 10-B

PART 3 · CHAPTER 15

Chapter 15: Why Online Is Better Than Offline (For Most of Our Patients)

Telehealth wins on convenience, privacy and follow-up. In-person still wins in a few important cases, and we say so.

> 💡 **Trivia**
>
> HHS lists "you may get an appointment faster" and "a wider choice of providers, including those far away" among telehealth's benefits. [1] ● Official
>
> For a rural patient two hours from the nearest obesity specialist, that's the difference between treatment and no treatment.

15.1 Side by side

Online (telehealth)	Offline (in-person clinic)
From home, any device	Travel to a clinic
Visits often within days	Specialist waits can be weeks
No waiting room or time off work	Half a day per visit
Private	Some patients avoid clinics because of stigma
Medication shipped to the door	Pharmacy pickup and stock issues
Weekly check-ins and messaging	Follow-up means another appointment
Same licensed-provider standard	Same licensed-provider standard
No physical exam or in-office vitals	Weight, blood pressure and exam in person

Sources: [1] 🟢 and 🔵 expert judgment

15.2 When in-person still wins

- Patients who need a **physical exam**.
- **Complex diabetes** management.
- **Bariatric surgery** evaluation.
- Patients who simply **prefer** it.

> 🪔 **Desi analogy: Zoom shaadi vs. asli shaadi**
>
> Lockdown mein Zoom pe shaadi hui: rishtedaar ghar baithe judd gaye, kharcha kam, sab khush. Par pheron ke liye dulha-dulhan ko saath hona hi pada.
>
> **Telehealth zyaadatar kaam ghar se kar deta hai. Kuch cheezein phir bhi aamne-saamne hi hoti hain.** Dono ko imaandari se batana hamara kaam hai.

15.3 How to write the comparison

✅ Compliant	❌ Risky
"For many patients, telehealth is faster and more private."	"Clinics are a waste of time."
"Some patients need an in-person exam; we'll tell you if you do."	"You never need to see a doctor in person."
"Same licensed-provider standard."	"Better doctors than any clinic."

15.4 Do's and Don'ts

✅ Do	❌ Don't
Show both columns honestly	Attack in-person care
Say when in-person is better	Claim telehealth suits everyone

✅ Do	❌ Don't
Use HHS-style benefit language	Invent wait-time statistics

15.5 Pros and cons of comparison content

✅ Pros	❌ Cons
Answers "is online safe?" directly	Easy to overclaim
Builds trust through balance	Needs regular updating
Good for MOFU search intent	Can feel like an attack on doctors

15.6 Where freshers go wrong in Chapter 15 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Skip the doctor's office forever"	Catchy copy	The "when in-person wins" list	Rewriting with balance
Made-up wait-time numbers	Wanting stats	Using sourced data or none	Removing the number
Ignoring the physical-exam gap	One-sided thinking	The side-by-side table	Adding the limitation

15.7 Thumb rules

👍 Faster and more private, not better at everything.

👍 Name the cases where in-person wins.

👍 No invented statistics.

15.8 Notes to remember

📝 Same licensed-provider standard online and offline.

📝 Physical exam, complex diabetes and bariatric evaluation favor in-person care.

Chapter 15 Glossary

Term	Simple meaning
In-office vitals	Weight, blood pressure and similar measurements taken in person
Bariatric surgery	Weight-loss surgery
Asynchronous care	Messaging that doesn't require both people online at once
MOFU	Middle of funnel: people comparing options

Chapter 15 Quiz

1. Which is a telehealth advantage?

- A) No waiting room or time off work
- B) In-office vitals
- C) A physical exam
- D) Pharmacy pickup lines

2. When does in-person care still win?

- A) For simple check-ins
- B) Never
- C) When a physical exam is needed
- D) For reading a blog

3. Is the provider standard different online?

- A) Only on weekends
- B) No; it's the same licensed-provider standard

- C) Yes, there's no provider
- D) Yes, lower

4. Which line is compliant?

- A) "For many patients, telehealth is faster and more private."
- B) "You never need an in-person exam."
- C) "Better doctors than any clinic."
- D) "Clinics are a waste of time."

5. What does HHS list as a telehealth benefit?

- A) No need for a provider
- B) Free medication
- C) Guaranteed prescriptions
- D) You may get an appointment faster

6. Bariatric surgery evaluation is...

- A) Better done in person
- B) Done by text only
- C) Not a real thing
- D) Part of every Ongo plan

7. Can we invent wait-time statistics to make telehealth look better?

- A) Only if rounded
- B) Yes
- C) No
- D) Only in ads

8. What's the main gap of telehealth?

- A) No physical exam or in-office vitals
- B) Too fast
- C) Too private
- D) Too many check-ins

9. Comparison content like this mostly serves which funnel stage?

- A) None
- B) Post-purchase only

- C) MOFU (comparing)
- D) Checkout only

10. In the Zoom shaadi analogy, what still needed to happen in person?

- A) The music
- B) The guest list
- C) The invitations
- D) The pheras (some things need face-to-face care)

Sources

1. Telehealth.HHS.gov, Why use telehealth?

Answer key, Chapter 15: 1-A · 2-C · 3-B · 4-A · 5-D · 6-A · 7-C · 8-A · 9-C · 10-D

04

PART 4

The Business and Your Role

Who we are, who we serve, what we charge, who we compete with, and what to fix.

16 · Why We Exist

17 · What We Do, and Do NOT Do

18 · Where We Work: Parent, Child and Sister Sites

19 · Our Ideal Audience in the USA

20 · Who Commonly Qualifies

21 · Who Does NOT Qualify (in Most Cases)

22 · Our Pricing

23 · Our Brand Voice

24 · Our Competitors

25 · What Else to Mention, and Fix, on Our Live Site

26 · Your First Day on the Team

PART 4 · CHAPTER 16

Chapter 16: Why We Exist

Our mission, the facts every teammate must know by heart, and the values behind every page.

> 💡 **Trivia**
>
> Ongo Weight Loss was founded on **December 18, 2024**, the same month the FDA confirmed the tirzepatide shortage was over. [2] 🔵 Ongo internal [3] 🟢 Official
>
> Ongo was born into the toughest compliance era this industry has seen. Building it carefully from day one is part of our identity.

16.1 Our mission, in one sentence

Make quality weight management easier to access, and easier to stay committed to. [1] 🔵

> 🪔 **Desi analogy: mohalle ka family doctor**
>
> Purane zamane mein mohalle ka family doctor sab ko jaanta tha: kaun kya khaata hai, kis ko sugar hai, kaun dawai bhool jaata hai. **Ilaaj ke saath saath saath bhi deta tha.**
>
> Ongo wahi family doctor wala bharosa online de raha hai: access aasaan, aur saath lagataar.

16.2 Quick facts

Item	Detail
Brand name	Ongo Weight Loss (written "Ongo", never "ONGO")
Legal entity	Ongo Weight Loss, LLC
Owner / Founder & CEO	Deep Sharma (the site's About page says "Deep Chand Sharma"; confirm one spelling)

Item	Detail
Founded	**December 18, 2024**
Medical Director	Dr. Johnathan Chance Miller, MD
Website	https://ongoweightloss.com/
Start / screening	https://ongoweightloss.com/weightloss-onboard/
Patient login	https://ongoweightloss.com/login/
Treatments today	Only two: compounded semaglutide and compounded tirzepatide. No brand-name GLP-1s
Service area	United States; site says all 50 states (confirm with operations)
Model	100% virtual, self-pay; no insurance
Certification	LegitScript-certified

Sources: [1][2] ● Ongo

> 🟧 **Ongo example**
>
> The founding date belongs on the About page and in Organization schema (`foundingDate: 2024-12-18`). It's a simple, strong E-E-A-T signal, and it isn't on the site yet (Chapter 25).

16.3 Our values

Value	What it means in practice
Whole-person care	Beyond the number on the scale
Care without judgment	Every patient feels heard
Modern care, human at heart	Technology for convenience, clinicians at the center
Trust over transactions	Care doesn't end when the prescription is sent
Transparency	Clear prices, limits and disclosures

Value	What it means in practice
Compliance	HIPAA, FDA, FTC, state law and YMYL standards everywhere

The first four come from our About page. [1]

16.4 Do's and Don'ts

✅ Do	❌ Don't
Write "Ongo"	Write "ONGO" or "OnGo"
Use the founding date December 18, 2024	Guess a different year or "since 2020"
Use one agreed spelling of the founder's name	Mix "Deep Sharma" and "Deep Chand Sharma"
Quote the mission word for word	Rewrite the mission in every page

16.5 Pros and cons of a clear brand identity

✅ Pros	❌ Cons
Consistent entity signals for Google	Needs discipline across every team
Easier onboarding	Changes need sign-off everywhere
Builds patient trust	Errors are visible across the web

16.6 Where freshers go wrong in Chapter 16 topics

Mistake	Why it happens	Avoid it by	Fix it by
Writing "ONGO"	It's in the logo	The brand-name rule	Find-and-replace
Wrong founding year	Guessing	Using December 18, 2024	Correcting the page and schema

Mistake	Why it happens	Avoid it by	Fix it by
Two founder spellings	Copying different pages	Confirming one spelling	Updating every mention
Saying "Ongo offers Wegovy"	Assumption	The treatments row	Correcting immediately

16.7 Thumb rules

👍 One brand name, one founder spelling, one founding date.

👍 Two treatments, both compounded.

👍 Values are behaviors, not slogans.

16.8 Notes to remember

📝 Founded December 18, 2024. Founder & CEO Deep Sharma. Medical Director Dr. Johnathan Chance Miller, MD.

📝 Mission: make quality weight management easier to access, and easier to stay committed to.

Chapter 16 Glossary

Term	Simple meaning
Entity	A distinct thing Google understands (e.g. Ongo, semaglutide)
Organization schema	Machine-readable code describing a company (name, founder, founding date)
E-E-A-T	Experience, Expertise, Authoritativeness, Trust
LegitScript	Certification required by ad platforms for prescription telehealth
Mission	The one-sentence reason the company exists

Chapter 16 Quiz

1. When was Ongo Weight Loss founded?

- A) January 1, 2020
- B) June 2026
- C) December 18, 2024
- D) March 2013

2. How is the brand written?

- A) "Ongo"
- B) "ONGO"
- C) "ongo!"
- D) "OnGo"

3. What is Ongo's mission?

- A) Make quality weight management easier to access, and easier to stay committed to
- B) Replace all doctors
- C) Sell the most injections
- D) Offer every GLP-1 brand

4. Who is the Founder & CEO?

- A) Dr. Johnathan Chance Miller
- B) Unknown
- C) Deep Sharma
- D) A pharmacy

5. Which treatments does Ongo offer?

- A) Supplements only
- B) Every GLP-1
- C) Wegovy and Zepbound
- D) Compounded semaglutide and compounded tirzepatide only

6. Where should the founding date appear?

- A) The About page and Organization schema
- B) Only in internal emails

- C) Nowhere
- D) Only in ads

7. Which is one of Ongo's values?

- A) Speed over safety
- B) Secrecy
- C) Care without judgment
- D) Sales over care

8. Does Ongo bill insurance?

- A) Only for tirzepatide
- B) Yes, always
- C) No; it's self-pay
- D) Only Medicare

9. Who is the Medical Director?

- A) A Wegovy representative
- B) Deep Sharma
- C) Dr. Johnathan Chance Miller, MD
- D) The SEO lead

10. In the family-doctor analogy, what does Ongo add besides treatment?

- A) Ongoing support and continuity
- B) Free food
- C) Gossip
- D) Nothing

Sources

1. Ongo Weight Loss, About (checked October 2026)
2. Ongo Weight Loss leadership: founding date confirmed internally (October 2026)
3. U.S. FDA, FDA clarifies policies for compounders as national GLP-1 supply begins to stabilize

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Answer key, Chapter 16: 1-C · 2-A · 3-A · 4-C · 5-D · 6-A · 7-C · 8-C · 9-C · 10-A

Chapter 17: What We Do, and Do NOT Do

Knowing the "do not" list protects patients, the brand and your own work.

> 💡 **Trivia**
>
> Several of the June 2026 FDA warning letters cited telehealth websites that described a compounding pharmacy as **"FDA-approved" or "FDA-licensed"**. [1] ● Industry
>
> Half of this chapter's "do NOT" list exists because someone else got a letter for it.

17.1 What we DO

Service	Detail
Virtual evaluations	Weight-management evaluations by licensed U.S. providers
Prescriptions	Compounded semaglutide or compounded tirzepatide, when clinically appropriate
Fulfilment	Shipped from a licensed U.S. partner pharmacy, with a starter kit
Ongoing care	Weekly check-ins, dose adjustments, biweekly doctor Q&A, lab guidance, lifestyle guidance
Education	Provider-reviewed patient education

● Ongo internal [2]

17.2 What we DO NOT do

❌ We never…
Prescribe without an evaluation, or guarantee a prescription
Promise specific weight loss
Call compounded medication FDA-approved, generic, "the same as" a brand, or "clinically proven"
Call any pharmacy "FDA-approved"
Offer Wegovy, Ozempic, Rybelsus, Zepbound, Mounjaro, Foundayo, Saxenda or any brand-name GLP-1
Sell research peptides, drops, patches or supplements marketed as GLP-1s
Treat anyone under 18, pregnant, breastfeeding or trying to conceive
Bill insurance
Publish outside our verticals (no cannabis; that's our sister brand My MMJ Doctor)
Use shaming language, hype, or before/afters without consent and "results vary" context
Publish AI-written content end to end. AI can assist; humans write; providers review

> 🍛 **Desi analogy: restaurant ka menu**
>
> Achha restaurant menu pe wahi likhta hai jo kitchen sach mein bana sakta hai. "Chinese bhi milega, Italian bhi, sushi bhi" likhne wala restaurant aksar kuch bhi achha nahi banata.
>
> **Hamara menu chhota hai, par har dish imaandaar hai.** Jo menu mein nahi, woh hum offer nahi karte.
>
> 🟧 **Stay in the lane.** Ongo is a medical weight-management brand first. Longevity, men's health and women's health may follow, but only as planned silos.

17.3 Do's and Don'ts

✅ Do	❌ Don't
Describe only services that exist	Promise features "coming soon" as if live

✅ Do	❌ Don't
Mention provider review on content	Publish unreviewed AI drafts
Say who we can't treat	Hide exclusions
Keep My MMJ Doctor separate	Cross-link brands without approval

17.4 Pros and cons of a narrow offer

✅ Pros	❌ Cons
Clear message and topical authority	Fewer keywords to target today
Easier compliance	Some patients want more options
Focused operations	Growth depends on adding silos carefully

17.5 Where freshers go wrong in Chapter 17 topics

Mistake	Why it happens	Avoid it by	Fix it by
Publishing AI text without review	Deadlines	Human writing + provider review	Pulling it for review
Writing about cannabis on Ongo	Sister-brand overlap	Staying in our verticals	Moving it to the right brand
"Lose 30 lbs in 3 months"	Copying ads	"Individual results vary"	Removing the promise
Listing brand drugs as options	Assumption	The "do NOT" list	Correcting the page

17.6 Thumb rules

👍 If it isn't on the menu, we don't offer it.

👍 AI assists; humans write; providers review.

👍 The "do NOT" list is a shield, not a restriction.

17.7 Notes to remember

📝 Five things we do: evaluate, prescribe when appropriate, ship, support, educate.

📝 Eleven things we never do. Memorize them.

Chapter 17 Glossary

Term	Simple meaning
Vertical	A business area (e.g. weight loss, longevity)
Silo	A group of related pages on one topic
Sister brand	A separate brand under the same leadership (My MMJ Doctor)
Provider-reviewed	Checked and signed off by a named licensed provider
Hype	Exaggerated claims ("miracle", "melt fat")

Chapter 17 Quiz

1. Which is something Ongo DOES?

- A) Guarantee prescriptions
- B) Sell research peptides
- C) Virtual evaluations by licensed U.S. providers
- D) Bill insurance

2. Can we call our partner pharmacy "FDA-approved"?

- A) No
- B) Only in ads
- C) Yes
- D) Only if asked

3. Can Ongo treat a pregnant patient?

- A) No
- B) Only with tirzepatide
- C) Only after the first trimester
- D) Yes

4. What's the rule for AI-written content?

- A) Publish AI drafts directly
- B) AI is banned from all work
- C) AI can assist; humans write; providers review
- D) Only AI may write

5. Where does cannabis content belong?

- A) Ongo's pricing page
- B) Ongo's blog
- C) The sister brand My MMJ Doctor, not Ongo
- D) Ongo's homepage

6. Which is on the "do NOT" list?

- A) Promise specific weight loss
- B) Shipping a starter kit
- C) Weekly check-ins
- D) Provider-reviewed education

7. Which treatments does Ongo prescribe?

- A) Patches and drops
- B) Compounded semaglutide or compounded tirzepatide, when appropriate
- C) Supplements
- D) Any GLP-1 the patient wants

8. Before/afters are allowed...

- A) Only if blurred
- B) Always
- C) Only with consent and "results vary" context
- D) Never in any form

9. Future verticals (longevity, men's, women's health) should be added...

- A) Randomly in the blog
- B) Never
- C) Only as planned silos
- D) Before weight loss is solid

10. In the restaurant menu analogy, what's the lesson?

- A) Bigger menus are always better
- B) Offer only what the kitchen can truly make
- C) Menus don't matter
- D) Copy every restaurant

Sources

1. McDermott, FDA takes aim at misleading online claims for compounded GLP-1 drugs (June 2026)
2. Ongo Weight Loss, Homepage and Pricing (checked October 2026)

Answer key, Chapter 17: 1-C · 2-A · 3-A · 4-C · 5-C · 6-A · 7-B · 8-C · 9-C · 10-B

PART 4 · CHAPTER 18

Chapter 18: Where We Work: Parent, Child and Sister Sites

Know every property you might touch, and the rules for each.

> 💡 **Trivia**
>
> A `noindex` tag tells Google to keep a page out of search results, even if other pages link to it. [2] ● Official
>
> Ongo's two money pages (compounded semaglutide and compounded tirzepatide) were noindexed when this book was written. One tag can hide your most important pages from every patient searching for them.

18.1 Our properties

URL	Type	Notes
https://ongoweightloss.com/	Parent site	Next.js. All organic investment goes here first
/weightloss-onboard/ and /login/	Patient platform	PHI from here on; track the start, never the answers
https://weightclinicsresults.com/	Child site	Architecture being planned; ask the lead before linking
https://mymmjdoctor.com/	Sister brand (medical marijuana)	Separate audience; no cross-linking without approval
/services/glp-1/compounded-semaglutide/ and /compounded-tirzepatide/	Money pages	Currently noindex, nofollow (Chapter 25)
/pricing/, /how-it-works/, /eligibility/	Conversion path	/pricing/ and /how-it-works/ also noindex

URL	Type	Notes
/medical-team/, /blog/, /clinical-studies/, /faq/, /locations/, /policies/...	Trust and education	Every clinical claim should trace to a named reviewer

- Ongo internal [1]

18.2 The target architecture

Branch	Plan
Services → Weight Loss → GLP-1 medications	Compounded Semaglutide and Compounded Tirzepatide. Any Wegovy, Zepbound, Rybelsus, Mounjaro or Foundayo pages come only after the launch phase (Chapter 9.6), are educational/comparison only, and must say Ongo does not offer them
Services → Longevity, Men's Health, Women's Health	Later
Supporting sections	Blog topic hubs, Tools, Policies, Contributors, Medical Team, Reviews, About, Contact
Deliberately NOT built	Separate state, pricing, patient-education or patient-journey silos

- Ongo architecture decisions

> 🍛 **Desi analogy: joint family ka ghar**
>
> Bada ghar (parent site) jahan sab rehte hain, upar ki manzil pe naya kamra ban raha hai (child site), aur bagal mein chacha ka alag ghar (sister brand). **Chacha ke ghar ki chaabi apni jeb mein rakh ke ghoomna theek nahi.**
>
> Har property ka apna niyam. Linking ya content share karne se pehle lead se poocho.

18.3 Do's and Don'ts

✅ Do	❌ Don't
Invest in the parent site first	Spread effort across every domain
Ask before linking to the child site	Cross-link Ongo and My MMJ Doctor freely
Check indexing status before promoting a page	Build links to noindexed pages
Keep brand-drug pages educational	Let them imply we sell brands

18.4 Pros and cons of a parent-first strategy

✅ Pros	❌ Cons
Concentrates authority	Child site grows slower
Simpler measurement	Some keywords wait
Clear brand entity	Needs patience

18.5 Where freshers go wrong in Chapter 18 topics

Mistake	Why it happens	Avoid it by	Fix it by
Building links to noindexed pages	Not checking status	URL Inspection in Search Console	Pausing outreach; raising the noindex issue
Linking Ongo ↔ My MMJ Doctor	Same team	Asking first	Removing the link
Creating a state silo	Competitor copying	The "not built" list	Merging into existing pages
Writing a Wegovy page that sounds like a product page	Keyword focus	"Educational only" rule	Adding "Ongo does not offer this"

18.6 Thumb rules

👍 Parent first, child later, sister separate.

👍 Check indexing before you promote.

👍 Brand pages educate; they never sell.

18.7 Notes to remember

📝 Money pages and /pricing/, /how-it-works/ were noindexed in October 2026; confirm whether intentional.

📝 No separate state, pricing, education or journey silos.

Chapter 18 Glossary

Term	Simple meaning
Parent site	The main brand website (ongoweightloss.com)
Child site	A related site under the same brand (weightclinicsresults.com)
Money page	A page meant to convert visitors into patients
Nofollow	A hint telling Google not to follow links on a page
Silo / Hub-and-spoke	Grouping related pages / a hub linked to related spokes
Architecture	How a website's pages are organized and linked

Chapter 18 Quiz

1. Where does organic investment go first?

- A) The child site
- B) Social media only

- C) The parent site, ongoweightloss.com
- D) The sister brand

2. What does a noindex tag do?

- A) Makes a page rank higher
- B) Adds schema
- C) Speeds up a page
- D) Keeps a page out of Google's search results

3. What is mymmjdoctor.com?

- A) A sister brand with a separate audience
- B) A competitor we copy
- C) The patient portal
- D) Ongo's pricing page

4. Pages about Wegovy or Zepbound must...

- A) Sell the brands
- B) Not exist
- C) Be noindexed forever
- D) Be educational and say Ongo does not offer them

5. Which silo is deliberately NOT built?

- A) The GLP-1 medications silo
- B) The blog
- C) The medical team
- D) A separate state silo

6. What should you do before building links to a page?

- A) Change its URL
- B) Check that it's indexable
- C) Nothing
- D) Add a pixel

7. Every clinical claim in trust pages should trace to...

- A) An AI tool
- B) A competitor

- C) A social post
- D) A named reviewer

8. weightclinicsresults.com is...

- A) The pharmacy
- B) The sister brand
- C) A competitor
- D) The child site being planned

9. Can you cross-link Ongo and My MMJ Doctor freely?

- A) Only in blogs
- B) No; only with approval
- C) Yes
- D) Only in footers

10. In the joint family analogy, the sister brand is...

- A) Chacha's separate house next door
- B) The new upstairs room
- C) The garden
- D) The main house

Sources

1. Ongo Weight Loss, Homepage and site crawl (October 2026)
2. Google Search Central, Block search indexing with noindex

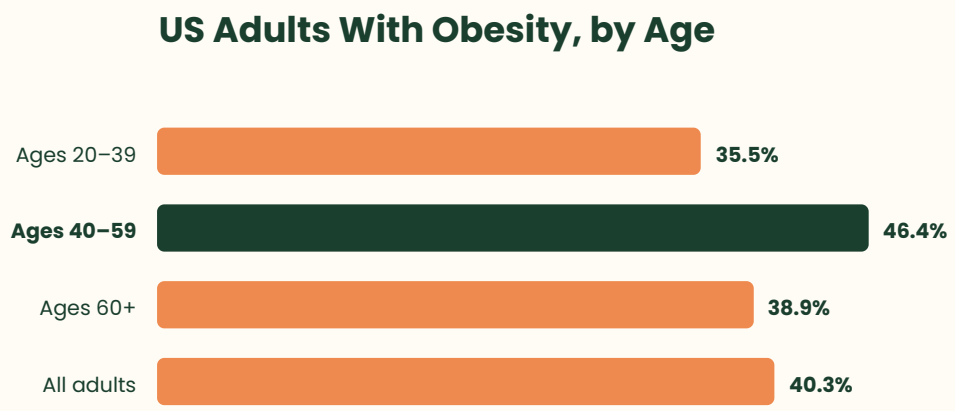
Answer key, Chapter 18: 1-C · 2-D · 3-A · 4-D · 5-D · 6-B · 7-D · 8-D · 9-B · 10-A

Chapter 19: Our Ideal Audience in the USA

Who our patients are, where they live, what they fear, and what they want.

> 💡 **Trivia**
>
> Obesity peaks in middle age: about **46.4%** of U.S. adults aged **40–59** have obesity, compared with about 35.5% at 20–39 and 38.9% at 60+. [1] ● Official
>
> That's why our core audience is 35–55: the age group with the highest need and, often, the busiest lives.

19.1 The market in numbers



Source: CDC NCHS Data Brief 508 (Aug 2021–Aug 2023).

Fact	Number	Source
U.S. adults with obesity	About **40.3%**	[1] ●

Fact	Number	Source
U.S. adults with severe obesity (BMI 40+)	About **9.4%**	[1]
Severe obesity, women vs. men	About **12.1% vs. 6.7%**	[1]
Obesity with a bachelor's degree or more	About **31.6%** (vs. about 45% without)	[1]
Regions with the highest obesity (2025)	**Midwest (35.5%)** and **South (34.8%)**	[2]
Adults who have ever taken a GLP-1	About **18%**; about **1 in 8** currently taking one (Nov 2025)	[3]

Obesity prevalence is also higher among American Indian or Alaska Native, Black and Hispanic adults in most states. [2]

19.2 The ideal Ongo patient: profile

Attribute	Ideal Ongo patient
Age	30–60, core 35–55
Gender	Mostly women (industry reports consistently show women are the majority of GLP-1 telehealth patients); a growing share of men
Health	BMI 30+, or 27+ with high blood pressure, prediabetes, high cholesterol, sleep apnea or joint pain
History	Has tried diets, apps or programs for years; regains weight; fights constant hunger
Money	Middle income; employed; no insurance coverage for weight medication (or denied); can budget $179–$249/month
Lifestyle	Busy (work, kids, shifts); values doing it from home
Location	Anywhere we serve; especially suburban and rural areas far from obesity specialists, and the South and Midwest
Digital habits	Searches Google, watches YouTube/TikTok, reads Reddit and reviews before choosing

Attribute	Ideal Ongo patient
Fears	Side effects, scams, being judged, hidden fees, "will it work for me?"
Wants	A real doctor, a clear price, privacy, support between visits, no insurance paperwork

- Ongo internal profile

19.3 Four personas (fictional)

Persona	Situation	What she or he needs from us
Maria, 47, Houston, TX	Office manager, BMI 33, prediabetes. Insurance denied Wegovy	A clear monthly price and a doctor who doesn't lecture her
David, 41, Columbus, OH	Shift worker, BMI 36, sleep apnea. Can't make clinic hours	An evening video visit and check-ins for side-effect questions
Janet, 55, Birmingham, AL	Weight gain after menopause, high blood pressure, BMI 29	Qualifies on BMI plus a condition; wants a slow, low-dose start
Keisha, 34, Atlanta, GA	Two young kids, BMI 31, tried every app	Searches "compounded semaglutide cost" at night on her phone; needs plain answers and trust signals before she books

> 🍛 **Desi analogy: shaadi ka guest list**
>
> Shaadi ka card sab ko nahi jaata. Pehle list banti hai: kaun aayega, kya khayega, kis ko wheelchair chahiye, kaun vegetarian hai. **Tabhi khana aur seating sahi banta hai.**
>
> Personas hamari guest list hain. Har page likhne se pehle socho: yeh Maria ke liye hai, ya Keisha ke liye?
>
> ⚠️ **Who we're NOT for.** Under 18; pregnant, breastfeeding or planning pregnancy; MTC/MEN 2 history; an active eating disorder; cosmetic goals below the thresholds; anyone wanting medication without an evaluation; people outside our served states.

19.4 Do's and Don'ts

✅ Do	❌ Don't
Write for a specific persona	Write for "everyone"
Answer fears directly (side effects, scams, fees)	Ignore objections
Use plain language at a mobile-friendly length	Write dense medical jargon
Respect who we're not for	Target teens or cosmetic searchers

19.5 Pros and cons of persona-based content

✅ Pros	❌ Cons
Content feels personal	Personas can become stereotypes
Better conversion	Need updating with real data
Easier briefs for writers	Fictional details must never look like real patients

19.6 Where freshers go wrong in Chapter 19 topics

Mistake	Why it happens	Avoid it by	Fix it by
Presenting personas as real patients	Wanting authenticity	Labeling them fictional	Removing from public pages
Writing only for women	Majority bias	Including men's concerns	Adding inclusive examples
Targeting "lose weight fast for teens"	Volume	The "not for" list	Removing the keyword
Ignoring rural patients	City bias	Using regional data	Adding access-focused content

19.7 Thumb rules

👍 Every page has a persona in mind.

👍 Answer the fear before you ask for the click.

👍 Personas are fictional, and labeled that way.

19.8 Notes to remember

📝 Core audience: 35–55, mostly women, BMI 30+ (or 27+ with a condition), uninsured for weight meds.

📝 Highest-need regions: Midwest and South.

Chapter 19 Glossary

Term	Simple meaning
Persona	A fictional profile representing a group of real patients
NHANES	The CDC's national health and nutrition survey
Prevalence	The share of a population with a condition
Objection	A reason a person hesitates to act
TOFU / MOFU / BOFU	Top / middle / bottom of funnel: learning, comparing, deciding

Chapter 19 Quiz

1. Which age group has the highest obesity rate (CDC)?

- A) 60+
- B) Under 18
- C) 40–59

- D) 20–39

2. Severe obesity is about how common in women vs. men?

- A) Twice as common in men
- B) Only in men
- C) About twice as common in women (12.1% vs. 6.7%)
- D) Equal

3. Which regions had the highest obesity prevalence in 2025?

- A) No differences
- B) Midwest and South
- C) Alaska and Hawaii only
- D) Northeast and West

4. What share of adults had ever taken a GLP-1 (KFF, Nov 2025)?

- A) About 80%
- B) None
- C) About 18%
- D) About 1%

5. What is Ongo's core age range?

- A) 35–55
- B) 70–90
- C) 18–21 only
- D) 13–19

6. Which is a common patient fear?

- A) Hidden fees
- B) Too clear a price
- C) Too much support
- D) Too private

7. Are the four personas real patients?

- A) Yes
- B) Some are
- C) No; they're fictional and must be labeled that way

- D) They're competitors' patients

8. Who are we NOT for?

- A) Busy parents
- B) People under 18
- C) Shift workers
- D) People in the South

9. Keisha searches "compounded semaglutide cost" at night. What does she need?

- A) A brand-drug offer
- B) A countdown timer
- C) Medical jargon
- D) Plain answers and trust signals

10. In the shaadi guest list analogy, personas are…

- A) The wedding venue
- B) The guest list that shapes the food and seating
- C) The decorations
- D) The DJ

Sources

1. CDC NCHS, Obesity and Severe Obesity Prevalence in Adults: United States, August 2021–August 2023 (Data Brief 508)
2. CDC, Adult Obesity Prevalence Maps (2025 data)
3. KFF, Poll: 1 in 8 adults say they are currently taking a GLP-1 drug (November 14, 2025)

Answer key, Chapter 19: 1-C · 2-C · 3-B · 4-C · 5-A · 6-A · 7-C · 8-B · 9-D · 10-B

Chapter 20: Who Commonly Qualifies

These criteria mirror the FDA labels for brand weight-management drugs. The provider always makes the final decision.

> 💡 **Trivia**
>
> The FDA labels for Wegovy and Zepbound use the same core thresholds for adults: **BMI 30 or higher**, or **BMI 27 or higher with at least one weight-related condition**. [1][2] ● Official
>
> Those two numbers are the backbone of every eligibility page, quiz and calculator we build.

20.1 The criteria

Route	Requirement
BMI route	BMI of 30 or higher
BMI + condition route	BMI of 27 or higher **plus at least one** weight-related condition

Weight-related conditions that commonly count

Condition	Note
High blood pressure	Common qualifier
Type 2 diabetes or prediabetes	Common qualifier
High cholesterol or triglycerides	Common qualifier
Obstructive sleep apnea	Common qualifier
Heart disease	Common qualifier

Condition	Note
Fatty liver disease (MASLD)	Common qualifier
Osteoarthritis made worse by weight	Provider judgment
PCOS	Case by case

- Ongo clinical guidance, based on labels [1][2]

> 🍛 **Desi analogy: cricket team ka selection**
>
> Selection ke liye minimum average chahiye (BMI 30), ya thoda kam average ke saath koi special skill (BMI 27 + condition). **Par final team selector hi chunta hai**, chahe stats kuch bhi kahein.
>
> Hamare provider wahi selector hain.
>
> 📝 **How we say it:** "Eligibility depends on your health history, BMI, conditions and medications. A licensed provider decides whether treatment is appropriate. Completing the screening doesn't guarantee a prescription."

20.2 Do's and Don'ts

✅ Do	❌ Don't
Show both routes clearly	Say "anyone can qualify"
List common conditions	Diagnose conditions for the reader
Repeat that the provider decides	Imply the quiz decides
Keep the BMI calculator accurate for all heights and weights	Block valid inputs

20.3 Pros and cons of publishing eligibility criteria

✅ Pros	❌ Cons
Sets honest expectations	Some visitors self-exclude

✅ Pros	❌ Cons
Reduces declined consultations	Needs careful wording
Strong "do I qualify" search match	Must stay aligned with labels

20.4 Where freshers go wrong in Chapter 20 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Qualify in 60 seconds"	Speed sells	"Provider decides" wording	Rewriting the CTA
Treating PCOS as automatic	Over-simplifying	"Case by case"	Correcting the list
Calculator errors for short patients	Bad input limits	Testing edge cases	Reporting to dev (Chapter 25)
Coaching what to enter	Wanting conversions	The integrity rule (Chapter 21)	Removing the advice

20.5 Thumb rules

👍 30, or 27 plus a condition.

👍 The screening informs; the provider decides.

👍 Never tell someone what to enter.

20.6 Notes to remember

📝 The criteria mirror brand weight-management labels (Wegovy, Zepbound).

📝 Completing the screening doesn't guarantee a prescription.

Chapter 20 Glossary

Term	Simple meaning
Weight-related condition	A health problem linked to excess weight, e.g. high blood pressure or sleep apnea
Prediabetes	Blood sugar higher than normal but below the diabetes range
PCOS	Polycystic ovary syndrome
Eligibility	Whether a patient may be considered for treatment
FDA label	The official prescribing information for an approved drug

Chapter 20 Quiz

1. What's the BMI-only route?

- A) BMI of 25 exactly
- B) BMI of 20 or higher
- C) Any BMI
- D) BMI of 30 or higher

2. What's the second route?

- A) BMI 27+ with no condition
- B) BMI 27+ plus at least one weight-related condition
- C) Any BMI with a doctor's note
- D) Age over 50

3. Who makes the final decision?

- A) The patient alone
- B) The licensed provider
- C) The marketing team
- D) The quiz

4. How is PCOS treated in our criteria?

- A) Automatic qualification

- B) Automatic exclusion
- C) Case by case
- D) Ignored

5. Which is a common qualifying condition?

- A) Short-sightedness
- B) A cold
- C) Hay fever
- D) Obstructive sleep apnea

6. Does completing the screening guarantee a prescription?

- A) Only with a 6-month plan
- B) Yes
- C) No
- D) Only for semaglutide

7. Where do these criteria come from?

- A) Guesswork
- B) Social media
- C) A competitor's ad
- D) The FDA labels for brand weight-management drugs

8. Which line is on-brand?

- A) "Qualify in 60 seconds, guaranteed."
- B) "Tell us you have diabetes to qualify."
- C) "Everyone qualifies!"
- D) "A licensed provider decides whether treatment is appropriate."

9. Why test the BMI calculator with short patients?

- A) For fun
- B) To slow the site
- C) Input limits can wrongly exclude people who qualify
- D) It's not necessary

10. In the cricket selection analogy, the provider is…

- A) The selector who makes the final call

- B) The scoreboard
- C) The crowd
- D) The batsman

Sources

1. Drugs.com, Wegovy prescribing information (indications)
2. Jastreboff AM et al., Tirzepatide Once Weekly for the Treatment of Obesity (SURMOUNT-1), NEJM (2022): enrolment thresholds BMI ≥30, or ≥27 with a weight-related condition

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Answer key, Chapter 20: 1-D · 2-B · 3-B · 4-C · 5-D · 6-C · 7-D · 8-D · 9-C · 10-A

Chapter 21: Who Does NOT Qualify (in Most Cases)

Saying no clearly is part of being a real medical brand.

> 💡 **Trivia**
>
> The American Diabetes Association recommends a **lower BMI screening threshold (23)** for Asian American adults, because health risks appear at lower BMIs. [1] 🟠 Industry report of an ADA position
>
> Cut-offs aren't one-size-fits-all, which is exactly why the provider's judgment matters more than any number on a page.

21.1 Common situations that, on their own, do NOT qualify

Situation	Why
BMI under 27	Below label thresholds
BMI 27–29.9 with no weight-related condition	Needs a condition to qualify
"The last 10 lbs" or a pre-event cosmetic goal	Not a medical indication
Athletic cutting or bodybuilding goals	Not a medical indication
Any contraindication in Chapter 6	Safety

21.2 The important nuance

Some people just outside the lines may still be appropriate:

- lower BMI thresholds are often used for people of Asian descent, [1] 🟠

- patients already on a GLP-1 are judged on their starting BMI. 🔵

And some inside the lines may not be appropriate (for example, a medication interaction). **The provider's judgment of the whole picture decides.**

> 🪔 **Desi analogy: IT office mein promotion**
>
> Promotion ke liye "3 saal ka experience" rule hai. Par kabhi 2.5 saal wala star performer bhi promote hota hai, aur kabhi 5 saal wala bhi nahi hota. **Rule guide karta hai, manager decide karta hai.**
>
> Eligibility bhi waisi hi hai: numbers guide karte hain, provider decide karta hai.
>
> ⚠️ **Our boundary.** We never tell a patient "we'll find a way to get you approved" or coach them on what to enter. If a provider declines, that's the integrity of the brand.

21.3 How to say no kindly

✅ Kind and honest	❌ Harsh or dishonest
"Based on your answers, medication may not be the right fit right now. Here are other options to discuss with your doctor."	"You don't qualify. Goodbye."
"A provider will review your full history."	"Just change your weight answer and try again."

21.4 Do's and Don'ts

✅ Do	❌ Don't
Explain why someone may not qualify	Shame people with lower BMIs
Offer helpful next steps	Coach answers
Respect provider decisions	Promise an appeal will approve them
Mention the Asian BMI nuance accurately	Turn the nuance into a loophole

21.5 Pros and cons of clear "no" messaging

✅ Pros	❌ Cons
Protects patients and the brand	Fewer sign-ups
Reduces refunds and complaints	Some negative reactions
Signals real medical care	Needs empathetic copy

21.6 Where freshers go wrong in Chapter 21 topics

Mistake	Why it happens	Avoid it by	Fix it by
Targeting "lose 10 pounds fast"	High volume	The "do not qualify" list	Re-targeting to medical intent
Telling someone how to qualify	Wanting to help	The boundary rule	Retracting; telling your lead
Cold rejection copy	Short on time	The kind-and-honest table	Rewriting with next steps
Calling the Asian threshold "a trick"	Misunderstanding	Explaining clinical reasons	Correcting the wording

21.7 Thumb rules

👍 Numbers guide; providers decide.

👍 We never coach answers.

👍 A kind no is still a no.

21.8 Notes to remember

📝 BMI under 27, cosmetic goals and contraindications usually don't qualify.

📝 Lower thresholds may apply for people of Asian descent; patients on a GLP-1 are judged on starting BMI.

Chapter 21 Glossary

Term	Simple meaning
Contraindication	A reason a treatment should not be used
Cosmetic goal	Weight loss for appearance only, below medical thresholds
Drug interaction	When one medicine changes how another works
Starting BMI	A patient's BMI before beginning treatment
Decline	A provider deciding treatment isn't appropriate

Chapter 21 Quiz

1. Does a BMI of 28 with no weight-related condition usually qualify?

- A) Only on a 6-month plan
- B) Only for tirzepatide
- C) No
- D) Yes

2. What BMI screening threshold does the ADA recommend for Asian American adults?

- A) 23
- B) 40
- C) 18
- D) 35

3. Patients already on a GLP-1 are judged on…

- A) Their current BMI only
- B) Their age
- C) Their payment plan
- D) Their starting BMI

4. Who decides in borderline cases?

- A) The provider, looking at the whole picture
- B) The quiz
- C) The marketing team
- D) The patient's friend

5. Which is NOT allowed?

- A) Explaining the criteria
- B) Coaching a patient on what to enter
- C) Respecting a provider's decision
- D) Offering next steps

6. A bodybuilder wants GLP-1s for cutting. They...

- A) Get brand drugs
- B) Get a discount
- C) Usually don't qualify
- D) Always qualify

7. Which rejection copy is on-brand?

- A) "You don't qualify. Goodbye."
- B) "Medication may not be the right fit right now. Here are other options to discuss with your doctor."
- C) "Try a competitor."
- D) "Change your answer and try again."

8. Can someone inside the BMI lines still be declined?

- A) Only in some states
- B) Yes, for example due to a medication interaction
- C) Only if they don't pay
- D) Never

9. A provider declining a patient represents…

- A) A bug
- B) The integrity of the brand
- C) A sales failure to fix
- D) A marketing problem

10. In the IT promotion analogy, what decides?

- A) The rule alone
- B) The employee alone
- C) The calendar
- D) The manager (the provider), guided by the rule

Sources

1. MLO Online, ADA releases position statement on new BMI screening cut points for diabetes in Asian Americans (screening at BMI ≥23 for Asian American adults)

Answer key, Chapter 21: 1-C · 2-A · 3-D · 4-A · 5-B · 6-C · 7-B · 8-B · 9-B · 10-D

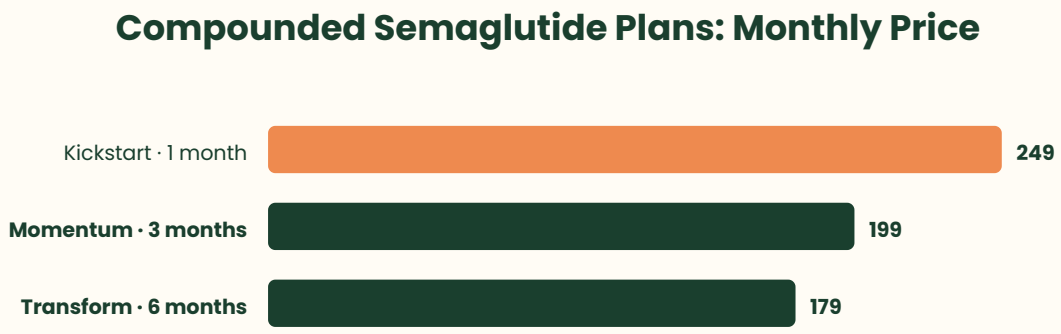
PART 4 · CHAPTER 22

Chapter 22: Our Pricing

Every team member must know the prices and keep them consistent everywhere.

> 💡 **Trivia**
>
> On Ongo's pricing page in October 2026, one FAQ says "medication costs are separate from Ongo's care fees", while a note on the same page says prices "reflect medication and ongoing care fees combined for illustration". [1] 🔵 Ongo live site
>
> Two sentences on one page that point in different directions. That's exactly the kind of thing patients notice, and exactly why this chapter has a "fix before you copy" box.

22.1 Plans by medication



US dollars per month. Confirm tirzepatide tiers with operations.

Plan	Commitment	Compounded semaglutide	Compounded tirzepatide	Savings vs. 1 month (sema)
Kickstart	1 month	$249 / month ($249 total)	From $269 / month	—
Momentum (most popular)	3 months	$199 / month ($597 total)	Tiers to confirm	About 20% ($50/month)

Plan	Commitment	Compounded semaglutide	Compounded tirzepatide	Savings vs. 1 month (sema)
Transform	6 months	$179 / month ($1,074 total)	Tiers to confirm	About 28% ($70/month)

Sources: [1] ● Ongo pricing page; tirzepatide "from $269" from the tirzepatide medication page

22.2 What every plan includes

Health history and goals review, personalized treatment, custom treatment plan, free health profile report, prescription evaluation, dosage plan, ongoing progress monitoring, priority support, lifestyle guidance and Ongo Premium Hub membership. No insurance needed. [1] ●

Plan	Physician consultations	Medication delivery
Kickstart	1	1 month
Momentum	3	3 months
Transform	6	6 months

22.3 How to explain it

Point	Plain explanation
One monthly price	Plans are positioned as care and medication together: provider visits, a personalized plan, prescription evaluation, medication delivery when prescribed, dose plan, check-ins, support and the member hub
Longer commitment, lower monthly price	Momentum saves about $50 a month and Transform about $70 a month compared with Kickstart
Why tirzepatide costs more	Tirzepatide is a dual-agonist molecule and its ingredient costs more, so its plans start higher
No insurance needed	Ongo is self-pay. The first step (the screening and information consultation) is $0

Point	Plain explanation
No guarantee	Paying for a plan doesn't guarantee a prescription or a result. Refunds and cancellation follow /policies/refund-policy/; quote it exactly

> 🍛 **Desi analogy: gym ki membership**
>
> Mohalle ka gym: ek mahine ka 2,500, teen mahine ka 6,000, chhe mahine ka 10,000. Lamba plan, sasta mahina. **Par membership se body guarantee nahi hoti**, aur rate board pe jo likha hai wahi counter pe bhi hona chahiye.
>
> Ongo ke plan bhi aise hi hain: lamba commitment, kam monthly price, aur har jagah ek hi rate.
>
> ⚠️ **Fix before you copy.** The tirzepatide page says "from $269", but the pricing page shows the same three plans under both tabs. The homepage says the price is "all-in", while a pricing FAQ says medication costs are separate. Until operations confirm, don't reuse either sentence or invent tirzepatide tiers.
>
> 📝 **The price-hygiene rule.** The price on any page must match checkout, schema, ads, emails and social posts. Change it anywhere → change it everywhere, and tell the on-page SEO lead. Competitor price comparisons must be dated and sourced.

22.4 Do's and Don'ts

✅ Do	❌ Don't
Quote prices exactly as confirmed	Round or "estimate" prices
Say "no insurance needed"	Say "cheapest in the U.S."
Mention no guarantee of prescription	Imply payment means approval
Update schema when prices change	Leave old prices in ads or emails

22.5 Pros and cons of flat monthly pricing

✅ Pros	❌ Cons
Easy to understand	Higher-dose costs may differ
No insurance paperwork	Commitment plans need clear refund terms
Strong "cost" search answer	Every channel must stay in sync

22.6 Where freshers go wrong in Chapter 22 topics

Mistake	Why it happens	Avoid it by	Fix it by
Inventing tirzepatide tiers	Filling gaps	"Tiers to confirm"	Removing until confirmed
Saying "all-in"	Copying the homepage	The fix-before-you-copy box	Using confirmed wording
Old price in Product schema	Forgot to update	Price-hygiene rule	Updating schema and resubmitting
"Cheaper than Wegovy"	Comparison temptation	Dated, sourced comparisons only	Removing or sourcing it

22.7 Thumb rules

👍 One price, everywhere.

👍 Paying never guarantees a prescription.

👍 If it isn't confirmed, it isn't published.

22.8 Notes to remember

📝 Semaglutide: $249 (1 month), $199 (3 months), $179 (6 months).

📝 Tirzepatide: "from $269"; other tiers to confirm.

Chapter 22 Glossary

Term	Simple meaning
Plan	A monthly care-and-medication package (Kickstart, Momentum, Transform)
Commitment	The number of months a plan covers
Price hygiene	Keeping the same price everywhere it appears
Product schema	Machine-readable code describing a product and its price
Refund policy	The published rules for cancellations and refunds

Chapter 22 Quiz

1. What is the Kickstart price for compounded semaglutide?

- A) $249 a month
- B) $1,349 a month
- C) $179 a month
- D) $99 a month

2. Which plan is labeled "most popular"?

- A) Kickstart (1 month)
- B) Transform (6 months)
- C) Momentum (3 months)
- D) None

3. What does Transform cost per month for semaglutide?

- A) $249
- B) $269
- C) $199
- D) $179

4. Why does tirzepatide cost more?

- A) It ships faster
- B) It doesn't cost more
- C) It's a dual-agonist molecule with a costlier ingredient
- D) It's a brand drug

5. What does the first step (screening and information consultation) cost?

- A) $50
- B) $249
- C) $0
- D) $179

6. Does paying for a plan guarantee a prescription?

- A) No
- B) Only for 6-month plans
- C) Yes
- D) Only for tirzepatide

7. What is the price-hygiene rule?

- A) Prices can differ by channel
- B) Only the homepage matters
- C) Round prices for simplicity
- D) The price must match everywhere: pages, checkout, schema, ads, emails and social

8. Can you publish tirzepatide 3- and 6-month tiers now?

- A) No; they're still to be confirmed
- B) Yes, estimate them
- C) Yes, copy semaglutide's
- D) Yes, from a competitor

9. How much does Momentum save vs. Kickstart per month?

- A) About $200
- B) Nothing
- C) About $50
- D) About $5

10. In the gym membership analogy, what doesn't membership guarantee?

- A) Gym access
- B) A result
- C) A lower monthly rate on longer plans
- D) The rate on the board

Sources

1. Ongo Weight Loss, Pricing and medication pages (checked October 2026)

Answer key, Chapter 22: 1-A · 2-C · 3-D · 4-C · 5-C · 6-A · 7-D · 8-A · 9-C · 10-B

PART 4 · CHAPTER 23

Chapter 23: Our Brand Voice

Warm, medical, calm and judgment-free. Not edgy, not shaming, not salesy.

> 💡 **Trivia**
>
> The FTC expects health claims to be backed by **competent and reliable scientific evidence**, and its 2024 rule bans fake reviews and testimonials, with civil penalties for violations. [1][2]
> ● Official
>
> Our voice isn't only a style choice. In health, the wrong sentence can be an illegal one.

23.1 The five voice attributes

Attribute	What it means
Empathetic	Patients have often been judged about their weight for years
Supportive	We guide; we don't pressure
Educational	Inform first, sell second
Professional	Accuracy before clever copy
Calm and clear	No countdown timers or fake urgency

● Ongo brand guidelines

23.2 Say this, not that

Good (compliant)	Bad (off-brand / risky)
"A licensed provider decides whether treatment is right for you."	"Guaranteed approval: get your shot today!"

Good (compliant)	Bad (off-brand / risky)
"Compounded semaglutide is not FDA-approved. Your provider decides if it's appropriate."	"Same as Ozempic for a fraction of the price."
"In clinical trials of Wegovy, adults lost about 15% of body weight on average."	"Lose up to 15% of your body weight with our semaglutide."
"Many patients see results; individual results vary."	"Melt fat fast, no diet needed!"
"Prepared by a licensed U.S. partner pharmacy."	"Made in an FDA-approved pharmacy."
"People living with obesity."	"Obese people" / "fat" / "ditch the belly".
"Reviewed by Dr. [Name], MD."	"Doctor-approved" with no name.

🍛 **Desi analogy: shaadi mein bua ji vs. maa**

Bua ji kehti hain "beta, thoda mota ho gaya hai, shaadi kaise hogi?" Maa kehti hai "beta, tabiyat kaisi hai, doctor ko dikhaya?" **Dono ki baat ka matlab ek ho sakta hai, par asar bilkul alag.**

Ongo ki awaaz maa wali hai: fikr ke saath, taane ke bina.

⚠️ **Words to avoid completely**

- Shame words: "obese people", "fat", "lazy"
- Guarantee words: "guaranteed", "everyone qualifies"
- Sameness words: "generic Ozempic", "same as Wegovy"
- Medical absolutes: "cures", "no side effects", "permanent"
- Approval words: "FDA-approved" for compounded products or pharmacies
- Hype: "miracle", "melt", "skinny shot"

23.3 Do's and Don'ts

✅ Do	❌ Don't
Name the reviewer	Write "doctor-approved" with no name

✅ Do	❌ Don't
Use calm CTAs ("Check if you qualify")	Use fake urgency ("Only 3 spots left!")
Name the product studied	Borrow brand trial results
Use real, consented reviews	Write or buy reviews

23.4 Pros and cons of a calm medical voice

✅ Pros	❌ Cons
Builds trust and repeat visits	Lower click-through than hype in some ads
Fewer compliance problems	Takes more skill to write
Fits YMYL expectations	Competitors may sound "louder"

23.5 Where freshers go wrong in Chapter 23 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Skinny shot" in social captions	Trend language	The words-to-avoid list	Editing the post
Countdown timers on landing pages	CRO templates	"Calm and clear" rule	Removing the timer
Anonymous "doctor-approved" badge	Template default	Named reviewer rule	Adding the reviewer's name and date
Writing a fake testimonial "for layout"	Placeholder habit	Using lorem ipsum labels	Deleting before publish

23.6 Thumb rules

👍 Speak like a caring doctor, not a hype ad.

👍 If it shames, guarantees or exaggerates, cut it.

👍 Named reviewer, or no "doctor" claim at all.

23.7 Notes to remember

📝 Five attributes: empathetic, supportive, educational, professional, calm and clear.

📝 Six word families to avoid: shame, guarantee, sameness, absolutes, approval, hype.

Chapter 23 Glossary

Term	Simple meaning
Brand voice	The consistent personality of our writing
Fake urgency	Pressure tactics like countdown timers or false scarcity
Testimonial	A patient's statement about their experience
Competent and reliable scientific evidence	The FTC's standard for health claims
CRO	Conversion rate optimization

Chapter 23 Quiz

1. Which phrase is on-brand?

- A) "Guaranteed approval!"
- B) "A licensed provider decides whether treatment is right for you."
- C) "Skinny shot"
- D) "Melt fat fast!"

2. Which is a "sameness" phrase to avoid?

- A) "Not FDA-approved"
- B) "Generic Ozempic"
- C) "Individual results vary"
- D) "Reviewed by Dr. Smith, MD"

3. What standard does the FTC expect for health claims?

- A) Competent and reliable scientific evidence
- B) No standard
- C) Customer opinions
- D) Social media likes

4. Countdown timers are...

- A) Required on all pages
- B) Fake urgency we don't use
- C) Fine for health claims
- D) Our signature style

5. Which is better?

- A) "Doctor-approved" with no name
- B) "Reviewed by Dr. [Name], MD"
- C) "Approved by experts"
- D) "Trusted by doctors everywhere"

6. Which is one of the five voice attributes?

- A) Sarcastic
- B) Educational
- C) Pushy
- D) Edgy

7. Which is a medical absolute to avoid?

- A) "No side effects"
- B) "Talk to your provider"
- C) "May cause nausea"
- D) "Results vary"

8. What does the FTC's 2024 rule ban?

- A) All reviews
- B) Real testimonials
- C) Star ratings
- D) Fake reviews and testimonials

9. "In clinical trials of Wegovy, adults lost about 15%..." is compliant because...

- A) It's short
- B) It uses a big number
- C) It mentions our product
- D) It names the product studied

10. In the bua ji vs. maa analogy, Ongo's voice is...

- A) Bua ji's: taunting
- B) Maa's: caring, without taunts
- C) Silent
- D) Loud and pushy

Sources

1. Federal Trade Commission, Health Products Compliance Guidance (2022)
2. Federal Trade Commission, Final rule banning fake reviews and testimonials (August 14, 2024)

Answer key, Chapter 23: 1-B · 2-B · 3-A · 4-B · 5-B · 6-B · 7-A · 8-D · 9-D · 10-B

Chapter 24: Our Competitors

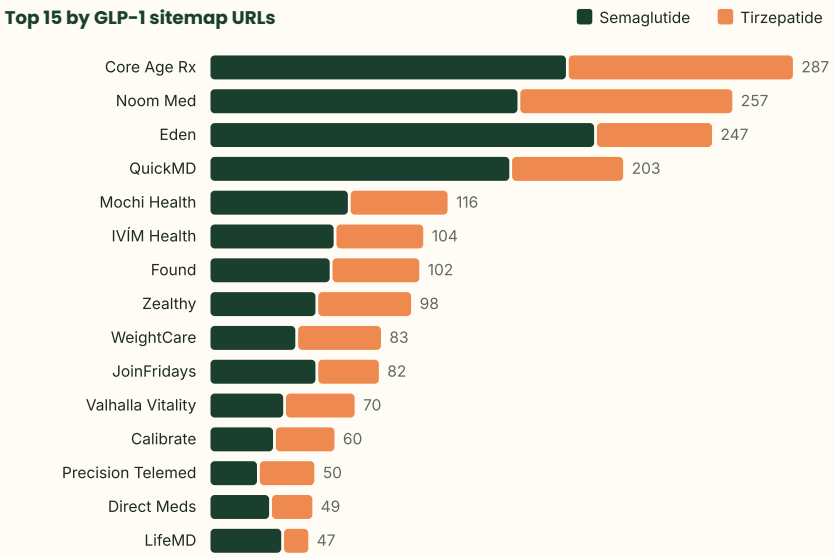
We operate in one of the most crowded telehealth categories in the U.S. We study competitors to understand why they built something, not to copy it.

> 💡 **Trivia**
>
> Across the 55 competitor sitemaps the team could read, there were about **1,420 semaglutide URLs** and **850 tirzepatide URLs**: semaglutide content outnumbers tirzepatide about **5 to 3**. 🔵 Ongo team scan, October 2026
>
> Fewer competitor pages on tirzepatide means a gap, and gaps are where a new brand can win.

24.1 What the 100-site scan shows

The team scanned 100 GLP-1 telehealth competitors' XML sitemaps. For each, the table shows whether the sitemap could be read, the total URLs, and how many URLs mention semaglutide or tirzepatide. 🔵

Finding	Number
Readable sitemaps	**55**
Blocked or gated (including Ro, Hims, Hers, Sequence/WeightWatchers, ShedRx, FuturHealth, Form Health, Remedy Meds and NextMed)	**45**, so the biggest brands are under-counted
Total URLs on readable sites	About **33,100**
Semaglutide URLs	About **1,420**
Tirzepatide URLs	About **850**
Readable competitors with zero sema/tirz URLs	**9**: they compete on brand and ads, not search



Source: team sitemap scan of 100 competitors (URLs containing each drug name). 45 sites blocked or gated the scan.

24.2 What it means for Ongo

Insight	Action
Topical depth wins	The leaders (Core Age Rx, Noom Med, Eden, QuickMD, Mochi) each have 100–290 semaglutide/tirzepatide URLs. Ongo's two money pages are currently noindex. We need a real GLP-1 → Semaglutide / Tirzepatide hub with supporting articles: quality over volume
Tirzepatide is under-served	Fewer competitor pages means an easier gap to fill with honest compounded-tirzepatide education
Size isn't the goal	Pure Pharmacy has 10,000+ URLs but only 19 semaglutide pages. Purpose matters more than page count
Verify before citing	Offers, prices and compliance status change often. Check a competitor's live site before naming them

> 🍛 **Desi analogy: cricket mein opposition analysis**
>
> Match se pehle team video analyst opposition ke har bowler ki footage dekhta hai: kaun short ball daalta hai, kaun yorker. **Copy karne ke liye nahi, apni strategy banane ke liye.**
>
> Competitor scan bhi wahi hai. Hum unki footage dekhte hain, apna game khelte hain.
>
> 📝 **How we win.** Not by being the loudest or cheapest. By being the clearest and most trustworthy: honest brand-vs-compounded education, named clinicians, real follow-up, consistent pricing, and a focused Weight Loss → GLP-1 silo that answers patient questions better than the other results.

24.3 The full list

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
1	Mochi Health	joinmochi.com	Readable	425	68	48
2	Henry Meds	henrymeds.com	Readable	43	4	1
3	OrderlyMeds	orderlymeds.com	Readable	139	4	2
4	Lavender Sky Health	lavenderskyhealth.com	Blocked / gated	—	—	—
5	Emerge Weight Loss	emergeweight.com	Readable	13	0	2
6	JoinFridays (Fridays Health)	joinfridays.com	Readable	166	52	30
7	IVÍM Health	ivimhealth.com	Readable	518	61	43
8	SlimDown Rx	slimdownrx.com	Blocked / gated	—	—	—
9	Pomegranate Health	joinpomegranate.com	Readable	76	4	3
10	Amble Health (JoinAmble)	joinamble.com	Readable	94	1	0

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
11	Brello Health	brellohealth.com	Readable	154	1	3
12	Eden (TryEden)	tryeden.com	Readable	1,042	190	57
13	FuturHealth	futurhealth.com	Blocked / gated	—	—	—
14	ShedRx (TryShed)	shedrx.com	Blocked / gated	—	—	—
15	ReflexMD	reflexmd.com	Readable	132	1	0
16	Remedy Meds	remedymeds.com	Blocked / gated	—	—	—
17	Fifty 410	fifty410.com	Readable	43	1	1
18	Zealthy	getzealthy.com	Readable	258	52	46
19	Ro (Ro Body)	ro.co	Blocked / gated	—	—	—
20	Hims Weight Loss	forhims.com	Blocked / gated	—	—	—
21	Hers Weight Loss	forhers.com	Blocked / gated	—	—	—
22	LifeMD	lifemd.com	Readable	1,403	35	12
23	WeightWatchers Clinic (Sequence)	sequence.com	Blocked / gated	—	—	—
24	Noom Med	noom.com	Readable	1,409	152	105
25	Found (JoinFound)	joinfound.com	Readable	507	59	43
26	Fella Health	fellahealth.com	Readable	1	0	0
27	Valhalla Vitality	valhallavitality.com	Readable	529	36	34
28	GobyMeds	gobymeds.com	Readable	240	28	17

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
29	TeleFitMD	telefitmd.com	Blocked / gated	—	—	—
30	Enbold	enbold.com	Readable	28	4	1
31	Trava Health	travahealth.com	Blocked / gated	—	—	—
32	Tree Top Health	treetophealth.net	Readable	23	0	0
33	Regenics	regenics.com	Readable	159	1	0
34	Strut Health	struthealth.com	Readable	273	1	4
35	Maximus Tribe	maximustribe.com	Readable	343	10	5
36	Defy Medical	defymedical.com	Readable	660	8	6
37	MinuteMD	minutemd.com	Readable	29	2	0
38	NextMed	nextmed.com	Blocked / gated	—	—	—
39	Skinny Rx	skinnyrx.com	Readable	41	5	11
40	Bright Meds	brightmeds.com	Blocked / gated	—	—	—
41	Morph Health	morphhealth.com	Blocked / gated	—	—	—
42	WeightCare	joinweightcare.com	Readable	238	42	41
43	AgelessRx	agelessrx.com	Readable	777	14	6
44	NuImage Medical	nuimagemedical.com	Readable	855	26	13
45	Fountain TRT / Fountain Health	fountaintrt.com	Blocked / gated	—	—	—
46	Zappy Health	zappyhealth.com	Readable	190	19	23

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
47	Reset Wellness	resetwellness.com	Blocked / gated	—	—	—
48	Core Age Rx	coreagerx.com	Readable	808	176	111
49	Alchemy Wellness Health	alchemywellnessweightcare.com	Readable	24	0	0
50	Unique Physique	uniquephysique.com	Readable	112	1	1
51	Good Life Meds	goodlifemeds.com	Blocked / gated	—	—	—
52	Direct Meds	directmeds.com	Readable	163	29	20
53	Helix MKE / Helix Mobile	helixmke.com	Blocked / gated	—	—	—
54	Invigor Medical	invigormedical.com	Readable	187	2	0
55	QuickMD	quick.md	Readable	1,154	148	55
56	Calibrate	joincalibrate.com	Readable	657	31	29
57	PlushCare	plushcare.com	Readable	1,098	7	9
58	Sesame Care	sesamecare.com	Blocked / gated	—	—	—
59	Everlywell	everlywell.com	Readable	3,649	40	2
60	Form Health	formhealth.com	Blocked / gated	—	—	—
61	JumpstartMD Telehealth	jumpstartmd.com	Readable	241	18	10
62	Red Mountain Weight Loss	redmountainweightloss.com	Readable	1,298	10	0
63	Renew Vitality	renewvitality.com	Blocked / gated	—	—	—

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
64	SemaSpace	semaspace.com	Blocked / gated	—	—	—
65	Peak Health MD	peakhealthmd.com	Blocked / gated	—	—	—
66	NuForm Health	nuformhealth.com	Readable	60	0	0
67	Opt Health	opthealth.com	Blocked / gated	—	—	—
68	Precision Telemed	precisiontelemed.com	Readable	120	23	27
69	Swift Clinic	swiftclinic.com	Blocked / gated	—	—	—
70	Royal Medical Center	royalmedicalcenters.com	Readable	112	1	3
71	Trimly	trimly.com	Blocked / gated	—	—	—
72	Vibrant Health Telemedicine	vibranthealth.com	Readable	528	0	0
73	Vitality Telehealth	vitalitytelehealth.com	Blocked / gated	—	—	—
74	Willow Weight Loss	willowweightloss.com	Blocked / gated	—	—	—
75	Zenith Weight Loss	zenithweightloss.com	Blocked / gated	—	—	—
76	Contour Lounge Telehealth	contourlounge.com	Blocked / gated	—	—	—
77	Balanced Hormone Health	balancedhormonehealth.com	Readable	580	26	19
78	Pure Harmony Clinic	pureharmonyclinic.com	Readable	116	0	0

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
79	Simple Scripts (Simple Health)	simplescripts.com	Blocked / gated	—	—	—
80	Lumos Care	lumoscare.com	Blocked / gated	—	—	—
81	PhySlim	physlim.com	Blocked / gated	—	—	—
82	Push Health (GLP-1 Provider Network)	pushhealth.com	Blocked / gated	—	—	—
83	Joy MD	joymd.com	Blocked / gated	—	—	—
84	Bioverse	getbioverse.com	Blocked / gated	—	—	—
85	Glow Diet / Glow Health	glowdiet.com	Readable	93	0	0
86	Levity (JoinLevity)	joinlevity.com	Readable	1	0	0
87	Alfie Health	alfiehealth.com	Blocked / gated	—	—	—
88	Enara Health	enarahealth.com	Readable	700	1	0
89	NuMed Telehealth	numedtelehealth.com	Blocked / gated	—	—	—
90	Hale Meds	halemeds.com	Blocked / gated	—	—	—
91	Empower Clinics Telehealth	empowerclinics.com	Blocked / gated	—	—	—
92	Pure Pharmacy Telehealth Solutions	purepharmacy.com	Readable	10,323	19	0

#	Competitor	Domain	Sitemap	Total URLs	Sema URLs	Tirz URLs
93	Trim Down Clinic	trimdownclinic.com	Blocked / gated	—	—	—
94	SlimRx (GetSlimRx)	getslimrx.com	Readable	17	1	1
95	Genesis Lifestyle Medicine	genesislifestylemedicine.com	Blocked / gated	—	—	—
96	Aspire Rejuvenation Telehealth	aspirerejuvenation.com	Readable	253	5	3
97	Revive Men's & Women's Telehealth	revivemed.com	Blocked / gated	—	—	—
98	Olympia Compounding Telehealth	olympiapharmacy.com	Blocked / gated	—	—	—
99	BeThin Telehealth	bethin.com	Blocked / gated	—	—	—
100	Matrix Hormones & Telemedicine	matrixreformed.com	Readable	6	0	0

Source: Ongo team sitemap scan, October 2026. "—" = sitemap blocked or gated. 🔵

24.4 Do's and Don'ts

✅ Do	❌ Don't
Ask "why did they build this?"	Copy pages because a competitor has them
Date any competitor fact	Quote an old price as current
Look for gaps (e.g. tirzepatide)	Chase raw page count
Compete on clarity and trust	Attack competitors by name

24.5 Pros and cons of sitemap scans

✅ Pros	❌ Cons
Fast view of content focus	Big brands often block sitemaps
Shows gaps and priorities	URL counts don't show quality
Repeatable every quarter	Needs manual checks for context

24.6 Where freshers go wrong in Chapter 24 topics

Mistake	Why it happens	Avoid it by	Fix it by
"They have 290 pages, so we need 300"	Volume thinking	"Size isn't the goal"	Planning a focused hub
Copying a competitor's claim	Assuming it's compliant	Checking against Chapter 7	Removing the claim
Naming competitors in negative copy	Wanting to stand out	Competing on our strengths	Rewriting neutrally
Treating blocked sites as small	Missing data	Remembering they're under-counted	Adding a manual review

24.7 Thumb rules

👍 Study why, not what.

👍 Depth beats volume.

👍 Never copy a claim you haven't checked.

24.8 Notes to remember

📝 55 readable, 45 blocked; about 1,420 sema and 850 tirz URLs.

📝 Tirzepatide is the under-served gap.

Chapter 24 Glossary

Term	Simple meaning
Sitemap (XML)	A file listing a site's URLs for search engines
Topical depth	How thoroughly a site covers a topic
Content gap	A topic competitors cover poorly or not at all
Hub	A main page linking to related supporting pages
Gated	Blocked from automated access

Chapter 24 Quiz

1. How many competitor sitemaps were readable?

- A) 100
- B) 55
- C) 5
- D) 0

2. Which drug has fewer competitor URLs?

- A) They're equal
- B) Semaglutide
- C) Neither has any
- D) Tirzepatide

3. Why are the biggest brands under-counted?

- A) They only use TikTok
- B) They have no pages
- C) Many of their sitemaps were blocked or gated
- D) They're not competitors

4. What does "size isn't the goal" mean?

- A) Purpose and quality matter more than page count
- B) Big sites always lose
- C) Nothing
- D) We should have fewer pages than everyone

5. What should Ongo build?

- A) A focused GLP-1 → Semaglutide / Tirzepatide hub with supporting articles
- B) Nothing
- C) A copy of a competitor's site
- D) 10,000 thin pages

6. Before naming a competitor fact, you should…

- A) Guess
- B) Ask social media
- C) Check their live site and date the fact
- D) Copy from an old article

7. Nine readable competitors had zero sema/tirz URLs. That means…

- A) They're out of business
- B) They don't sell anything
- C) They compete on brand and ads, not search
- D) They're Ongo's partners

8. How does Ongo plan to win?

- A) By being the loudest
- B) By attacking competitors
- C) By being the clearest and most trustworthy
- D) By being the cheapest at any cost

9. Pure Pharmacy has 10,000+ URLs but only 19 semaglutide pages. The lesson?

- A) They're the market leader
- B) Copy them
- C) Page count isn't topical depth
- D) More pages always win

10. In the cricket analysis analogy, why watch the opposition's footage?

- A) To copy their bowlers
- B) For entertainment
- C) To build our own strategy, not to copy them
- D) To report them

Sources

1. Ongo team, competitor XML sitemap scan of 100 GLP-1 telehealth brands (October 2026)

Answer key, Chapter 24: 1-B · 2-D · 3-C · 4-A · 5-A · 6-C · 7-C · 8-C · 9-C · 10-C

Chapter 25: What Else to Mention, and Fix, on Our Live Site

We scanned ongoweightloss.com in October 2026. These items aren't covered elsewhere in this book, but every team member should know them. Each one has an owner to confirm.

> 💡 **Trivia**
>
> FDA warning letters have repeatedly cited telehealth sites that attributed **brand trial results** (like "up to 15%" or "up to 20%" weight loss) to compounded products. [1] 🟠 Industry
>
> Our own semaglutide and tirzepatide pages used exactly that kind of line when this book was written. Fixing it is item #2 below.

25.1 Must fix (compliance and SEO risk)

#	Issue	Where	Why it matters
1	Service pages, How It Works and Pricing are set to noindex, nofollow	/services/glp-1/..., /how-it-works/, /pricing/	Our money pages can't rank or pass link value. Confirm whether intentional [2] 🟢
2	Compounded pages cite "up to 15% / 20%" weight loss	Semaglutide and tirzepatide pages	Brand trial results attributed to compounded products: the exact claim in FDA warning letters [1] 🟠
3	Footer says "FDA-approved medications are prescribed when clinically appropriate"	Site-wide footer	We don't offer FDA-approved medications
4	How It Works hero image shows Wegovy/Ozempic-style pens	/how-it-works/	Implies we ship brand pens; our product is vial + syringe

#	Issue	Where	Why it matters
5	Price wording conflicts	Homepage vs. pricing FAQ; tirzepatide $269 not on the pricing page	Inconsistent prices erode trust and break the price-hygiene rule
6	Before/after testimonials	Homepage, reviews	Need written consent and FTC "results not typical" disclosures [3] ●
7	"Board-certified" claim	Medical team / homepage	Dr. Miller is listed as General Practice; confirm the exact certification

● Ongo site scan, October 2026

25.2 Should fix (trust and clarity)

Item	Fix
No author or medical reviewer on service pages	Add "Reviewed by Dr. [Name], MD" with a date
"65+ quality checks" isn't explained anywhere	Say what they are, or remove the number
How It Works doesn't show the numbers for steps 05 and 07	Add them
Check-in frequency	"Weekly check-ins" on How It Works vs. "biweekly doctor Q&A" elsewhere. Explain how they differ
Founder name	"Deep Chand Sharma" vs. "Deep Sharma". Pick one everywhere, including schema
Founding date (December 18, 2024) isn't on the About page	Add it to the About page and Organization schema (`foundingDate`) for E-E-A-T
BMI calculator only accepts weights of 150 lb or more	It fails some shorter patients who still qualify
Kit contents aren't listed	Tell patients exactly what's in the box (vials, syringes, swabs, sharps guidance, tutorials)

25.3 Strengths to mention in our content

- The **Quality & Safety** page and the **Patient Bill of Rights**: link to them from money pages.
- **LegitScript certification**: show the badge near every CTA.
- Published **Consumer Health Privacy** and **California Privacy** notices.
- The **$0 information consultation** and no-insurance model.

> 🪔 **Desi analogy: shaadi se pehle ghar ki safai**
>
> Rishta dekhne log aane wale hain. Ghar mein sab achha hai, par ek nal tapak raha hai aur deewar pe purani tasveer ulti lagi hai. **Mehmaan wahi dekhte hain jo galat hai.**
>
> Patients aur Google bhi aise hi hain. Pehle "must fix" theek karo, phir "strengths" sajao.

25.4 Do's and Don'ts

✅ Do	❌ Don't
Report issues to the owner, with the URL	Quietly edit live pages without approval
Fix must-fix items before new content	Build links to noindexed pages
Use strengths as trust signals near CTAs	Repeat a known issue in new content
Re-check the list every month	Assume it's fixed because someone said so

25.5 Pros and cons of a public fix list (internal)

✅ Pros	❌ Cons
Everyone knows the risks	Can feel negative to new teammates
Clear owners and priorities	Goes stale if not updated
Prevents repeating mistakes	Needs leadership follow-through

25.6 Where freshers go wrong in Chapter 25 topics

Mistake	Why it happens	Avoid it by	Fix it by
Copying "up to 20%" into a new blog	It's on our own site	This chapter	Removing; flagging the source page
Promoting a noindexed page	Not checking	URL Inspection	Pausing promotion
Using the pen image in a new ad	Reusing assets	Vial-and-syringe imagery	Swapping the image
Writing a different founding year	Guessing	December 18, 2024	Correcting everywhere

25.7 Thumb rules

👍 Fix before you build.

👍 Never repeat a known issue in new work.

👍 Every issue has an owner and a date.

25.8 Notes to remember

📝 Seven must-fix items: noindex, brand trial claims, footer, pen image, prices, testimonials, "board-certified".

📝 Add the founding date (December 18, 2024) to the About page and schema.

Chapter 25 Glossary

Term	Simple meaning
Must fix	An issue with compliance or major SEO risk

Term	Simple meaning
Trust signal	Evidence of legitimacy near a decision point
foundingDate	The schema property for when an organization was founded
Patient Bill of Rights	A published statement of patient rights
Board-certified	Certified by a recognized medical specialty board

Chapter 25 Quiz

1. Which issue blocks our money pages from ranking?

- A) They're too long
- B) They're set to noindex, nofollow
- C) They load fast
- D) They have images

2. What's wrong with "up to 15% / 20%" on our compounded pages?

- A) Nothing
- B) It attributes brand trial results to compounded products
- C) It should say 30%
- D) The numbers are too low

3. What's wrong with the site footer line?

- A) It's in the wrong font
- B) It says FDA-approved medications are prescribed, which we don't offer
- C) It's too short
- D) Nothing

4. Why replace the How It Works hero image?

- A) It shows brand-style pens; we ship vial + syringe
- B) It's too small
- C) It's a stock photo of food
- D) It's too bright

5. What should be added to the About page?

- A) Patient names
- B) A countdown timer
- C) Competitor prices
- D) The founding date, December 18, 2024

6. What do before/after testimonials need?

- A) Nothing
- B) Written consent and "results not typical" disclosures
- C) Bigger photos
- D) Filters

7. Which is a strength to mention?

- A) The pen image
- B) The noindex tags
- C) The footer line
- D) LegitScript certification

8. What's wrong with the BMI calculator?

- A) It only accepts weights of 150 lb or more
- B) Nothing
- C) It's too accurate
- D) It's in kilograms

9. Should you quietly edit live pages to fix issues?

- A) No; report to the owner and get approval
- B) Yes, always
- C) Only the footer
- D) Only at night

10. In the ghar ki safai analogy, what do guests notice first?

- A) The furniture price
- B) What's wrong, so fix the must-fix items first
- C) Nothing
- D) The music

Sources

1. Igility, What all 113 FDA telehealth warning letters actually say (analysis of FDA letters, 2025–2026)
2. Google Search Central, Block search indexing with noindex
3. Venable, FTC finalizes updated Endorsement and Testimonial Guides (July 5, 2023)

Answer key, Chapter 25: 1-B · 2-B · 3-B · 4-A · 5-D · 6-B · 7-D · 8-A · 9-A · 10-B

Chapter 26: Your First Day on the Team

Find yourself below. Each role gets its own deeper handbook later. Book Two covers off-page SEO.

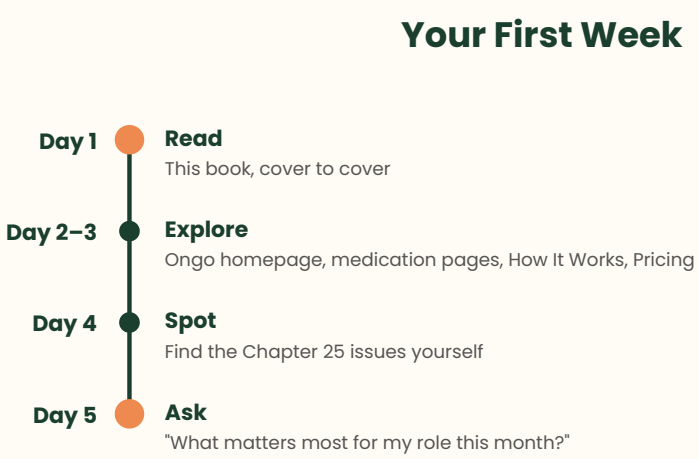
> 💡 **Trivia**
>
> Google says content on health topics needs especially high trust, and that clearly showing **who** created content, and their expertise, helps people judge it. [1] ● Official
>
> That's why every role in this chapter, from writer to developer, connects back to one thing: making Ongo trustworthy.

26.1 Roles and what they own

Role	Owns
Manager / Project Lead	Priorities and scope sign-off
Team Leader	One workstream (content, SEO, off-page, social)
Senior SEO / Strategist	Architecture, entities, keyword research, briefs, the master content map
On-Page SEO	Titles, metas, headings, internal links, schema, canonicals
Off-Page SEO	Backlinks, citations, NAP, digital PR (see Book Two)
Technical SEO	Next.js rendering, sitemaps, robots, redirects, indexation (start with the noindex issue), Core Web Vitals
Measurement / Analytics	GTM, GA4, GSC, CTA events and attribution; no PHI
Content Writer / Senior Editor	Drafts, clusters, fact-checks, regulatory language
Medical Reviewer (MD)	Named sign-off on every clinical claim

Role	Owns
Compliance Reviewer	FDA/FTC/state-law language before anything ships
Graphic Designer	Trust badges, doctor headshots, diagrams like the ones in this book, injection-guide visuals
Video Editor	Injection guides, doctor intros, "what to expect" walkthroughs
Social Media Manager	Educational, on-brand content within platform health-ad rules
Front-End / Back-End Developers	Next.js pages, speed, intake integrations, consent and tag governance, the HIPAA boundary

26.2 Your first week



When	Task
Day 1	Read this book cover to cover
Days 2–3	Spend time on https://ongoweightloss.com/: the homepage, both medication pages, How It Works, Pricing and Quality & Safety
Day 4	Spot the issues in Chapter 25 yourself
Day 5	Ask your team lead: "What's the single most important thing for my role this month?"

> 🪔 **Desi analogy: IT office ka pehla din**
>
> Naye joiner ko pehle din koi production server ka access nahi deta. Pehle induction, phir buddy, phir chhota task. **Jaldi mein sab chhoo liya toh pehle hafte hi incident ho jaata hai.**
>
> Ongo mein bhi pehla hafta seekhne ka hai. Kaam baad mein, samajh pehle.
>
> 📝 **Welcome to the team.** People arriving at Ongo have often spent years being judged about their weight, and many are nervous about a new medication. Treat every page, video, post and line of code with that in mind. We are a healthcare brand first and a marketing operation second.

26.3 Do's and Don'ts

✅ Do	❌ Don't
Read this book before your first task	Publish anything in week one without review
Know who reviews your work	Skip medical or compliance review
Ask questions early	Guess on regulatory or medical points
Note the Chapter 25 issues	Repeat them in new work

26.4 Pros and cons of a slow, structured onboarding

✅ Pros	❌ Cons
Fewer costly mistakes	Feels slow at first
Shared language across roles	Requires leads' time
Faster work after week one	Needs updated materials

26.5 Where freshers go wrong in Chapter 26 topics

Mistake	Why it happens	Avoid it by	Fix it by
Publishing on day two	Eagerness	The first-week plan	Pulling it for review
Not knowing your reviewer	Never asked	Asking on day one	Adding it to your notes
Working outside your role	Wanting to help	The roles table	Handing it to the right owner
Skipping the site walk-through	"I know websites"	Doing days 2–3 properly	Doing it now

26.6 Thumb rules

👍 Learn first, publish second.

👍 Every piece of work has a named reviewer.

👍 Healthcare brand first, marketing operation second.

26.7 Notes to remember

📝 Medical Reviewer signs every clinical claim; Compliance Reviewer signs regulatory language.

📝 Your first question to your lead: "What's the single most important thing for my role this month?"

Chapter 26 Glossary

Term	Simple meaning
SEO	Search Engine Optimization

Term	Simple meaning
GSC / GA4 / GTM	Search Console / Analytics / Tag Manager
CTA / Conversion / Attribution	Call to action / completed action / linking a conversion to its source
Schema	Machine-readable code describing a page
NAP	Name, address, phone: consistent business details across the web
Core Web Vitals	Google's page-experience speed and stability metrics

Chapter 26 Quiz

1. What should you do on Day 1?

- A) Change the pricing page
- B) Read this book cover to cover
- C) Add a tracking pixel
- D) Publish a blog post

2. Who signs off every clinical claim?

- A) The Social Media Manager
- B) The Graphic Designer
- C) The Medical Reviewer (MD)
- D) Nobody

3. Who owns indexation and the noindex issue?

- A) Social media
- B) The Video Editor
- C) The Compliance Reviewer
- D) Technical SEO

4. What must Measurement / Analytics never collect?

- A) Page views
- B) CTA clicks
- C) PHI

- D) Traffic sources

5. Which book in the series covers off-page SEO?

- A) Book Two
- B) Book Ten
- C) None
- D) Book One

6. What's the question to ask your lead on Day 5?

- A) "What's the single most important thing for my role this month?"
- B) "When is my raise?"
- C) "Can I add a pixel?"
- D) "Can I skip review?"

7. Who reviews FDA/FTC/state-law language?

- A) The Front-End Developer
- B) The Compliance Reviewer
- C) The Off-Page SEO
- D) The Graphic Designer

8. What does Google say helps people judge health content?

- A) More ads
- B) Longer word counts
- C) Clearly showing who created it and their expertise
- D) Hidden authors

9. Ongo is...

- A) A healthcare brand first and a marketing operation second
- B) A pharmacy
- C) A marketing operation first
- D) An insurance company

10. In the IT office analogy, why no production access on day one?

- A) Servers are off on Mondays
- B) Rushing in causes incidents; learning comes first
- C) New joiners can't use computers

- D) No reason

Sources

1. Google Search Central, Creating helpful, reliable, people-first content (last updated October 5, 2026)

Answer key, Chapter 26: 1-B · 2-C · 3-D · 4-C · 5-A · 6-A · 7-B · 8-C · 9-A · 10-B

Back Matter

Quick-Reference Cards

Card 1: Ongo quick facts

Item	Detail
Brand	Ongo Weight Loss (written "Ongo")
Legal entity	Ongo Weight Loss, LLC
Founded	December 18, 2024
Founder & CEO	Deep Sharma (confirm one spelling everywhere)
Medical Director	Dr. Johnathan Chance Miller, MD
Treatments	Compounded semaglutide and compounded tirzepatide only
Model	100% virtual, self-pay, no insurance
Certification	LegitScript-certified
Start	https://ongoweightloss.com/weightloss-onboard/

Card 2: The patient journey

Step	What happens
1	Book an appointment (online screening)
2	Meet the doctor (video or phone)
3	The doctor evaluates and can say no
4	Prescription emailed to the patient and sent to the partner pharmacy
5	Medication prepared, checked and shipped (usually 3–5 business days)

Card 3: Say this, not that

✅ Say	❌ Never say
"Compounded semaglutide is not FDA-approved."	"Generic Ozempic" / "same as Wegovy"
"A licensed provider decides."	"Guaranteed approval"
"In the STEP 1 trial of Wegovy..."	"Lose up to 15% with our semaglutide"
"Licensed U.S. partner pharmacy"	"FDA-approved pharmacy"
"People living with obesity"	"Obese people"
"Individual results vary."	"No side effects" / "permanent results"

Card 4: Red-word list

Family	Words
Shame	"obese people", "fat", "lazy", "chubby"
Guarantee	"guaranteed", "everyone qualifies", "instant approval"
Sameness	"generic", "same as", "equivalent to", "same active ingredient"
Absolutes	"cures", "no side effects", "permanent"
Approval	"FDA-approved" (for compounded products or pharmacies), "FDA-licensed"
Hype	"miracle", "melt", "skinny shot", "fat-burning"

Master Checklists

Checklist 1: Before you publish anything about medication

☐	Check
☐	Compounded products are clearly labeled "not FDA-approved"
☐	No sameness words (generic, same as, equivalent, same active ingredient)
☐	Any trial result names the brand product studied

☐	Check
☐	Benefits are paired with limits and "results vary"
☐	Contraindications and side effects are listed or linked near the CTA
☐	Pharmacy is described as "licensed U.S. partner pharmacy" (never named or "FDA-approved")
☐	Prices match the confirmed pricing everywhere
☐	Named medical reviewer and review date are shown
☐	Compliance has reviewed any regulatory statement
☐	No red words (Card 4)

Checklist 2: Privacy and tracking

☐	Check
☐	No PHI on the marketing site, CMS, Slack, email or design files
☐	GA4 tracks the CTA click or intake start, never the answers
☐	No ad pixels on intake or portal pages
☐	Any new tag approved by the analytics and compliance owners
☐	Patient stories have documented written consent and "results vary" context

Checklist 3: Before creating a state or comparison page

☐	Check
☐	Operations confirms the state is actively served (and its category)
☐	Content is genuinely unique, not a city-name swap
☐	Brand-drug pages say "Ongo does not offer this medication" above the fold
☐	No brand names (Wegovy, Ozempic, Zepbound, Mounjaro...) in ads, ad keywords, landing pages or commercial meta titles during the launch phase
☐	Every approval date or status is dated and sourced

Glossary (A to Z)

Term	Simple meaning	Ch.
503A pharmacy	State-licensed compounding pharmacy filling patient-specific prescriptions	4
503B bulks list	The list of bulk ingredients outsourcing facilities may use	7
503B outsourcing facility	FDA-registered facility that can compound larger batches	4
Amylin	A pancreatic hormone that adds fullness	2
API	Active pharmaceutical ingredient, the molecule that does the work	4
Architecture	How a website's pages are organized and linked	18
Aspiration	Stomach contents entering the lungs, a risk during anesthesia	6
Asynchronous care	Messaging that doesn't require both people online at once	15
BAA	Business Associate Agreement: the vendor contract for PHI	12
Bariatric surgery	Weight-loss surgery	15
BMI	Body Mass Index: weight (kg) ÷ height (m)². A screening tool, not a diagnosis	1
Board-certified	Certified by a recognized medical specialty board	25
Boxed warning	The FDA's strongest label warning; brand GLP-1s carry one for thyroid C-cell tumors	6
Brand voice	The consistent personality of our writing	23
Breach	Unauthorized access to or disclosure of PHI	12
BUD	Beyond-use date: the date after which a compounded medication shouldn't be used	4
Cease-and-desist letter	A formal demand to stop an activity before a lawsuit	9
Check-in	A scheduled follow-up on progress and side effects	11
Child site	A related site under the same brand (weightclinicsresults.com)	18

Term	Simple meaning	Ch.
Cleanroom	A controlled, filtered space where sterile medications are prepared	4
"Clinically proven"	A claim we never make for compounded products	5
Clinical reason	A documented medical justification for a specific patient	9
Cold chain	Temperature-controlled shipping	4
Commitment	The number of months a plan covers	22
Competent and reliable scientific evidence	The FTC's standard for health claims	23
Compounded medication	A drug prepared by a licensed pharmacy for one patient's prescription; not FDA-approved	4
Consumer health data	Health data covered by state laws like Washington's My Health My Data Act	8
Content gap	A topic competitors cover poorly or not at all	24
Continuity of care	Ongoing follow-up after the first prescription	10
Contraindication	A reason a treatment should not be used	6
Core Web Vitals	Google's page-experience speed and stability metrics	26
Cosmetic goal	Weight loss for appearance only, below medical thresholds	21
Counterfeit	A fake product made to look like the real brand	10
CRO	Conversion rate optimization	23
CTA / Conversion / Attribution	Call to action / completed action / linking a conversion to its source	26
Decline	A provider deciding treatment isn't appropriate	21
Discontinuation	Stopping treatment safely with provider guidance	14
Doorway page	A thin page built only to rank (e.g. near-identical city pages). Forbidden	8

Term	Simple meaning	Ch.
Dose adjustment	A provider changing the dose based on response	11
DPP-4	The enzyme that breaks down natural GLP-1 within minutes	2
Drug interaction	When one medicine changes how another works	21
Drug shortage list	The FDA list that temporarily allowed wide compounding until 2024–2025	7
Dual agonist	A drug that activates two hormone receptors	2
Duopoly	A market controlled by two companies	9
E-E-A-T	Experience, Expertise, Authoritativeness, Trust	16
Eligibility	Whether a patient may be considered for treatment	20
Entity	A distinct thing Google understands (e.g. Ongo, semaglutide)	16
"Essentially a copy"	The FDA's term for a compounded drug too similar to an approved one	7
Evaluation	The provider's medical review of the patient	14
Fake urgency	Pressure tactics like countdown timers or false scarcity	23
FDA-approved	A specific drug the FDA reviewed for safety, effectiveness and quality. Pharmacies are never "FDA-approved"	7
FDA label	The official prescribing information for an approved drug	20
Food noise	Patient term for constant thoughts about food	2
foundingDate	The schema property for when an organization was founded	25
FTC	Federal Trade Commission, which polices advertising claims	7
Gastroparesis	Very slow stomach emptying	6
Gated	Blocked from automated access	24
GIP	A second gut hormone; tirzepatide activates both GIP and GLP-1 receptors	2

Term	Simple meaning	Ch.
GLP-1	Glucagon-like peptide-1, a gut hormone that affects insulin, stomach emptying and fullness	2
GLP-1 receptor agonist	A drug that activates the GLP-1 receptor (e.g. semaglutide)	2
Google Ads certification	Google's approval to advertise restricted healthcare services	9
GSC / GA4 / GTM	Search Console / Analytics / Tag Manager	26
Health Breach Notification Rule	An FTC rule requiring breach notices from non-HIPAA health apps	12
HIPAA	The U.S. health-privacy law (1996)	12
Hub	A main page linking to related supporting pages	24
Hype	Exaggerated claims ("miracle", "melt fat")	17
In-office vitals	Weight, blood pressure and similar measurements taken in person	15
Intake	The online screening a patient completes	13
Integration	A connection that passes data between systems	13
Keyword targeting	Choosing search terms that trigger an ad	9
Kit	Supplies, instructions and tutorials shipped with the medication	14
Lanham Act	The U.S. federal law on trademarks and false advertising	9
LegitScript	Certification Google requires for telemedicine advertisers; Ongo is certified	7
Licensure	Legal permission to practice or operate in a state	8
Liraglutide	Older daily GLP-1; brands Saxenda and Victoza, now also generic	3
List price	The manufacturer's official price before discounts or insurance	10
Major adverse cardiovascular events (MACE)	Heart attack, stroke or heart-related death	5
Manufacturer program	A brand maker's direct cash-pay pharmacy (e.g. NovoCare, LillyDirect)	9

Term	Simple meaning	Ch.
MASLD	Metabolic dysfunction-associated steatotic liver disease (fatty liver)	1
Mission	The one-sentence reason the company exists	16
MOFU	Middle of funnel: people comparing options	15
Money page	A page meant to convert visitors into patients	18
MTC / MEN 2	Medullary thyroid carcinoma / Multiple Endocrine Neoplasia type 2	6
Must fix	An issue with compliance or major SEO risk	25
NAP	Name, address, phone: consistent business details across the web	26
Next.js	The web framework Ongo's marketing site is built on	13
NHANES	The CDC's national health and nutrition survey	19
Nofollow	A hint telling Google not to follow links on a page	18
Noindex	A tag telling Google not to show a page in search results	13
Non-responder	A person who loses little weight on a treatment	6
Obesity	A chronic disease of excess body fat that harms health; usually screened as BMI 30+	1
Objection	A reason a person hesitates to act	19
Off-label	Prescribing a drug for a use not on its FDA label	3
Oral GLP-1	A GLP-1 taken as a pill (Rybelsus, Wegovy pill, Foundayo)	3
Orforglipron	Lilly's small-molecule daily GLP-1 pill, brand Foundayo	3
Organization schema	Machine-readable code describing a company (name, founder, founding date)	16
Overweight	BMI 25–29.9	1
Pancreatitis	Inflammation of the pancreas	6

Term	Simple meaning	Ch.
Parent site	The main brand website (ongoweightloss.com)	18
Partner pharmacy	The licensed U.S. pharmacy that prepares and ships our medication	9
Patent	A time-limited legal right to stop others making or selling an invention	9
Patient Bill of Rights	A published statement of patient rights	25
Patient portal	The logged-in area where PHI lives	13
PCOS	Polycystic ovary syndrome	20
Persona	A fictional profile representing a group of real patients	19
Person-first language	Putting the person before the condition ("a person with obesity")	1
Phase 3	Large late-stage trial before an FDA application	3
PHI	Protected health information	12
Placebo	An inactive treatment used for comparison	5
Plan	A monthly care-and-medication package (Kickstart, Momentum, Transform)	22
Prediabetes	Blood sugar higher than normal but below the diabetes range	20
Prevalence	The share of a population with a condition	19
Price hygiene	Keeping the same price everywhere it appears	22
Prior authorization	Insurer approval required before covering a drug	9
Privacy notice	A published explanation of how we collect and use data	8
Product schema	Machine-readable code describing a product and its price	22
Provider	A licensed prescriber (MD, DO, NP or PA, depending on state)	11
Provider interface	The system providers use to review and prescribe	13
Provider-reviewed	Checked and signed off by a named licensed provider	17

Term	Simple meaning	Ch.
Refund policy	The published rules for cancellations and refunds	22
Research peptides	Unapproved products sold "for research"; never for patients	3
Salt form	e.g. semaglutide acetate; treated by the FDA as a different, unapproved ingredient	4
Schema	Machine-readable code describing a page	26
Screening	The short online health questionnaire	14
Search intent	What a person actually wants when they type a search	10
SELECT	Wegovy's heart-outcomes trial	5
Self-pay	Patient pays directly; no insurance billing	9
Semaglutide	GLP-1 drug; brands Wegovy (weight), Ozempic and Rybelsus (diabetes), made by Novo Nordisk	3
SEO	Search Engine Optimization	26
Served state	A state where Ongo actively evaluates and ships	8
Severe obesity	BMI 40 or higher (class 3)	1
Silo	A group of related pages on one topic	17
Silo / Hub-and-spoke	Grouping related pages / a hub linked to related spokes	18
Sister brand	A separate brand under the same leadership (My MMJ Doctor)	17
Sitemap (XML)	A file listing a site's URLs for search engines	24
Starting BMI	A patient's BMI before beginning treatment	21
STEP 1	Wegovy's main weight-loss trial (68 weeks)	5
SURMOUNT-1	Zepbound's main weight-loss trial (72 weeks)	5
Telehealth	Healthcare delivered remotely by video, phone or secure messaging	11
Testimonial	A patient's statement about their experience	23

Term	Simple meaning	Ch.
Tirzepatide	Dual GIP/GLP-1 drug; brands Zepbound (weight) and Mounjaro (diabetes), made by Eli Lilly	3
Titration	Starting low and increasing the dose step by step	2
TOFU / MOFU / BOFU	Top / middle / bottom of funnel: learning, comparing, deciding	19
Topical depth	How thoroughly a site covers a topic	24
Tracking technology	Pixels, tags or scripts that collect user activity	12
Trademark	A protected brand name or logo (e.g. "Wegovy")	9
Treatment plan	Medication, dosing approach, tracking and support in one plan	14
Triple agonist	A drug that activates three receptors (GLP-1, GIP and glucagon)	2
Trust signal	Evidence of legitimacy, such as LegitScript certification or a named reviewer	11
USP <797>	The national standard for sterile compounding	4
Vertical	A business area (e.g. weight loss, longevity)	17
Warning letter	A formal FDA notice of violations	7
Weight-related condition	A health problem linked to excess weight, e.g. high blood pressure or sleep apnea	20
YMYL	"Your Money or Your Life": topics held to the highest standard	6

MCQ Answer Keys (all chapters)

Try every quiz first, then turn the page upside down to check.

Chapter	Answers
1	1-C · 2-D · 3-A · 4-D · 5-B · 6-D · 7-C · 8-A · 9-B · 10-C
2	1-A · 2-A · 3-D · 4-C · 5-D · 6-A · 7-D · 8-C · 9-D · 10-A
3	1-B · 2-C · 3-A · 4-A · 5-A · 6-D · 7-B · 8-C · 9-C · 10-C
4	1-D · 2-A · 3-A · 4-A · 5-B · 6-C · 7-D · 8-A · 9-A · 10-D
5	1-D · 2-C · 3-B · 4-D · 5-B · 6-B · 7-D · 8-C · 9-D · 10-A
6	1-A · 2-C · 3-D · 4-D · 5-D · 6-C · 7-C · 8-B · 9-A · 10-C
7	1-B · 2-A · 3-A · 4-C · 5-A · 6-A · 7-C · 8-D · 9-A · 10-C
8	1-B · 2-C · 3-D · 4-B · 5-B · 6-D · 7-B · 8-C · 9-C · 10-C
9	1-D · 2-D · 3-B · 4-A · 5-A · 6-D · 7-B · 8-A · 9-D · 10-C
10	1-A · 2-A · 3-C · 4-C · 5-B · 6-D · 7-B · 8-D · 9-D · 10-B
11	1-B · 2-A · 3-D · 4-B · 5-C · 6-A · 7-C · 8-D · 9-B · 10-A
12	1-B · 2-D · 3-A · 4-D · 5-A · 6-D · 7-A · 8-B · 9-B · 10-D
13	1-A · 2-B · 3-A · 4-C · 5-A · 6-D · 7-A · 8-A · 9-D · 10-B
14	1-A · 2-D · 3-B · 4-B · 5-D · 6-A · 7-A · 8-B · 9-D · 10-B
15	1-A · 2-C · 3-B · 4-A · 5-D · 6-A · 7-C · 8-A · 9-C · 10-D
16	1-C · 2-A · 3-A · 4-C · 5-D · 6-A · 7-C · 8-C · 9-C · 10-A

Chapter	Answers
17	1-C · 2-A · 3-A · 4-C · 5-C · 6-A · 7-B · 8-C · 9-C · 10-B
18	1-C · 2-D · 3-A · 4-D · 5-D · 6-B · 7-D · 8-D · 9-B · 10-A
19	1-C · 2-C · 3-B · 4-C · 5-A · 6-A · 7-C · 8-B · 9-D · 10-B
20	1-D · 2-B · 3-B · 4-C · 5-D · 6-C · 7-D · 8-D · 9-C · 10-A
21	1-C · 2-A · 3-D · 4-A · 5-B · 6-C · 7-B · 8-B · 9-B · 10-D
22	1-A · 2-C · 3-D · 4-C · 5-C · 6-A · 7-D · 8-A · 9-C · 10-B
23	1-B · 2-B · 3-A · 4-B · 5-B · 6-B · 7-A · 8-D · 9-D · 10-B
24	1-B · 2-D · 3-C · 4-A · 5-A · 6-C · 7-C · 8-C · 9-C · 10-C
25	1-B · 2-B · 3-B · 4-A · 5-D · 6-B · 7-D · 8-A · 9-A · 10-B
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A healthcare brand first.

People arriving at Ongo have often spent years being judged about their weight, and many are nervous about a new medication. Every page, video, post and line of code we make should be written with that in mind.

Edition: October 2026. Approvals, rules and prices change; every source is dated so you can recheck it.

Prepared by **Ashish Osten**, Team Lead, Ongo Weight Loss.

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